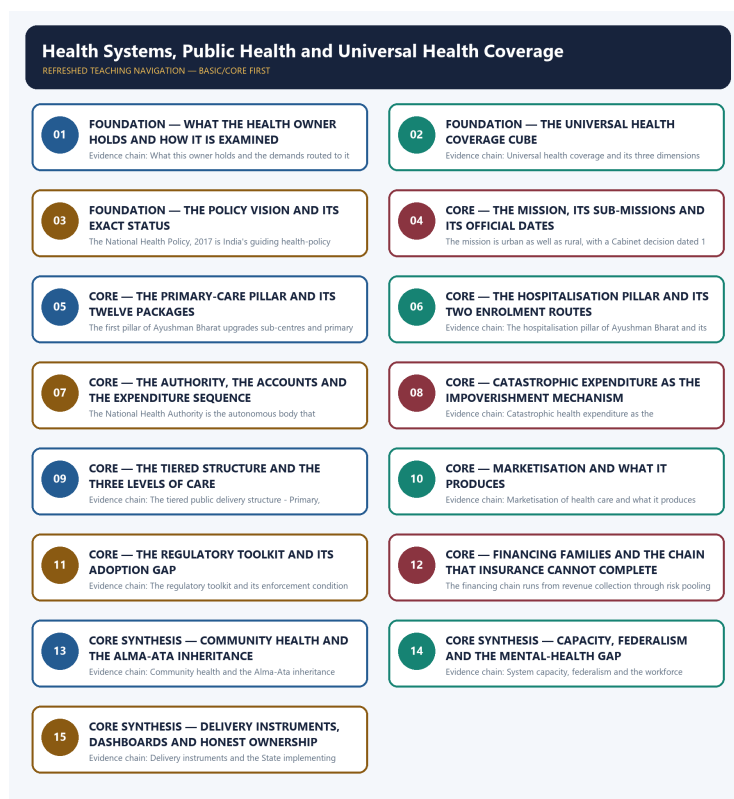


SOLVED PRACTICE WORKBOOK

Health Systems, Public Health and Universal Health Coverage - Solved Practice Workbook

UPSC complete learning session | Foundation to advanced | Concepts, evidence, practice and answer writing



Original deterministic study visual; labels are explanatory, not textual quotations.

Source order: repository knowledge -> official current linkage -> clearly marked analysis. No fabricated PYQs or statistics.

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BASIC MCQS / REMEDIATION

Q1. Which statement correctly identifies What this owner holds and the demands routed to it?

A. This topic owns the health half of the capability argument, namely the universal-health-coverage standard, the policy vision, the mission that funds public delivery, the two pillars of the flagship health programme, the tiered public infrastructure and the financial-protection measure by which the whole system is judged, and its distinctive feature is the densest direct-question record in the folder: four General Studies Mains demands from 2018, 2020, 2021 and 2024 and three objective demands from 2023 and 2024 are routed to it in the audited ledgers; the macro economics of health financing belongs to the Economy owner, the elderly-care share of the 2020 demand is jointly owned with the senior-citizens topic, and this topic argues coverage, provision and protection. B. The National Health Policy, 2017 is India's guiding health-policy document, and the owners record its goals as raising public health expenditure to two and a half per cent of gross domestic product, strengthening primary care, achieving universal health coverage and reducing out-of-pocket expenditure; the examinable discipline is that it is a policy document and not an enacted statute, so its targets are commitments rather than justiciable entitlements, and describing them as legally binding is a status error of exactly the kind this folder is built to prevent. C. Universal health coverage means ensuring that all people have access to the promotive, preventive, curative and rehabilitative health services they need, of sufficient quality, without suffering financial hardship, and it is analysed on three dimensions that the World Health Organization presents as a cube: population, which is the breadth of who is covered, services, which is the depth of what is covered, and financial protection, which is the height of how little the user pays at the point of service; the analytical value of the cube is that a system can expand on one dimension while contracting on another, so an answer that reports only beneficiary numbers has measured one edge of a three-dimensional claim. D. The National Health Mission's own official page, read on 2 September 2026, records that the National Rural Health Mission was launched by the Prime Minister on 12 April 2005 to provide accessible, affordable and quality health care to the rural population and especially to vulnerable groups, and that the Union Cabinet, by its decision dated 1 May 2013, approved the launch of the National Urban Health Mission as a sub-mission of an over-arching National Health Mission with the National Rural Health Mission being the other sub-mission; the same page records special focus on the Empowered Action Group States, the North Eastern States, Jammu and Kashmir and Himachal Pradesh, a thrust on a fully functional, community-owned, decentralised health delivery system with inter-sectoral convergence acting on determinants such as water, sanitation, education, nutrition and social and gender equality, and measurement of facilities against the Indian Public Health Standards, so the mission is urban as well as rural and treating it as rural-only is a factual error.

Answer: A. Explanation: This topic owns the health half of the capability argument, namely the universal-health-coverage standard, the policy vision, the mission that funds public delivery, the two pillars of the flagship health programme, the tiered public infrastructure and the financial-protection measure by which the whole system is judged, and its distinctive feature is the densest direct-question record in the folder: four General Studies Mains demands from 2018, 2020, 2021 and 2024 and three objective demands from 2023 and 2024 are routed to it in the audited ledgers; the macro economics of health financing belongs to the Economy owner, the elderly-care share of the 2020 demand is jointly owned with the senior-citizens topic, and this topic argues coverage, provision and protection. The remaining options belong to different chronology, actor or analytical categories.

Q2. Which chronology card should be filed under What this owner holds and the demands routed to it?

A. The National Health Policy, 2017 is India's guiding health-policy document, and the owners record its goals as raising public health expenditure to two and a half per cent of gross domestic product, strengthening primary care, achieving universal health coverage and reducing out-of-pocket expenditure; the examinable discipline is that it is a policy document and not an enacted statute, so its targets are commitments rather than justiciable entitlements, and describing them as legally binding is a status error of exactly the kind this folder is built to prevent. B. This topic owns the health half of the capability argument, namely the universal-health-coverage standard, the policy vision, the mission that funds public delivery, the two pillars of the flagship health programme, the tiered public infrastructure and the financial-protection measure by which the whole system is judged, and its distinctive feature is the densest direct-question record in the folder: four General Studies Mains demands from 2018, 2020, 2021 and 2024 and three objective demands from 2023 and 2024 are routed to it in the audited ledgers; the macro economics of health financing belongs to the Economy owner, the elderly-care share of the 2020 demand is jointly owned with the senior-citizens topic, and this topic argues coverage, provision and protection. C. The first pillar of Ayushman Bharat upgrades sub-centres and primary health centres into Health and Wellness Centres, now carried as Ayushman Arogya Mandirs, which deliver comprehensive primary health care through twelve service packages that extend beyond reproductive and child health to non-communicable diseases, mental health, ear, nose and throat care, ophthalmology, oral health, geriatric and palliative care and emergency care, staffed by a community health officer or mid-level health provider; the design intent is to shift care-seeking from hospitals to a local first-contact layer, which is why describing this pillar as reproductive and child health alone understates it. D. The National Health Mission's own official page, read on 2 September 2026, records that the National Rural Health Mission was launched by the Prime Minister on 12 April 2005 to provide accessible, affordable and quality health care to the rural population and especially to vulnerable groups, and that the Union Cabinet, by its decision dated 1 May 2013, approved the launch of the National Urban Health Mission as a sub-mission of an over-arching National Health Mission with the National Rural Health Mission being the other sub-mission; the same page records special focus on the Empowered Action Group States, the North Eastern States, Jammu and Kashmir and Himachal Pradesh, a thrust on a fully functional, community-owned, decentralised health delivery system with inter-sectoral convergence acting on determinants such as water, sanitation, education, nutrition and social and gender equality, and measurement of facilities against the Indian Public Health Standards, so the mission is urban as well as rural and treating it as rural-only is a factual error.

Answer: B. Explanation: This topic owns the health half of the capability argument, namely the universal-health-coverage standard, the policy vision, the mission that funds public delivery, the two pillars of the flagship health programme, the tiered public infrastructure and the financial-protection measure by which the whole system is judged, and its distinctive feature is the densest direct-question record in the folder: four General Studies Mains demands from 2018, 2020, 2021 and 2024 and three objective demands from 2023 and 2024 are routed to it in the audited ledgers; the macro economics of health financing belongs to the Economy owner, the elderly-care share of the 2020 demand is jointly owned with the senior-citizens topic, and this topic argues coverage, provision and protection. The remaining options belong to different chronology, actor or analytical categories.

Q3. Which option preserves the source-bounded meaning of What this owner holds and the demands routed to it?

A. The second pillar, Pradhan Mantri Jan Arogya Yojana, provides cashless secondary and tertiary hospitalisation cover of up to five lakh rupees per family per year at empanelled public and private hospitals for its core eligible households, which are identified through the deprivation criteria of the socio-economic and caste census, and the owners separately record a distinct expansion under which all citizens aged seventy years and above can enrol regardless of income or that census status; the discipline required is exact, because the core scheme is eligibility-based while the age-based expansion is universal only for the seventy-plus group, so an age-specific expansion must never be written up as universal coverage for every age. B. The National Health Mission's own official page, read on 2 September 2026, records that the National Rural Health Mission was launched by the Prime Minister on 12 April 2005 to provide accessible, affordable and quality health care to the rural population and especially to vulnerable groups, and that the Union Cabinet, by its decision dated 1 May 2013, approved the launch of the National Urban Health Mission as a sub-mission of an over-arching National Health Mission with the National Rural Health Mission being the other sub-mission; the same page records special focus on the Empowered Action Group States,

the North Eastern States, Jammu and Kashmir and Himachal Pradesh, a thrust on a fully functional, community-owned, decentralised health delivery system with inter-sectoral convergence acting on determinants such as water, sanitation, education, nutrition and social and gender equality, and measurement of facilities against the Indian Public Health Standards, so the mission is urban as well as rural and treating it as rural-only is a factual error. C. This topic owns the health half of the capability argument, namely the universal-health-coverage standard, the policy vision, the mission that funds public delivery, the two pillars of the flagship health programme, the tiered public infrastructure and the financial-protection measure by which the whole system is judged, and its distinctive feature is the densest direct-question record in the folder: four General Studies Mains demands from 2018, 2020, 2021 and 2024 and three objective demands from 2023 and 2024 are routed to it in the audited ledgers; the macro economics of health financing belongs to the Economy owner, the elderly-care share of the 2020 demand is jointly owned with the senior-citizens topic, and this topic argues coverage, provision and protection. D. The first pillar of Ayushman Bharat upgrades sub-centres and primary health centres into Health and Wellness Centres, now carried as Ayushman Arogya Mandirs, which deliver comprehensive primary health care through twelve service packages that extend beyond reproductive and child health to non-communicable diseases, mental health, ear, nose and throat care, ophthalmology, oral health, geriatric and palliative care and emergency care, staffed by a community health officer or mid-level health provider; the design intent is to shift care-seeking from hospitals to a local first-contact layer, which is why describing this pillar as reproductive and child health alone understates it.

Answer: C. Explanation: This topic owns the health half of the capability argument, namely the universal-health-coverage standard, the policy vision, the mission that funds public delivery, the two pillars of the flagship health programme, the tiered public infrastructure and the financial-protection measure by which the whole system is judged, and its distinctive feature is the densest direct-question record in the folder: four General Studies Mains demands from 2018, 2020, 2021 and 2024 and three objective demands from 2023 and 2024 are routed to it in the audited ledgers; the macro economics of health financing belongs to the Economy owner, the elderly-care share of the 2020 demand is jointly owned with the senior-citizens topic, and this topic argues coverage, provision and protection. The remaining options belong to different chronology, actor or analytical categories.

Q4. Which statement avoids a close-option trap about What this owner holds and the demands routed to it?

A. The National Health Authority is the autonomous body that implements the hospitalisation pillar, maintaining the beneficiary database, empanelling hospitals and processing claims, while the National Health Accounts estimates are the official source for the breakdown of health expenditure between public, private and out-of-pocket spending and are prepared through the national health accounts system; the owners record expressly that these two must not be confused, because one is an implementing agency and the other is an accounting estimate, and citing an expenditure share to the implementing agency is a sourcing error. B. The first pillar of Ayushman Bharat upgrades sub-centres and primary health centres into Health and Wellness Centres, now carried as Ayushman Arogya Mandirs, which deliver comprehensive primary health care through twelve service packages that extend beyond reproductive and child health to non-communicable diseases, mental health, ear, nose and throat care, ophthalmology, oral health, geriatric and palliative care and emergency care, staffed by a community health officer or mid-level health provider; the design intent is to shift care-seeking from hospitals to a local first-contact layer, which is why describing this pillar as reproductive and child health alone understates it. C. The second pillar, Pradhan Mantri Jan Arogya Yojana, provides cashless secondary and tertiary hospitalisation cover of up to five lakh rupees per family per year at empanelled public and private hospitals for its core eligible households, which are identified through the deprivation criteria of the socio-economic and caste census, and the owners separately record a distinct expansion under which all citizens aged seventy years and above can enrol regardless of income or that census status; the discipline required is exact, because the core scheme is eligibility-based while the age-based expansion is universal only for the seventy-plus group, so an age-specific expansion must never be written up as universal coverage for every age. D. This topic owns the health half of the capability argument, namely the universal-health-coverage standard, the policy vision, the mission that funds public delivery, the two pillars of the flagship health programme, the tiered public infrastructure and the financial-protection measure by which the whole system is judged, and its distinctive feature is the densest direct-question record in the folder: four General Studies Mains demands from 2018, 2020, 2021 and 2024 and three objective demands from 2023 and 2024 are routed to it in the audited ledgers; the macro economics of health financing belongs to the Economy owner, the elderly-care share of the 2020 demand is jointly owned with the

senior-citizens topic, and this topic argues coverage, provision and protection.

Answer: D. Explanation: This topic owns the health half of the capability argument, namely the universal-health-coverage standard, the policy vision, the mission that funds public delivery, the two pillars of the flagship health programme, the tiered public infrastructure and the financial-protection measure by which the whole system is judged, and its distinctive feature is the densest direct-question record in the folder: four General Studies Mains demands from 2018, 2020, 2021 and 2024 and three objective demands from 2023 and 2024 are routed to it in the audited ledgers; the macro economics of health financing belongs to the Economy owner, the elderly-care share of the 2020 demand is jointly owned with the senior-citizens topic, and this topic argues coverage, provision and protection. The remaining options belong to different chronology, actor or analytical categories.

Q5. Which statement correctly identifies Universal health coverage and its three dimensions?

A. Universal health coverage means ensuring that all people have access to the promotive, preventive, curative and rehabilitative health services they need, of sufficient quality, without suffering financial hardship, and it is analysed on three dimensions that the World Health Organization presents as a cube: population, which is the breadth of who is covered, services, which is the depth of what is covered, and financial protection, which is the height of how little the user pays at the point of service; the analytical value of the cube is that a system can expand on one dimension while contracting on another, so an answer that reports only beneficiary numbers has measured one edge of a three-dimensional claim. B. The first pillar of Ayushman Bharat upgrades sub-centres and primary health centres into Health and Wellness Centres, now carried as Ayushman Arogya Mandirs, which deliver comprehensive primary health care through twelve service packages that extend beyond reproductive and child health to non-communicable diseases, mental health, ear, nose and throat care, ophthalmology, oral health, geriatric and palliative care and emergency care, staffed by a community health officer or mid-level health provider; the design intent is to shift care-seeking from hospitals to a local first-contact layer, which is why describing this pillar as reproductive and child health alone understates it. C. The National Health Mission's own official page, read on 2 September 2026, records that the National Rural Health Mission was launched by the Prime Minister on 12 April 2005 to provide accessible, affordable and quality health care to the rural population and especially to vulnerable groups, and that the Union Cabinet, by its decision dated 1 May 2013, approved the launch of the National Urban Health Mission as a sub-mission of an over-arching National Health Mission with the National Rural Health Mission being the other sub-mission; the same page records special focus on the Empowered Action Group States, the North Eastern States, Jammu and Kashmir and Himachal Pradesh, a thrust on a fully functional, community-owned, decentralised health delivery system with inter-sectoral convergence acting on determinants such as water, sanitation, education, nutrition and social and gender equality, and measurement of facilities against the Indian Public Health Standards, so the mission is urban as well as rural and treating it as rural-only is a factual error. D. The National Health Policy, 2017 is India's guiding health-policy document, and the owners record its goals as raising public health expenditure to two and a half per cent of gross domestic product, strengthening primary care, achieving universal health coverage and reducing out-of-pocket expenditure; the examinable discipline is that it is a policy document and not an enacted statute, so its targets are commitments rather than justiciable entitlements, and describing them as legally binding is a status error of exactly the kind this folder is built to prevent.

Answer: A. Explanation: Universal health coverage means ensuring that all people have access to the promotive, preventive, curative and rehabilitative health services they need, of sufficient quality, without suffering financial hardship, and it is analysed on three dimensions that the World Health Organization presents as a cube: population, which is the breadth of who is covered, services, which is the depth of what is covered, and financial protection, which is the height of how little the user pays at the point of service; the analytical value of the cube is that a system can expand on one dimension while contracting on another, so an answer that reports only beneficiary numbers has measured one edge of a three-dimensional claim. The remaining options belong to different chronology, actor or analytical categories.

Q6. Which chronology card should be filed under Universal health coverage and its three dimensions?

A. The National Health Mission's own official page, read on 2 September 2026, records that the National Rural Health Mission was launched by the Prime Minister on 12 April 2005 to provide accessible, affordable and quality health care to the rural population and especially to vulnerable groups, and that the Union Cabinet, by its decision dated 1 May 2013, approved the launch of the National Urban Health Mission as a sub-mission of an over-arching National Health Mission with the National Rural Health Mission being the other sub-mission; the same page records special focus on the Empowered Action Group States, the North Eastern States, Jammu and Kashmir and Himachal Pradesh, a thrust on a fully functional, community-owned, decentralised health delivery system with inter-sectoral convergence acting on determinants such as water, sanitation, education, nutrition and social and gender equality, and measurement of facilities against the Indian Public Health Standards, so the mission is urban as well as rural and treating it as rural-only is a factual error. B. Universal health coverage means ensuring that all people have access to the promotive, preventive, curative and rehabilitative health services they need, of sufficient quality, without suffering financial hardship, and it is analysed on three dimensions that the World Health Organization presents as a cube: population, which is the breadth of who is covered, services, which is the depth of what is covered, and financial protection, which is the height of how little the user pays at the point of service; the analytical value of the cube is that a system can expand on one dimension while contracting on another, so an answer that reports only beneficiary numbers has measured one edge of a three-dimensional claim. C. The second pillar, Pradhan Mantri Jan Arogya Yojana, provides cashless secondary and tertiary hospitalisation cover of up to five lakh rupees per family per year at empanelled public and private hospitals for its core eligible households, which are identified through the deprivation criteria of the socio-economic and caste census, and the owners separately record a distinct expansion under which all citizens aged seventy years and above can enrol regardless of income or that census status; the discipline required is exact, because the core scheme is eligibility-based while the age-based expansion is universal only for the seventy-plus group, so an age-specific expansion must never be written up as universal coverage for every age. D. The first pillar of Ayushman Bharat upgrades sub-centres and primary health centres into Health and Wellness Centres, now carried as Ayushman Arogya Mandirs, which deliver comprehensive primary health care through twelve service packages that extend beyond reproductive and child health to non-communicable diseases, mental health, ear, nose and throat care, ophthalmology, oral health, geriatric and palliative care and emergency care, staffed by a community health officer or mid-level health provider; the design intent is to shift care-seeking from hospitals to a local first-contact layer, which is why describing this pillar as reproductive and child health alone understates it.

Answer: B. Explanation: Universal health coverage means ensuring that all people have access to the promotive, preventive, curative and rehabilitative health services they need, of sufficient quality, without suffering financial hardship, and it is analysed on three dimensions that the World Health Organization presents as a cube: population, which is the breadth of who is covered, services, which is the depth of what is covered, and financial protection, which is the height of how little the user pays at the point of service; the analytical value of the cube is that a system can expand on one dimension while contracting on another, so an answer that reports only beneficiary numbers has measured one edge of a three-dimensional claim. The remaining options belong to different chronology, actor or analytical categories.

Q7. Which option preserves the source-bounded meaning of Universal health coverage and its three dimensions?

A. The second pillar, Pradhan Mantri Jan Arogya Yojana, provides cashless secondary and tertiary hospitalisation cover of up to five lakh rupees per family per year at empanelled public and private hospitals for its core eligible households, which are identified through the deprivation criteria of the socio-economic and caste census, and the owners separately record a distinct expansion under which all citizens aged seventy years and above can enrol regardless of income or that census status; the discipline required is exact, because the core scheme is eligibility-based while the age-based expansion is universal only for the seventy-plus group, so an age-specific expansion must never be written up as universal coverage for every age. B. The National Health Authority is the autonomous body that implements the hospitalisation pillar, maintaining the beneficiary database, empanelling hospitals and processing claims, while the National Health Accounts estimates are the official source for the breakdown of health expenditure between public, private and out-of-pocket spending and are prepared through the national health accounts system; the owners record expressly that these two must not be confused, because one is an implementing agency and the other is an accounting estimate, and citing an expenditure share to the implementing

agency is a sourcing error. C. Universal health coverage means ensuring that all people have access to the promotive, preventive, curative and rehabilitative health services they need, of sufficient quality, without suffering financial hardship, and it is analysed on three dimensions that the World Health Organization presents as a cube: population, which is the breadth of who is covered, services, which is the depth of what is covered, and financial protection, which is the height of how little the user pays at the point of service; the analytical value of the cube is that a system can expand on one dimension while contracting on another, so an answer that reports only beneficiary numbers has measured one edge of a three-dimensional claim. D. The first pillar of Ayushman Bharat upgrades sub-centres and primary health centres into Health and Wellness Centres, now carried as Ayushman Arogya Mandirs, which deliver comprehensive primary health care through twelve service packages that extend beyond reproductive and child health to non-communicable diseases, mental health, ear, nose and throat care, ophthalmology, oral health, geriatric and palliative care and emergency care, staffed by a community health officer or mid-level health provider; the design intent is to shift care-seeking from hospitals to a local first-contact layer, which is why describing this pillar as reproductive and child health alone understates it.

Answer: C. Explanation: Universal health coverage means ensuring that all people have access to the promotive, preventive, curative and rehabilitative health services they need, of sufficient quality, without suffering financial hardship, and it is analysed on three dimensions that the World Health Organization presents as a cube: population, which is the breadth of who is covered, services, which is the depth of what is covered, and financial protection, which is the height of how little the user pays at the point of service; the analytical value of the cube is that a system can expand on one dimension while contracting on another, so an answer that reports only beneficiary numbers has measured one edge of a three-dimensional claim. The remaining options belong to different chronology, actor or analytical categories.

Q8. Which statement avoids a close-option trap about Universal health coverage and its three dimensions?

A. The second pillar, Pradhan Mantri Jan Arogya Yojana, provides cashless secondary and tertiary hospitalisation cover of up to five lakh rupees per family per year at empanelled public and private hospitals for its core eligible households, which are identified through the deprivation criteria of the socio-economic and caste census, and the owners separately record a distinct expansion under which all citizens aged seventy years and above can enrol regardless of income or that census status; the discipline required is exact, because the core scheme is eligibility-based while the age-based expansion is universal only for the seventy-plus group, so an age-specific expansion must never be written up as universal coverage for every age. B. Out-of-pocket expenditure is health spending paid directly by households at the point of service, and the owners record the National Health Accounts sequence as 47.1 per cent of total health expenditure in 2019-20, 44.4 per cent in 2020-21 and 39.4 per cent in 2021-22, with the latest cited official estimate being 39.4 per cent for the accounting year 2021-22; two disciplines follow, first that the publication date must never be confused with the accounting year, and second that a falling national share is a dated aggregate rather than evidence that every household faces a lighter burden, and high out-of-pocket expenditure indicates inadequate public provision rather than high utilisation. C. The National Health Authority is the autonomous body that implements the hospitalisation pillar, maintaining the beneficiary database, empanelling hospitals and processing claims, while the National Health Accounts estimates are the official source for the breakdown of health expenditure between public, private and out-of-pocket spending and are prepared through the national health accounts system; the owners record expressly that these two must not be confused, because one is an implementing agency and the other is an accounting estimate, and citing an expenditure share to the implementing agency is a sourcing error. D. Universal health coverage means ensuring that all people have access to the promotive, preventive, curative and rehabilitative health services they need, of sufficient quality, without suffering financial hardship, and it is analysed on three dimensions that the World Health Organization presents as a cube: population, which is the breadth of who is covered, services, which is the depth of what is covered, and financial protection, which is the height of how little the user pays at the point of service; the analytical value of the cube is that a system can expand on one dimension while contracting on another, so an answer that reports only beneficiary numbers has measured one edge of a three-dimensional claim.

Answer: D. Explanation: Universal health coverage means ensuring that all people have access to the promotive, preventive, curative and rehabilitative health services they need, of sufficient quality, without suffering financial hardship, and it is analysed on three dimensions that the World Health Organization presents as a cube: population, which is the breadth of who is covered, services, which is the depth of what

is covered, and financial protection, which is the height of how little the user pays at the point of service; the analytical value of the cube is that a system can expand on one dimension while contracting on another, so an answer that reports only beneficiary numbers has measured one edge of a three-dimensional claim. The remaining options belong to different chronology, actor or analytical categories.

Q9. Which statement correctly identifies The National Health Policy of 2017 and what it is not?

A. The National Health Policy, 2017 is India's guiding health-policy document, and the owners record its goals as raising public health expenditure to two and a half per cent of gross domestic product, strengthening primary care, achieving universal health coverage and reducing out-of-pocket expenditure; the examinable discipline is that it is a policy document and not an enacted statute, so its targets are commitments rather than justiciable entitlements, and describing them as legally binding is a status error of exactly the kind this folder is built to prevent. B. The National Health Mission's own official page, read on 2 September 2026, records that the National Rural Health Mission was launched by the Prime Minister on 12 April 2005 to provide accessible, affordable and quality health care to the rural population and especially to vulnerable groups, and that the Union Cabinet, by its decision dated 1 May 2013, approved the launch of the National Urban Health Mission as a sub-mission of an over-arching National Health Mission with the National Rural Health Mission being the other sub-mission; the same page records special focus on the Empowered Action Group States, the North Eastern States, Jammu and Kashmir and Himachal Pradesh, a thrust on a fully functional, community-owned, decentralised health delivery system with inter-sectoral convergence acting on determinants such as water, sanitation, education, nutrition and social and gender equality, and measurement of facilities against the Indian Public Health Standards, so the mission is urban as well as rural and treating it as rural-only is a factual error. C. The first pillar of Ayushman Bharat upgrades sub-centres and primary health centres into Health and Wellness Centres, now carried as Ayushman Arogya Mandirs, which deliver comprehensive primary health care through twelve service packages that extend beyond reproductive and child health to non-communicable diseases, mental health, ear, nose and throat care, ophthalmology, oral health, geriatric and palliative care and emergency care, staffed by a community health officer or mid-level health provider; the design intent is to shift care-seeking from hospitals to a local first-contact layer, which is why describing this pillar as reproductive and child health alone understates it. D. The second pillar, Pradhan Mantri Jan Arogya Yojana, provides cashless secondary and tertiary hospitalisation cover of up to five lakh rupees per family per year at empanelled public and private hospitals for its core eligible households, which are identified through the deprivation criteria of the socio-economic and caste census, and the owners separately record a distinct expansion under which all citizens aged seventy years and above can enrol regardless of income or that census status; the discipline required is exact, because the core scheme is eligibility-based while the age-based expansion is universal only for the seventy-plus group, so an age-specific expansion must never be written up as universal coverage for every age.

Answer: A. Explanation: The National Health Policy, 2017 is India's guiding health-policy document, and the owners record its goals as raising public health expenditure to two and a half per cent of gross domestic product, strengthening primary care, achieving universal health coverage and reducing out-of-pocket expenditure; the examinable discipline is that it is a policy document and not an enacted statute, so its targets are commitments rather than justiciable entitlements, and describing them as legally binding is a status error of exactly the kind this folder is built to prevent. The remaining options belong to different chronology, actor or analytical categories.

Q10. Which chronology card should be filed under The National Health Policy of 2017 and what it is not?

A. The first pillar of Ayushman Bharat upgrades sub-centres and primary health centres into Health and Wellness Centres, now carried as Ayushman Arogya Mandirs, which deliver comprehensive primary health care through twelve service packages that extend beyond reproductive and child health to non-communicable diseases, mental health, ear, nose and throat care, ophthalmology, oral health, geriatric and palliative care and emergency care, staffed by a community health officer or mid-level health provider; the design intent is to shift care-seeking from hospitals to a local first-contact layer, which is why describing this pillar as reproductive and child health alone understates it. B. The National Health Policy, 2017 is India's guiding health-policy document, and the owners record its goals as raising public health expenditure to two and a half per cent of gross domestic product, strengthening primary care, achieving universal health coverage and reducing out-of-pocket expenditure; the examinable discipline is that it is a policy document and not an enacted statute, so its targets are commitments rather than justiciable entitlements, and

describing them as legally binding is a status error of exactly the kind this folder is built to prevent. C. The second pillar, Pradhan Mantri Jan Arogya Yojana, provides cashless secondary and tertiary hospitalisation cover of up to five lakh rupees per family per year at empanelled public and private hospitals for its core eligible households, which are identified through the deprivation criteria of the socio-economic and caste census, and the owners separately record a distinct expansion under which all citizens aged seventy years and above can enrol regardless of income or that census status; the discipline required is exact, because the core scheme is eligibility-based while the age-based expansion is universal only for the seventy-plus group, so an age-specific expansion must never be written up as universal coverage for every age. D. The National Health Authority is the autonomous body that implements the hospitalisation pillar, maintaining the beneficiary database, empanelling hospitals and processing claims, while the National Health Accounts estimates are the official source for the breakdown of health expenditure between public, private and out-of-pocket spending and are prepared through the national health accounts system; the owners record expressly that these two must not be confused, because one is an implementing agency and the other is an accounting estimate, and citing an expenditure share to the implementing agency is a sourcing error.

Answer: B. Explanation: The National Health Policy, 2017 is India's guiding health-policy document, and the owners record its goals as raising public health expenditure to two and a half per cent of gross domestic product, strengthening primary care, achieving universal health coverage and reducing out-of-pocket expenditure; the examinable discipline is that it is a policy document and not an enacted statute, so its targets are commitments rather than justiciable entitlements, and describing them as legally binding is a status error of exactly the kind this folder is built to prevent. The remaining options belong to different chronology, actor or analytical categories.

Q11. Which option preserves the source-bounded meaning of The National Health Policy of 2017 and what it is not?

A. The second pillar, Pradhan Mantri Jan Arogya Yojana, provides cashless secondary and tertiary hospitalisation cover of up to five lakh rupees per family per year at empanelled public and private hospitals for its core eligible households, which are identified through the deprivation criteria of the socio-economic and caste census, and the owners separately record a distinct expansion under which all citizens aged seventy years and above can enrol regardless of income or that census status; the discipline required is exact, because the core scheme is eligibility-based while the age-based expansion is universal only for the seventy-plus group, so an age-specific expansion must never be written up as universal coverage for every age. B. The National Health Authority is the autonomous body that implements the hospitalisation pillar, maintaining the beneficiary database, empanelling hospitals and processing claims, while the National Health Accounts estimates are the official source for the breakdown of health expenditure between public, private and out-of-pocket spending and are prepared through the national health accounts system; the owners record expressly that these two must not be confused, because one is an implementing agency and the other is an accounting estimate, and citing an expenditure share to the implementing agency is a sourcing error. C. The National Health Policy, 2017 is India's guiding health-policy document, and the owners record its goals as raising public health expenditure to two and a half per cent of gross domestic product, strengthening primary care, achieving universal health coverage and reducing out-of-pocket expenditure; the examinable discipline is that it is a policy document and not an enacted statute, so its targets are commitments rather than justiciable entitlements, and describing them as legally binding is a status error of exactly the kind this folder is built to prevent. D. Out-of-pocket expenditure is health spending paid directly by households at the point of service, and the owners record the National Health Accounts sequence as 47.1 per cent of total health expenditure in 2019-20, 44.4 per cent in 2020-21 and 39.4 per cent in 2021-22, with the latest cited official estimate being 39.4 per cent for the accounting year 2021-22; two disciplines follow, first that the publication date must never be confused with the accounting year, and second that a falling national share is a dated aggregate rather than evidence that every household faces a lighter burden, and high out-of-pocket expenditure indicates inadequate public provision rather than high utilisation.

Answer: C. Explanation: The National Health Policy, 2017 is India's guiding health-policy document, and the owners record its goals as raising public health expenditure to two and a half per cent of gross domestic product, strengthening primary care, achieving universal health coverage and reducing out-of-pocket expenditure; the examinable discipline is that it is a policy document and not an enacted statute, so its targets are commitments rather than justiciable entitlements, and describing them as legally binding is a status error of exactly the kind this folder is built to prevent. The remaining options belong to different

Q12. Which statement avoids a close-option trap about The National Health Policy of 2017 and what it is not?

A. Catastrophic health expenditure is health spending that exceeds a threshold share of household income or consumption, commonly stated in the range of ten to twenty-five per cent, and it pushes households into poverty or debt, with high out-of-pocket expenditure as its primary driver; this is the mechanism that makes health a social-justice question rather than only a service-delivery question, because a single hospitalisation can undo years of asset accumulation, and it is the bridge that connects this topic to the poverty and nutrition owner. B. The National Health Authority is the autonomous body that implements the hospitalisation pillar, maintaining the beneficiary database, empanelling hospitals and processing claims, while the National Health Accounts estimates are the official source for the breakdown of health expenditure between public, private and out-of-pocket spending and are prepared through the national health accounts system; the owners record expressly that these two must not be confused, because one is an implementing agency and the other is an accounting estimate, and citing an expenditure share to the implementing agency is a sourcing error. C. Out-of-pocket expenditure is health spending paid directly by households at the point of service, and the owners record the National Health Accounts sequence as 47.1 per cent of total health expenditure in 2019-20, 44.4 per cent in 2020-21 and 39.4 per cent in 2021-22, with the latest cited official estimate being 39.4 per cent for the accounting year 2021-22; two disciplines follow, first that the publication date must never be confused with the accounting year, and second that a falling national share is a dated aggregate rather than evidence that every household faces a lighter burden, and high out-of-pocket expenditure indicates inadequate public provision rather than high utilisation. D. The National Health Policy, 2017 is India's guiding health-policy document, and the owners record its goals as raising public health expenditure to two and a half per cent of gross domestic product, strengthening primary care, achieving universal health coverage and reducing out-of-pocket expenditure; the examinable discipline is that it is a policy document and not an enacted statute, so its targets are commitments rather than justiciable entitlements, and describing them as legally binding is a status error of exactly the kind this folder is built to prevent.

Answer: D. Explanation: The National Health Policy, 2017 is India's guiding health-policy document, and the owners record its goals as raising public health expenditure to two and a half per cent of gross domestic product, strengthening primary care, achieving universal health coverage and reducing out-of-pocket expenditure; the examinable discipline is that it is a policy document and not an enacted statute, so its targets are commitments rather than justiciable entitlements, and describing them as legally binding is a status error of exactly the kind this folder is built to prevent. The remaining options belong to different chronology, actor or analytical categories.

Q13. Which statement correctly identifies The National Health Mission and its two sub-missions?

A. The National Health Mission's own official page, read on 2 September 2026, records that the National Rural Health Mission was launched by the Prime Minister on 12 April 2005 to provide accessible, affordable and quality health care to the rural population and especially to vulnerable groups, and that the Union Cabinet, by its decision dated 1 May 2013, approved the launch of the National Urban Health Mission as a sub-mission of an over-arching National Health Mission with the National Rural Health Mission being the other sub-mission; the same page records special focus on the Empowered Action Group States, the North Eastern States, Jammu and Kashmir and Himachal Pradesh, a thrust on a fully functional, community-owned, decentralised health delivery system with inter-sectoral convergence acting on determinants such as water, sanitation, education, nutrition and social and gender equality, and measurement of facilities against the Indian Public Health Standards, so the mission is urban as well as rural and treating it as rural-only is a factual error. B. The second pillar, Pradhan Mantri Jan Arogya Yojana, provides cashless secondary and tertiary hospitalisation cover of up to five lakh rupees per family per year at empanelled public and private hospitals for its core eligible households, which are identified through the deprivation criteria of the socio-economic and caste census, and the owners separately record a distinct expansion under which all citizens aged seventy years and above can enrol regardless of income or that census status; the discipline required is exact, because the core scheme is eligibility-based while the age-based expansion is universal only for the seventy-plus group, so an age-specific expansion must never be written up as universal coverage for every age. C. The first pillar of Ayushman Bharat upgrades sub-centres and primary health centres into Health and Wellness Centres, now carried as Ayushman Arogya Mandirs, which deliver comprehensive primary health care through twelve service packages that extend

beyond reproductive and child health to non-communicable diseases, mental health, ear, nose and throat care, ophthalmology, oral health, geriatric and palliative care and emergency care, staffed by a community health officer or mid-level health provider; the design intent is to shift care-seeking from hospitals to a local first-contact layer, which is why describing this pillar as reproductive and child health alone understates it. D. The National Health Authority is the autonomous body that implements the hospitalisation pillar, maintaining the beneficiary database, empanelling hospitals and processing claims, while the National Health Accounts estimates are the official source for the breakdown of health expenditure between public, private and out-of-pocket spending and are prepared through the national health accounts system; the owners record expressly that these two must not be confused, because one is an implementing agency and the other is an accounting estimate, and citing an expenditure share to the implementing agency is a sourcing error.

Answer: A. Explanation: The National Health Mission's own official page, read on 2 September 2026, records that the National Rural Health Mission was launched by the Prime Minister on 12 April 2005 to provide accessible, affordable and quality health care to the rural population and especially to vulnerable groups, and that the Union Cabinet, by its decision dated 1 May 2013, approved the launch of the National Urban Health Mission as a sub-mission of an over-arching National Health Mission with the National Rural Health Mission being the other sub-mission; the same page records special focus on the Empowered Action Group States, the North Eastern States, Jammu and Kashmir and Himachal Pradesh, a thrust on a fully functional, community-owned, decentralised health delivery system with inter-sectoral convergence acting on determinants such as water, sanitation, education, nutrition and social and gender equality, and measurement of facilities against the Indian Public Health Standards, so the mission is urban as well as rural and treating it as rural-only is a factual error. The remaining options belong to different chronology, actor or analytical categories.

Q14. Which chronology card should be filed under The National Health Mission and its two sub-missions?

A. The National Health Authority is the autonomous body that implements the hospitalisation pillar, maintaining the beneficiary database, empanelling hospitals and processing claims, while the National Health Accounts estimates are the official source for the breakdown of health expenditure between public, private and out-of-pocket spending and are prepared through the national health accounts system; the owners record expressly that these two must not be confused, because one is an implementing agency and the other is an accounting estimate, and citing an expenditure share to the implementing agency is a sourcing error. B. The National Health Mission's own official page, read on 2 September 2026, records that the National Rural Health Mission was launched by the Prime Minister on 12 April 2005 to provide accessible, affordable and quality health care to the rural population and especially to vulnerable groups, and that the Union Cabinet, by its decision dated 1 May 2013, approved the launch of the National Urban Health Mission as a sub-mission of an over-arching National Health Mission with the National Rural Health Mission being the other sub-mission; the same page records special focus on the Empowered Action Group States, the North Eastern States, Jammu and Kashmir and Himachal Pradesh, a thrust on a fully functional, community-owned, decentralised health delivery system with inter-sectoral convergence acting on determinants such as water, sanitation, education, nutrition and social and gender equality, and measurement of facilities against the Indian Public Health Standards, so the mission is urban as well as rural and treating it as rural-only is a factual error. C. The second pillar, Pradhan Mantri Jan Arogya Yojana, provides cashless secondary and tertiary hospitalisation cover of up to five lakh rupees per family per year at empanelled public and private hospitals for its core eligible households, which are identified through the deprivation criteria of the socio-economic and caste census, and the owners separately record a distinct expansion under which all citizens aged seventy years and above can enrol regardless of income or that census status; the discipline required is exact, because the core scheme is eligibility-based while the age-based expansion is universal only for the seventy-plus group, so an age-specific expansion must never be written up as universal coverage for every age. D. Out-of-pocket expenditure is health spending paid directly by households at the point of service, and the owners record the National Health Accounts sequence as 47.1 per cent of total health expenditure in 2019-20, 44.4 per cent in 2020-21 and 39.4 per cent in 2021-22, with the latest cited official estimate being 39.4 per cent for the accounting year 2021-22; two disciplines follow, first that the publication date must never be confused with the accounting year, and second that a falling national share is a dated aggregate rather than evidence that every household faces a lighter burden, and high out-of-pocket expenditure indicates inadequate public provision rather than high utilisation.

Answer: B. Explanation: The National Health Mission's own official page, read on 2 September 2026, records that the National Rural Health Mission was launched by the Prime Minister on 12 April 2005 to provide accessible, affordable and quality health care to the rural population and especially to vulnerable groups, and that the Union Cabinet, by its decision dated 1 May 2013, approved the launch of the National Urban Health Mission as a sub-mission of an over-arching National Health Mission with the National Rural Health Mission being the other sub-mission; the same page records special focus on the Empowered Action Group States, the North Eastern States, Jammu and Kashmir and Himachal Pradesh, a thrust on a fully functional, community-owned, decentralised health delivery system with inter-sectoral convergence acting on determinants such as water, sanitation, education, nutrition and social and gender equality, and measurement of facilities against the Indian Public Health Standards, so the mission is urban as well as rural and treating it as rural-only is a factual error. The remaining options belong to different chronology, actor or analytical categories.

Q15. Which option preserves the source-bounded meaning of The National Health Mission and its two sub-missions?

A. Catastrophic health expenditure is health spending that exceeds a threshold share of household income or consumption, commonly stated in the range of ten to twenty-five per cent, and it pushes households into poverty or debt, with high out-of-pocket expenditure as its primary driver; this is the mechanism that makes health a social-justice question rather than only a service-delivery question, because a single hospitalisation can undo years of asset accumulation, and it is the bridge that connects this topic to the poverty and nutrition owner. B. Out-of-pocket expenditure is health spending paid directly by households at the point of service, and the owners record the National Health Accounts sequence as 47.1 per cent of total health expenditure in 2019-20, 44.4 per cent in 2020-21 and 39.4 per cent in 2021-22, with the latest cited official estimate being 39.4 per cent for the accounting year 2021-22; two disciplines follow, first that the publication date must never be confused with the accounting year, and second that a falling national share is a dated aggregate rather than evidence that every household faces a lighter burden, and high out-of-pocket expenditure indicates inadequate public provision rather than high utilisation. C. The National Health Mission's own official page, read on 2 September 2026, records that the National Rural Health Mission was launched by the Prime Minister on 12 April 2005 to provide accessible, affordable and quality health care to the rural population and especially to vulnerable groups, and that the Union Cabinet, by its decision dated 1 May 2013, approved the launch of the National Urban Health Mission as a sub-mission of an over-arching National Health Mission with the National Rural Health Mission being the other sub-mission; the same page records special focus on the Empowered Action Group States, the North Eastern States, Jammu and Kashmir and Himachal Pradesh, a thrust on a fully functional, community-owned, decentralised health delivery system with inter-sectoral convergence acting on determinants such as water, sanitation, education, nutrition and social and gender equality, and measurement of facilities against the Indian Public Health Standards, so the mission is urban as well as rural and treating it as rural-only is a factual error. D. The National Health Authority is the autonomous body that implements the hospitalisation pillar, maintaining the beneficiary database, empanelling hospitals and processing claims, while the National Health Accounts estimates are the official source for the breakdown of health expenditure between public, private and out-of-pocket spending and are prepared through the national health accounts system; the owners record expressly that these two must not be confused, because one is an implementing agency and the other is an accounting estimate, and citing an expenditure share to the implementing agency is a sourcing error.

Answer: C. Explanation: The National Health Mission's own official page, read on 2 September 2026, records that the National Rural Health Mission was launched by the Prime Minister on 12 April 2005 to provide accessible, affordable and quality health care to the rural population and especially to vulnerable groups, and that the Union Cabinet, by its decision dated 1 May 2013, approved the launch of the National Urban Health Mission as a sub-mission of an over-arching National Health Mission with the National Rural Health Mission being the other sub-mission; the same page records special focus on the Empowered Action Group States, the North Eastern States, Jammu and Kashmir and Himachal Pradesh, a thrust on a fully functional, community-owned, decentralised health delivery system with inter-sectoral convergence acting on determinants such as water, sanitation, education, nutrition and social and gender equality, and measurement of facilities against the Indian Public Health Standards, so the mission is urban as well as rural and treating it as rural-only is a factual error. The remaining options belong to different chronology, actor or analytical categories.

Q16. Which statement avoids a close-option trap about The National Health Mission and its two sub-missions?

A. Catastrophic health expenditure is health spending that exceeds a threshold share of household income or consumption, commonly stated in the range of ten to twenty-five per cent, and it pushes households into poverty or debt, with high out-of-pocket expenditure as its primary driver; this is the mechanism that makes health a social-justice question rather than only a service-delivery question, because a single hospitalisation can undo years of asset accumulation, and it is the bridge that connects this topic to the poverty and nutrition owner. B. The public health system is tiered from the sub-centre through the primary health centre and the community health centre to the sub-district and district hospital, and this hierarchy is the referral spine on which the first pillar's upgrade and the second pillar's empanelment both sit; naming the tier at which a failure occurs is what converts a general complaint about public hospitals into a diagnosis, because a vacancy at the sub-centre, a diagnostic gap at the primary health centre and a specialist shortage at the community health centre call for three different corrections. C. Out-of-pocket expenditure is health spending paid directly by households at the point of service, and the owners record the National Health Accounts sequence as 47.1 per cent of total health expenditure in 2019-20, 44.4 per cent in 2020-21 and 39.4 per cent in 2021-22, with the latest cited official estimate being 39.4 per cent for the accounting year 2021-22; two disciplines follow, first that the publication date must never be confused with the accounting year, and second that a falling national share is a dated aggregate rather than evidence that every household faces a lighter burden, and high out-of-pocket expenditure indicates inadequate public provision rather than high utilisation. D. The National Health Mission's own official page, read on 2 September 2026, records that the National Rural Health Mission was launched by the Prime Minister on 12 April 2005 to provide accessible, affordable and quality health care to the rural population and especially to vulnerable groups, and that the Union Cabinet, by its decision dated 1 May 2013, approved the launch of the National Urban Health Mission as a sub-mission of an over-arching National Health Mission with the National Rural Health Mission being the other sub-mission; the same page records special focus on the Empowered Action Group States, the North Eastern States, Jammu and Kashmir and Himachal Pradesh, a thrust on a fully functional, community-owned, decentralised health delivery system with inter-sectoral convergence acting on determinants such as water, sanitation, education, nutrition and social and gender equality, and measurement of facilities against the Indian Public Health Standards, so the mission is urban as well as rural and treating it as rural-only is a factual error.

Answer: D. Explanation: The National Health Mission's own official page, read on 2 September 2026, records that the National Rural Health Mission was launched by the Prime Minister on 12 April 2005 to provide accessible, affordable and quality health care to the rural population and especially to vulnerable groups, and that the Union Cabinet, by its decision dated 1 May 2013, approved the launch of the National Urban Health Mission as a sub-mission of an over-arching National Health Mission with the National Rural Health Mission being the other sub-mission; the same page records special focus on the Empowered Action Group States, the North Eastern States, Jammu and Kashmir and Himachal Pradesh, a thrust on a fully functional, community-owned, decentralised health delivery system with inter-sectoral convergence acting on determinants such as water, sanitation, education, nutrition and social and gender equality, and measurement of facilities against the Indian Public Health Standards, so the mission is urban as well as rural and treating it as rural-only is a factual error. The remaining options belong to different chronology, actor or analytical categories.

Q17. Which statement correctly identifies The primary-care pillar of Ayushman Bharat?

A. The first pillar of Ayushman Bharat upgrades sub-centres and primary health centres into Health and Wellness Centres, now carried as Ayushman Arogya Mandirs, which deliver comprehensive primary health care through twelve service packages that extend beyond reproductive and child health to non-communicable diseases, mental health, ear, nose and throat care, ophthalmology, oral health, geriatric and palliative care and emergency care, staffed by a community health officer or mid-level health provider; the design intent is to shift care-seeking from hospitals to a local first-contact layer, which is why describing this pillar as reproductive and child health alone understates it. B. The National Health Authority is the autonomous body that implements the hospitalisation pillar, maintaining the beneficiary database, empanelling hospitals and processing claims, while the National Health Accounts estimates are the official source for the breakdown of health expenditure between public, private and out-of-pocket spending and are prepared through the national health accounts system; the owners record expressly that these two must not be confused, because one is an implementing agency and the other is an accounting estimate, and citing an expenditure

share to the implementing agency is a sourcing error. C. The second pillar, Pradhan Mantri Jan Arogya Yojana, provides cashless secondary and tertiary hospitalisation cover of up to five lakh rupees per family per year at empanelled public and private hospitals for its core eligible households, which are identified through the deprivation criteria of the socio-economic and caste census, and the owners separately record a distinct expansion under which all citizens aged seventy years and above can enrol regardless of income or that census status; the discipline required is exact, because the core scheme is eligibility-based while the age-based expansion is universal only for the seventy-plus group, so an age-specific expansion must never be written up as universal coverage for every age. D. Out-of-pocket expenditure is health spending paid directly by households at the point of service, and the owners record the National Health Accounts sequence as 47.1 per cent of total health expenditure in 2019-20, 44.4 per cent in 2020-21 and 39.4 per cent in 2021-22, with the latest cited official estimate being 39.4 per cent for the accounting year 2021-22; two disciplines follow, first that the publication date must never be confused with the accounting year, and second that a falling national share is a dated aggregate rather than evidence that every household faces a lighter burden, and high out-of-pocket expenditure indicates inadequate public provision rather than high utilisation.

Answer: A. Explanation: The first pillar of Ayushman Bharat upgrades sub-centres and primary health centres into Health and Wellness Centres, now carried as Ayushman Arogya Mandirs, which deliver comprehensive primary health care through twelve service packages that extend beyond reproductive and child health to non-communicable diseases, mental health, ear, nose and throat care, ophthalmology, oral health, geriatric and palliative care and emergency care, staffed by a community health officer or mid-level health provider; the design intent is to shift care-seeking from hospitals to a local first-contact layer, which is why describing this pillar as reproductive and child health alone understates it. The remaining options belong to different chronology, actor or analytical categories.

Q18. Which chronology card should be filed under The primary-care pillar of Ayushman Bharat?

A. The National Health Authority is the autonomous body that implements the hospitalisation pillar, maintaining the beneficiary database, empanelling hospitals and processing claims, while the National Health Accounts estimates are the official source for the breakdown of health expenditure between public, private and out-of-pocket spending and are prepared through the national health accounts system; the owners record expressly that these two must not be confused, because one is an implementing agency and the other is an accounting estimate, and citing an expenditure share to the implementing agency is a sourcing error. B. The first pillar of Ayushman Bharat upgrades sub-centres and primary health centres into Health and Wellness Centres, now carried as Ayushman Arogya Mandirs, which deliver comprehensive primary health care through twelve service packages that extend beyond reproductive and child health to non-communicable diseases, mental health, ear, nose and throat care, ophthalmology, oral health, geriatric and palliative care and emergency care, staffed by a community health officer or mid-level health provider; the design intent is to shift care-seeking from hospitals to a local first-contact layer, which is why describing this pillar as reproductive and child health alone understates it. C. Catastrophic health expenditure is health spending that exceeds a threshold share of household income or consumption, commonly stated in the range of ten to twenty-five per cent, and it pushes households into poverty or debt, with high out-of-pocket expenditure as its primary driver; this is the mechanism that makes health a social-justice question rather than only a service-delivery question, because a single hospitalisation can undo years of asset accumulation, and it is the bridge that connects this topic to the poverty and nutrition owner. D. Out-of-pocket expenditure is health spending paid directly by households at the point of service, and the owners record the National Health Accounts sequence as 47.1 per cent of total health expenditure in 2019-20, 44.4 per cent in 2020-21 and 39.4 per cent in 2021-22, with the latest cited official estimate being 39.4 per cent for the accounting year 2021-22; two disciplines follow, first that the publication date must never be confused with the accounting year, and second that a falling national share is a dated aggregate rather than evidence that every household faces a lighter burden, and high out-of-pocket expenditure indicates inadequate public provision rather than high utilisation.

Answer: B. Explanation: The first pillar of Ayushman Bharat upgrades sub-centres and primary health centres into Health and Wellness Centres, now carried as Ayushman Arogya Mandirs, which deliver comprehensive primary health care through twelve service packages that extend beyond reproductive and child health to non-communicable diseases, mental health, ear, nose and throat care, ophthalmology, oral health, geriatric and palliative care and emergency care, staffed by a community health officer or mid-level health provider; the design intent is to shift care-seeking from hospitals to a local first-contact layer, which is why describing this pillar as reproductive and child health alone understates it. The remaining options belong to different

Q19. Which option preserves the source-bounded meaning of The primary-care pillar of Ayushman Bharat?

A. The public health system is tiered from the sub-centre through the primary health centre and the community health centre to the sub-district and district hospital, and this hierarchy is the referral spine on which the first pillar's upgrade and the second pillar's empanelment both sit; naming the tier at which a failure occurs is what converts a general complaint about public hospitals into a diagnosis, because a vacancy at the sub-centre, a diagnostic gap at the primary health centre and a specialist shortage at the community health centre call for three different corrections. B. Out-of-pocket expenditure is health spending paid directly by households at the point of service, and the owners record the National Health Accounts sequence as 47.1 per cent of total health expenditure in 2019-20, 44.4 per cent in 2020-21 and 39.4 per cent in 2021-22, with the latest cited official estimate being 39.4 per cent for the accounting year 2021-22; two disciplines follow, first that the publication date must never be confused with the accounting year, and second that a falling national share is a dated aggregate rather than evidence that every household faces a lighter burden, and high out-of-pocket expenditure indicates inadequate public provision rather than high utilisation. C. The first pillar of Ayushman Bharat upgrades sub-centres and primary health centres into Health and Wellness Centres, now carried as Ayushman Arogya Mandirs, which deliver comprehensive primary health care through twelve service packages that extend beyond reproductive and child health to non-communicable diseases, mental health, ear, nose and throat care, ophthalmology, oral health, geriatric and palliative care and emergency care, staffed by a community health officer or mid-level health provider; the design intent is to shift care-seeking from hospitals to a local first-contact layer, which is why describing this pillar as reproductive and child health alone understates it. D. Catastrophic health expenditure is health spending that exceeds a threshold share of household income or consumption, commonly stated in the range of ten to twenty-five per cent, and it pushes households into poverty or debt, with high out-of-pocket expenditure as its primary driver; this is the mechanism that makes health a social-justice question rather than only a service-delivery question, because a single hospitalisation can undo years of asset accumulation, and it is the bridge that connects this topic to the poverty and nutrition owner.

Answer: C. Explanation: The first pillar of Ayushman Bharat upgrades sub-centres and primary health centres into Health and Wellness Centres, now carried as Ayushman Arogya Mandirs, which deliver comprehensive primary health care through twelve service packages that extend beyond reproductive and child health to non-communicable diseases, mental health, ear, nose and throat care, ophthalmology, oral health, geriatric and palliative care and emergency care, staffed by a community health officer or mid-level health provider; the design intent is to shift care-seeking from hospitals to a local first-contact layer, which is why describing this pillar as reproductive and child health alone understates it. The remaining options belong to different chronology, actor or analytical categories.

Q20. Which statement avoids a close-option trap about The primary-care pillar of Ayushman Bharat?

A. Primary care is first-contact preventive, promotive and basic curative care delivered at the wellness centre and the primary health centre, secondary care is specialist outpatient and inpatient care at the community health centre and the district hospital, and tertiary care is super-specialty and advanced procedural care at medical colleges and referral centres; the flagship programme is deliberately split along this line, with the first pillar addressing primary care and the second addressing secondary and tertiary care, so confusing the pillars is simultaneously a factual error and a conceptual one. B. The public health system is tiered from the sub-centre through the primary health centre and the community health centre to the sub-district and district hospital, and this hierarchy is the referral spine on which the first pillar's upgrade and the second pillar's empanelment both sit; naming the tier at which a failure occurs is what converts a general complaint about public hospitals into a diagnosis, because a vacancy at the sub-centre, a diagnostic gap at the primary health centre and a specialist shortage at the community health centre call for three different corrections. C. Catastrophic health expenditure is health spending that exceeds a threshold share of household income or consumption, commonly stated in the range of ten to twenty-five per cent, and it pushes households into poverty or debt, with high out-of-pocket expenditure as its primary driver; this is the mechanism that makes health a social-justice question rather than only a service-delivery question, because a single hospitalisation can undo years of asset accumulation, and it is the bridge that connects this topic to the poverty and nutrition owner. D. The first pillar of Ayushman Bharat upgrades sub-centres and primary health centres into Health and Wellness

Centres, now carried as Ayushman Arogya Mandirs, which deliver comprehensive primary health care through twelve service packages that extend beyond reproductive and child health to non-communicable diseases, mental health, ear, nose and throat care, ophthalmology, oral health, geriatric and palliative care and emergency care, staffed by a community health officer or mid-level health provider; the design intent is to shift care-seeking from hospitals to a local first-contact layer, which is why describing this pillar as reproductive and child health alone understates it.

Answer: D. Explanation: The first pillar of Ayushman Bharat upgrades sub-centres and primary health centres into Health and Wellness Centres, now carried as Ayushman Arogya Mandirs, which deliver comprehensive primary health care through twelve service packages that extend beyond reproductive and child health to non-communicable diseases, mental health, ear, nose and throat care, ophthalmology, oral health, geriatric and palliative care and emergency care, staffed by a community health officer or mid-level health provider; the design intent is to shift care-seeking from hospitals to a local first-contact layer, which is why describing this pillar as reproductive and child health alone understates it. The remaining options belong to different chronology, actor or analytical categories.

Q21. Which statement correctly identifies The hospitalisation pillar of Ayushman Bharat and its two enrolment routes?

A. The second pillar, Pradhan Mantri Jan Arogya Yojana, provides cashless secondary and tertiary hospitalisation cover of up to five lakh rupees per family per year at empanelled public and private hospitals for its core eligible households, which are identified through the deprivation criteria of the socio-economic and caste census, and the owners separately record a distinct expansion under which all citizens aged seventy years and above can enrol regardless of income or that census status; the discipline required is exact, because the core scheme is eligibility-based while the age-based expansion is universal only for the seventy-plus group, so an age-specific expansion must never be written up as universal coverage for every age. B. The National Health Authority is the autonomous body that implements the hospitalisation pillar, maintaining the beneficiary database, empanelling hospitals and processing claims, while the National Health Accounts estimates are the official source for the breakdown of health expenditure between public, private and out-of-pocket spending and are prepared through the national health accounts system; the owners record expressly that these two must not be confused, because one is an implementing agency and the other is an accounting estimate, and citing an expenditure share to the implementing agency is a sourcing error. C. Catastrophic health expenditure is health spending that exceeds a threshold share of household income or consumption, commonly stated in the range of ten to twenty-five per cent, and it pushes households into poverty or debt, with high out-of-pocket expenditure as its primary driver; this is the mechanism that makes health a social-justice question rather than only a service-delivery question, because a single hospitalisation can undo years of asset accumulation, and it is the bridge that connects this topic to the poverty and nutrition owner. D. Out-of-pocket expenditure is health spending paid directly by households at the point of service, and the owners record the National Health Accounts sequence as 47.1 per cent of total health expenditure in 2019-20, 44.4 per cent in 2020-21 and 39.4 per cent in 2021-22, with the latest cited official estimate being 39.4 per cent for the accounting year 2021-22; two disciplines follow, first that the publication date must never be confused with the accounting year, and second that a falling national share is a dated aggregate rather than evidence that every household faces a lighter burden, and high out-of-pocket expenditure indicates inadequate public provision rather than high utilisation.

Answer: A. Explanation: The second pillar, Pradhan Mantri Jan Arogya Yojana, provides cashless secondary and tertiary hospitalisation cover of up to five lakh rupees per family per year at empanelled public and private hospitals for its core eligible households, which are identified through the deprivation criteria of the socio-economic and caste census, and the owners separately record a distinct expansion under which all citizens aged seventy years and above can enrol regardless of income or that census status; the discipline required is exact, because the core scheme is eligibility-based while the age-based expansion is universal only for the seventy-plus group, so an age-specific expansion must never be written up as universal coverage for every age. The remaining options belong to different chronology, actor or analytical categories.

Q22. Which chronology card should be filed under The hospitalisation pillar of Ayushman Bharat and its two enrolment routes?

A. The public health system is tiered from the sub-centre through the primary health centre and the community health centre to the sub-district and district hospital, and this hierarchy is the referral spine on which the first pillar's upgrade and the second pillar's empanelment both sit; naming the tier at which a failure occurs is what converts a general complaint about public hospitals into a diagnosis, because a vacancy at the sub-centre, a diagnostic gap at the primary health centre and a specialist shortage at the community health centre call for three different corrections. B. The second pillar, Pradhan Mantri Jan Arogya Yojana, provides cashless secondary and tertiary hospitalisation cover of up to five lakh rupees per family per year at empanelled public and private hospitals for its core eligible households, which are identified through the deprivation criteria of the socio-economic and caste census, and the owners separately record a distinct expansion under which all citizens aged seventy years and above can enrol regardless of income or that census status; the discipline required is exact, because the core scheme is eligibility-based while the age-based expansion is universal only for the seventy-plus group, so an age-specific expansion must never be written up as universal coverage for every age. C. Out-of-pocket expenditure is health spending paid directly by households at the point of service, and the owners record the National Health Accounts sequence as 47.1 per cent of total health expenditure in 2019-20, 44.4 per cent in 2020-21 and 39.4 per cent in 2021-22, with the latest cited official estimate being 39.4 per cent for the accounting year 2021-22; two disciplines follow, first that the publication date must never be confused with the accounting year, and second that a falling national share is a dated aggregate rather than evidence that every household faces a lighter burden, and high out-of-pocket expenditure indicates inadequate public provision rather than high utilisation. D. Catastrophic health expenditure is health spending that exceeds a threshold share of household income or consumption, commonly stated in the range of ten to twenty-five per cent, and it pushes households into poverty or debt, with high out-of-pocket expenditure as its primary driver; this is the mechanism that makes health a social-justice question rather than only a service-delivery question, because a single hospitalisation can undo years of asset accumulation, and it is the bridge that connects this topic to the poverty and nutrition owner.

Answer: B. Explanation: The second pillar, Pradhan Mantri Jan Arogya Yojana, provides cashless secondary and tertiary hospitalisation cover of up to five lakh rupees per family per year at empanelled public and private hospitals for its core eligible households, which are identified through the deprivation criteria of the socio-economic and caste census, and the owners separately record a distinct expansion under which all citizens aged seventy years and above can enrol regardless of income or that census status; the discipline required is exact, because the core scheme is eligibility-based while the age-based expansion is universal only for the seventy-plus group, so an age-specific expansion must never be written up as universal coverage for every age. The remaining options belong to different chronology, actor or analytical categories.

Q23. Which option preserves the source-bounded meaning of The hospitalisation pillar of Ayushman Bharat and its two enrolment routes?

A. Primary care is first-contact preventive, promotive and basic curative care delivered at the wellness centre and the primary health centre, secondary care is specialist outpatient and inpatient care at the community health centre and the district hospital, and tertiary care is super-specialty and advanced procedural care at medical colleges and referral centres; the flagship programme is deliberately split along this line, with the first pillar addressing primary care and the second addressing secondary and tertiary care, so confusing the pillars is simultaneously a factual error and a conceptual one. B. The public health system is tiered from the sub-centre through the primary health centre and the community health centre to the sub-district and district hospital, and this hierarchy is the referral spine on which the first pillar's upgrade and the second pillar's empanelment both sit; naming the tier at which a failure occurs is what converts a general complaint about public hospitals into a diagnosis, because a vacancy at the sub-centre, a diagnostic gap at the primary health centre and a specialist shortage at the community health centre call for three different corrections. C. The second pillar, Pradhan Mantri Jan Arogya Yojana, provides cashless secondary and tertiary hospitalisation cover of up to five lakh rupees per family per year at empanelled public and private hospitals for its core eligible households, which are identified through the deprivation criteria of the socio-economic and caste census, and the owners separately record a distinct expansion under which all citizens aged seventy years and above can enrol regardless of income or that census status; the discipline required is exact, because the core scheme is eligibility-based while the age-based expansion is universal only for the seventy-plus group, so an age-specific expansion must never be written up as universal coverage for every age. D. Catastrophic health expenditure is health spending that exceeds a threshold share of household income or consumption, commonly stated in the range of ten to

twenty-five per cent, and it pushes households into poverty or debt, with high out-of-pocket expenditure as its primary driver; this is the mechanism that makes health a social-justice question rather than only a service-delivery question, because a single hospitalisation can undo years of asset accumulation, and it is the bridge that connects this topic to the poverty and nutrition owner.

Answer: C. Explanation: The second pillar, Pradhan Mantri Jan Arogya Yojana, provides cashless secondary and tertiary hospitalisation cover of up to five lakh rupees per family per year at empanelled public and private hospitals for its core eligible households, which are identified through the deprivation criteria of the socio-economic and caste census, and the owners separately record a distinct expansion under which all citizens aged seventy years and above can enrol regardless of income or that census status; the discipline required is exact, because the core scheme is eligibility-based while the age-based expansion is universal only for the seventy-plus group, so an age-specific expansion must never be written up as universal coverage for every age. The remaining options belong to different chronology, actor or analytical categories.

Q24. Which statement avoids a close-option trap about The hospitalisation pillar of Ayushman Bharat and its two enrolment routes?

A. Marketisation refers to the dominance of private providers in curative care, often unregulated or under-regulated, and the owners record its consequences as high costs, unnecessary procedures, catastrophic spending and inequitable access; this is the exact concern raised by the 2024 demand, so an answer to it must define the phenomenon and its harms before proposing measures, and must avoid the two symmetrical errors of denying the private sector's real capacity contribution and of treating regulation as automatic once a statute exists. B. The public health system is tiered from the sub-centre through the primary health centre and the community health centre to the sub-district and district hospital, and this hierarchy is the referral spine on which the first pillar's upgrade and the second pillar's empanelment both sit; naming the tier at which a failure occurs is what converts a general complaint about public hospitals into a diagnosis, because a vacancy at the sub-centre, a diagnostic gap at the primary health centre and a specialist shortage at the community health centre call for three different corrections. C. Primary care is first-contact preventive, promotive and basic curative care delivered at the wellness centre and the primary health centre, secondary care is specialist outpatient and inpatient care at the community health centre and the district hospital, and tertiary care is super-specialty and advanced procedural care at medical colleges and referral centres; the flagship programme is deliberately split along this line, with the first pillar addressing primary care and the second addressing secondary and tertiary care, so confusing the pillars is simultaneously a factual error and a conceptual one. D. The second pillar, Pradhan Mantri Jan Arogya Yojana, provides cashless secondary and tertiary hospitalisation cover of up to five lakh rupees per family per year at empanelled public and private hospitals for its core eligible households, which are identified through the deprivation criteria of the socio-economic and caste census, and the owners separately record a distinct expansion under which all citizens aged seventy years and above can enrol regardless of income or that census status; the discipline required is exact, because the core scheme is eligibility-based while the age-based expansion is universal only for the seventy-plus group, so an age-specific expansion must never be written up as universal coverage for every age.

Answer: D. Explanation: The second pillar, Pradhan Mantri Jan Arogya Yojana, provides cashless secondary and tertiary hospitalisation cover of up to five lakh rupees per family per year at empanelled public and private hospitals for its core eligible households, which are identified through the deprivation criteria of the socio-economic and caste census, and the owners separately record a distinct expansion under which all citizens aged seventy years and above can enrol regardless of income or that census status; the discipline required is exact, because the core scheme is eligibility-based while the age-based expansion is universal only for the seventy-plus group, so an age-specific expansion must never be written up as universal coverage for every age. The remaining options belong to different chronology, actor or analytical categories.

Q25. Which statement correctly identifies The health authority and the health accounts must not be conflated?

A. The National Health Authority is the autonomous body that implements the hospitalisation pillar, maintaining the beneficiary database, empanelling hospitals and processing claims, while the National Health Accounts estimates are the official source for the breakdown of health expenditure between public, private and out-of-pocket spending and are prepared through the national health accounts system; the owners record expressly that these two must not be confused, because one is an implementing agency and the other is an accounting estimate, and citing an expenditure share to the implementing agency is a sourcing error. B. Catastrophic health expenditure is health spending that exceeds a threshold share of household income or consumption, commonly stated in the range of ten to twenty-five per cent, and it pushes households into poverty or debt, with high out-of-pocket expenditure as its primary driver; this is the mechanism that makes health a social-justice question rather than only a service-delivery question, because a single hospitalisation can undo years of asset accumulation, and it is the bridge that connects this topic to the poverty and nutrition owner. C. Out-of-pocket expenditure is health spending paid directly by households at the point of service, and the owners record the National Health Accounts sequence as 47.1 per cent of total health expenditure in 2019-20, 44.4 per cent in 2020-21 and 39.4 per cent in 2021-22, with the latest cited official estimate being 39.4 per cent for the accounting year 2021-22; two disciplines follow, first that the publication date must never be confused with the accounting year, and second that a falling national share is a dated aggregate rather than evidence that every household faces a lighter burden, and high out-of-pocket expenditure indicates inadequate public provision rather than high utilisation. D. The public health system is tiered from the sub-centre through the primary health centre and the community health centre to the sub-district and district hospital, and this hierarchy is the referral spine on which the first pillar's upgrade and the second pillar's empanelment both sit; naming the tier at which a failure occurs is what converts a general complaint about public hospitals into a diagnosis, because a vacancy at the sub-centre, a diagnostic gap at the primary health centre and a specialist shortage at the community health centre call for three different corrections.

Answer: A. Explanation: The National Health Authority is the autonomous body that implements the hospitalisation pillar, maintaining the beneficiary database, empanelling hospitals and processing claims, while the National Health Accounts estimates are the official source for the breakdown of health expenditure between public, private and out-of-pocket spending and are prepared through the national health accounts system; the owners record expressly that these two must not be confused, because one is an implementing agency and the other is an accounting estimate, and citing an expenditure share to the implementing agency is a sourcing error. The remaining options belong to different chronology, actor or analytical categories.

Q26. Which chronology card should be filed under The health authority and the health accounts must not be conflated?

A. Catastrophic health expenditure is health spending that exceeds a threshold share of household income or consumption, commonly stated in the range of ten to twenty-five per cent, and it pushes households into poverty or debt, with high out-of-pocket expenditure as its primary driver; this is the mechanism that makes health a social-justice question rather than only a service-delivery question, because a single hospitalisation can undo years of asset accumulation, and it is the bridge that connects this topic to the poverty and nutrition owner. B. The National Health Authority is the autonomous body that implements the hospitalisation pillar, maintaining the beneficiary database, empanelling hospitals and processing claims, while the National Health Accounts estimates are the official source for the breakdown of health expenditure between public, private and out-of-pocket spending and are prepared through the national health accounts system; the owners record expressly that these two must not be confused, because one is an implementing agency and the other is an accounting estimate, and citing an expenditure share to the implementing agency is a sourcing error. C. The public health system is tiered from the sub-centre through the primary health centre and the community health centre to the sub-district and district hospital, and this hierarchy is the referral spine on which the first pillar's upgrade and the second pillar's empanelment both sit; naming the tier at which a failure occurs is what converts a general complaint about public hospitals into a diagnosis, because a vacancy at the sub-centre, a diagnostic gap at the primary health centre and a specialist shortage at the community health centre call for three different corrections. D. Primary care is first-contact preventive, promotive and basic curative care delivered at the wellness centre and the primary health centre, secondary care is specialist outpatient and inpatient care at the community health centre and the district hospital, and tertiary care is super-specialty and advanced procedural care at medical colleges and referral centres; the flagship programme is deliberately split along this line, with the first pillar

addressing primary care and the second addressing secondary and tertiary care, so confusing the pillars is simultaneously a factual error and a conceptual one.

Answer: B. Explanation: The National Health Authority is the autonomous body that implements the hospitalisation pillar, maintaining the beneficiary database, empanelling hospitals and processing claims, while the National Health Accounts estimates are the official source for the breakdown of health expenditure between public, private and out-of-pocket spending and are prepared through the national health accounts system; the owners record expressly that these two must not be confused, because one is an implementing agency and the other is an accounting estimate, and citing an expenditure share to the implementing agency is a sourcing error. The remaining options belong to different chronology, actor or analytical categories.

Q27. Which option preserves the source-bounded meaning of The health authority and the health accounts must not be conflated?

A. Marketisation refers to the dominance of private providers in curative care, often unregulated or under-regulated, and the owners record its consequences as high costs, unnecessary procedures, catastrophic spending and inequitable access; this is the exact concern raised by the 2024 demand, so an answer to it must define the phenomenon and its harms before proposing measures, and must avoid the two symmetrical errors of denying the private sector's real capacity contribution and of treating regulation as automatic once a statute exists. B. The public health system is tiered from the sub-centre through the primary health centre and the community health centre to the sub-district and district hospital, and this hierarchy is the referral spine on which the first pillar's upgrade and the second pillar's empanelment both sit; naming the tier at which a failure occurs is what converts a general complaint about public hospitals into a diagnosis, because a vacancy at the sub-centre, a diagnostic gap at the primary health centre and a specialist shortage at the community health centre call for three different corrections. C. The National Health Authority is the autonomous body that implements the hospitalisation pillar, maintaining the beneficiary database, empanelling hospitals and processing claims, while the National Health Accounts estimates are the official source for the breakdown of health expenditure between public, private and out-of-pocket spending and are prepared through the national health accounts system; the owners record expressly that these two must not be confused, because one is an implementing agency and the other is an accounting estimate, and citing an expenditure share to the implementing agency is a sourcing error. D. Primary care is first-contact preventive, promotive and basic curative care delivered at the wellness centre and the primary health centre, secondary care is specialist outpatient and inpatient care at the community health centre and the district hospital, and tertiary care is super-specialty and advanced procedural care at medical colleges and referral centres; the flagship programme is deliberately split along this line, with the first pillar addressing primary care and the second addressing secondary and tertiary care, so confusing the pillars is simultaneously a factual error and a conceptual one.

Answer: C. Explanation: The National Health Authority is the autonomous body that implements the hospitalisation pillar, maintaining the beneficiary database, empanelling hospitals and processing claims, while the National Health Accounts estimates are the official source for the breakdown of health expenditure between public, private and out-of-pocket spending and are prepared through the national health accounts system; the owners record expressly that these two must not be confused, because one is an implementing agency and the other is an accounting estimate, and citing an expenditure share to the implementing agency is a sourcing error. The remaining options belong to different chronology, actor or analytical categories.

Q28. Which statement avoids a close-option trap about The health authority and the health accounts must not be conflated?

A. Marketisation refers to the dominance of private providers in curative care, often unregulated or under-regulated, and the owners record its consequences as high costs, unnecessary procedures, catastrophic spending and inequitable access; this is the exact concern raised by the 2024 demand, so an answer to it must define the phenomenon and its harms before proposing measures, and must avoid the two symmetrical errors of denying the private sector's real capacity contribution and of treating regulation as automatic once a statute exists. B. The regulatory instruments recorded by the owners are the Clinical Establishments Act, 2010, a Central Act providing for the registration and regulation of clinical establishments which States must adopt or replace with equivalent legislation, price capping of the kind exercised by the national pharmaceutical pricing authority over devices, and standard treatment guidelines that promote rational care and cost control; each carries a stated enforcement condition, because State adoption of the Central Act varies and some States have not adopted or notified equivalent

rules, and adoption of treatment guidelines varies across public and empanelled private providers, so a recommendation to regulate must name the adoption gap it is closing. C. Primary care is first-contact preventive, promotive and basic curative care delivered at the wellness centre and the primary health centre, secondary care is specialist outpatient and inpatient care at the community health centre and the district hospital, and tertiary care is super-specialty and advanced procedural care at medical colleges and referral centres; the flagship programme is deliberately split along this line, with the first pillar addressing primary care and the second addressing secondary and tertiary care, so confusing the pillars is simultaneously a factual error and a conceptual one. D. The National Health Authority is the autonomous body that implements the hospitalisation pillar, maintaining the beneficiary database, empanelling hospitals and processing claims, while the National Health Accounts estimates are the official source for the breakdown of health expenditure between public, private and out-of-pocket spending and are prepared through the national health accounts system; the owners record expressly that these two must not be confused, because one is an implementing agency and the other is an accounting estimate, and citing an expenditure share to the implementing agency is a sourcing error.

Answer: D. Explanation: The National Health Authority is the autonomous body that implements the hospitalisation pillar, maintaining the beneficiary database, empanelling hospitals and processing claims, while the National Health Accounts estimates are the official source for the breakdown of health expenditure between public, private and out-of-pocket spending and are prepared through the national health accounts system; the owners record expressly that these two must not be confused, because one is an implementing agency and the other is an accounting estimate, and citing an expenditure share to the implementing agency is a sourcing error. The remaining options belong to different chronology, actor or analytical categories.

Q29. Which statement correctly identifies Out-of-pocket expenditure and its dated official sequence?

A. Out-of-pocket expenditure is health spending paid directly by households at the point of service, and the owners record the National Health Accounts sequence as 47.1 per cent of total health expenditure in 2019-20, 44.4 per cent in 2020-21 and 39.4 per cent in 2021-22, with the latest cited official estimate being 39.4 per cent for the accounting year 2021-22; two disciplines follow, first that the publication date must never be confused with the accounting year, and second that a falling national share is a dated aggregate rather than evidence that every household faces a lighter burden, and high out-of-pocket expenditure indicates inadequate public provision rather than high utilisation. B. The public health system is tiered from the sub-centre through the primary health centre and the community health centre to the sub-district and district hospital, and this hierarchy is the referral spine on which the first pillar's upgrade and the second pillar's empanelment both sit; naming the tier at which a failure occurs is what converts a general complaint about public hospitals into a diagnosis, because a vacancy at the sub-centre, a diagnostic gap at the primary health centre and a specialist shortage at the community health centre call for three different corrections. C. Primary care is first-contact preventive, promotive and basic curative care delivered at the wellness centre and the primary health centre, secondary care is specialist outpatient and inpatient care at the community health centre and the district hospital, and tertiary care is super-specialty and advanced procedural care at medical colleges and referral centres; the flagship programme is deliberately split along this line, with the first pillar addressing primary care and the second addressing secondary and tertiary care, so confusing the pillars is simultaneously a factual error and a conceptual one. D. Catastrophic health expenditure is health spending that exceeds a threshold share of household income or consumption, commonly stated in the range of ten to twenty-five per cent, and it pushes households into poverty or debt, with high out-of-pocket expenditure as its primary driver; this is the mechanism that makes health a social-justice question rather than only a service-delivery question, because a single hospitalisation can undo years of asset accumulation, and it is the bridge that connects this topic to the poverty and nutrition owner.

Answer: A. Explanation: Out-of-pocket expenditure is health spending paid directly by households at the point of service, and the owners record the National Health Accounts sequence as 47.1 per cent of total health expenditure in 2019-20, 44.4 per cent in 2020-21 and 39.4 per cent in 2021-22, with the latest cited official estimate being 39.4 per cent for the accounting year 2021-22; two disciplines follow, first that the publication date must never be confused with the accounting year, and second that a falling national share is a dated aggregate rather than evidence that every household faces a lighter burden, and high out-of-pocket expenditure indicates inadequate public provision rather than high utilisation. The remaining options belong to different chronology, actor or analytical categories.

Q30. Which chronology card should be filed under Out-of-pocket expenditure and its dated official sequence?

A. Primary care is first-contact preventive, promotive and basic curative care delivered at the wellness centre and the primary health centre, secondary care is specialist outpatient and inpatient care at the community health centre and the district hospital, and tertiary care is super-specialty and advanced procedural care at medical colleges and referral centres; the flagship programme is deliberately split along this line, with the first pillar addressing primary care and the second addressing secondary and tertiary care, so confusing the pillars is simultaneously a factual error and a conceptual one. B. Out-of-pocket expenditure is health spending paid directly by households at the point of service, and the owners record the National Health Accounts sequence as 47.1 per cent of total health expenditure in 2019-20, 44.4 per cent in 2020-21 and 39.4 per cent in 2021-22, with the latest cited official estimate being 39.4 per cent for the accounting year 2021-22; two disciplines follow, first that the publication date must never be confused with the accounting year, and second that a falling national share is a dated aggregate rather than evidence that every household faces a lighter burden, and high out-of-pocket expenditure indicates inadequate public provision rather than high utilisation. C. Marketisation refers to the dominance of private providers in curative care, often unregulated or under-regulated, and the owners record its consequences as high costs, unnecessary procedures, catastrophic spending and inequitable access; this is the exact concern raised by the 2024 demand, so an answer to it must define the phenomenon and its harms before proposing measures, and must avoid the two symmetrical errors of denying the private sector's real capacity contribution and of treating regulation as automatic once a statute exists. D. The public health system is tiered from the sub-centre through the primary health centre and the community health centre to the sub-district and district hospital, and this hierarchy is the referral spine on which the first pillar's upgrade and the second pillar's empanelment both sit; naming the tier at which a failure occurs is what converts a general complaint about public hospitals into a diagnosis, because a vacancy at the sub-centre, a diagnostic gap at the primary health centre and a specialist shortage at the community health centre call for three different corrections.

Answer: B. Explanation: Out-of-pocket expenditure is health spending paid directly by households at the point of service, and the owners record the National Health Accounts sequence as 47.1 per cent of total health expenditure in 2019-20, 44.4 per cent in 2020-21 and 39.4 per cent in 2021-22, with the latest cited official estimate being 39.4 per cent for the accounting year 2021-22; two disciplines follow, first that the publication date must never be confused with the accounting year, and second that a falling national share is a dated aggregate rather than evidence that every household faces a lighter burden, and high out-of-pocket expenditure indicates inadequate public provision rather than high utilisation. The remaining options belong to different chronology, actor or analytical categories.

Q31. Which option preserves the source-bounded meaning of Out-of-pocket expenditure and its dated official sequence?

A. Primary care is first-contact preventive, promotive and basic curative care delivered at the wellness centre and the primary health centre, secondary care is specialist outpatient and inpatient care at the community health centre and the district hospital, and tertiary care is super-specialty and advanced procedural care at medical colleges and referral centres; the flagship programme is deliberately split along this line, with the first pillar addressing primary care and the second addressing secondary and tertiary care, so confusing the pillars is simultaneously a factual error and a conceptual one. B. Marketisation refers to the dominance of private providers in curative care, often unregulated or under-regulated, and the owners record its consequences as high costs, unnecessary procedures, catastrophic spending and inequitable access; this is the exact concern raised by the 2024 demand, so an answer to it must define the phenomenon and its harms before proposing measures, and must avoid the two symmetrical errors of denying the private sector's real capacity contribution and of treating regulation as automatic once a statute exists. C.

Out-of-pocket expenditure is health spending paid directly by households at the point of service, and the owners record the National Health Accounts sequence as 47.1 per cent of total health expenditure in 2019-20, 44.4 per cent in 2020-21 and 39.4 per cent in 2021-22, with the latest cited official estimate being 39.4 per cent for the accounting year 2021-22; two disciplines follow, first that the publication date must never be confused with the accounting year, and second that a falling national share is a dated aggregate rather than evidence that every household faces a lighter burden, and high out-of-pocket expenditure indicates inadequate public provision rather than high utilisation. D. The regulatory instruments recorded by the owners are the Clinical Establishments Act, 2010, a Central Act providing for the registration and regulation of clinical establishments which States must adopt or replace with equivalent legislation, price capping of the kind exercised by the national pharmaceutical pricing authority over devices, and

standard treatment guidelines that promote rational care and cost control; each carries a stated enforcement condition, because State adoption of the Central Act varies and some States have not adopted or notified equivalent rules, and adoption of treatment guidelines varies across public and empanelled private providers, so a recommendation to regulate must name the adoption gap it is closing.

Answer: C. Explanation: Out-of-pocket expenditure is health spending paid directly by households at the point of service, and the owners record the National Health Accounts sequence as 47.1 per cent of total health expenditure in 2019-20, 44.4 per cent in 2020-21 and 39.4 per cent in 2021-22, with the latest cited official estimate being 39.4 per cent for the accounting year 2021-22; two disciplines follow, first that the publication date must never be confused with the accounting year, and second that a falling national share is a dated aggregate rather than evidence that every household faces a lighter burden, and high out-of-pocket expenditure indicates inadequate public provision rather than high utilisation. The remaining options belong to different chronology, actor or analytical categories.

Q32. Which statement avoids a close-option trap about Out-of-pocket expenditure and its dated official sequence?

A. Social health insurance pools contribution-linked cover, of which the employees' state insurance system is the Indian example, whereas tax-funded programmes draw on general revenue, which is how both the health mission and the hospitalisation pillar are financed, and the owners record that India uses a mixed and fragmented architecture rather than one comprehensive national pool; the standing objective-question trap is to describe the hospitalisation pillar as contribution-based social health insurance, which reverses the two financing families. B. The regulatory instruments recorded by the owners are the Clinical Establishments Act, 2010, a Central Act providing for the registration and regulation of clinical establishments which States must adopt or replace with equivalent legislation, price capping of the kind exercised by the national pharmaceutical pricing authority over devices, and standard treatment guidelines that promote rational care and cost control; each carries a stated enforcement condition, because State adoption of the Central Act varies and some States have not adopted or notified equivalent rules, and adoption of treatment guidelines varies across public and empanelled private providers, so a recommendation to regulate must name the adoption gap it is closing. C. Marketisation refers to the dominance of private providers in curative care, often unregulated or under-regulated, and the owners record its consequences as high costs, unnecessary procedures, catastrophic spending and inequitable access; this is the exact concern raised by the 2024 demand, so an answer to it must define the phenomenon and its harms before proposing measures, and must avoid the two symmetrical errors of denying the private sector's real capacity contribution and of treating regulation as automatic once a statute exists. D. Out-of-pocket expenditure is health spending paid directly by households at the point of service, and the owners record the National Health Accounts sequence as 47.1 per cent of total health expenditure in 2019-20, 44.4 per cent in 2020-21 and 39.4 per cent in 2021-22, with the latest cited official estimate being 39.4 per cent for the accounting year 2021-22; two disciplines follow, first that the publication date must never be confused with the accounting year, and second that a falling national share is a dated aggregate rather than evidence that every household faces a lighter burden, and high out-of-pocket expenditure indicates inadequate public provision rather than high utilisation.

Answer: D. Explanation: Out-of-pocket expenditure is health spending paid directly by households at the point of service, and the owners record the National Health Accounts sequence as 47.1 per cent of total health expenditure in 2019-20, 44.4 per cent in 2020-21 and 39.4 per cent in 2021-22, with the latest cited official estimate being 39.4 per cent for the accounting year 2021-22; two disciplines follow, first that the publication date must never be confused with the accounting year, and second that a falling national share is a dated aggregate rather than evidence that every household faces a lighter burden, and high out-of-pocket expenditure indicates inadequate public provision rather than high utilisation. The remaining options belong to different chronology, actor or analytical categories.

Q33. Which statement correctly identifies Catastrophic health expenditure as the impoverishment mechanism?

A. Catastrophic health expenditure is health spending that exceeds a threshold share of household income or consumption, commonly stated in the range of ten to twenty-five per cent, and it pushes households into poverty or debt, with high out-of-pocket expenditure as its primary driver; this is the mechanism that makes health a social-justice question rather than only a service-delivery question, because a single hospitalisation can undo years of asset accumulation, and it is the bridge that connects this topic to the poverty and nutrition owner. B. The public health system is tiered from the sub-centre through the primary health centre and the community health centre to the sub-district and district hospital, and this hierarchy is the referral spine on which the first pillar's upgrade and the second pillar's empanelment both sit; naming the tier at which a failure occurs is what converts a general complaint about public hospitals into a diagnosis, because a vacancy at the sub-centre, a diagnostic gap at the primary health centre and a specialist shortage at the community health centre call for three different corrections. C. Primary care is first-contact preventive, promotive and basic curative care delivered at the wellness centre and the primary health centre, secondary care is specialist outpatient and inpatient care at the community health centre and the district hospital, and tertiary care is super-specialty and advanced procedural care at medical colleges and referral centres; the flagship programme is deliberately split along this line, with the first pillar addressing primary care and the second addressing secondary and tertiary care, so confusing the pillars is simultaneously a factual error and a conceptual one. D. Marketisation refers to the dominance of private providers in curative care, often unregulated or under-regulated, and the owners record its consequences as high costs, unnecessary procedures, catastrophic spending and inequitable access; this is the exact concern raised by the 2024 demand, so an answer to it must define the phenomenon and its harms before proposing measures, and must avoid the two symmetrical errors of denying the private sector's real capacity contribution and of treating regulation as automatic once a statute exists.

Answer: A. Explanation: Catastrophic health expenditure is health spending that exceeds a threshold share of household income or consumption, commonly stated in the range of ten to twenty-five per cent, and it pushes households into poverty or debt, with high out-of-pocket expenditure as its primary driver; this is the mechanism that makes health a social-justice question rather than only a service-delivery question, because a single hospitalisation can undo years of asset accumulation, and it is the bridge that connects this topic to the poverty and nutrition owner. The remaining options belong to different chronology, actor or analytical categories.

Q34. Which chronology card should be filed under Catastrophic health expenditure as the impoverishment mechanism?

A. Marketisation refers to the dominance of private providers in curative care, often unregulated or under-regulated, and the owners record its consequences as high costs, unnecessary procedures, catastrophic spending and inequitable access; this is the exact concern raised by the 2024 demand, so an answer to it must define the phenomenon and its harms before proposing measures, and must avoid the two symmetrical errors of denying the private sector's real capacity contribution and of treating regulation as automatic once a statute exists. B. Catastrophic health expenditure is health spending that exceeds a threshold share of household income or consumption, commonly stated in the range of ten to twenty-five per cent, and it pushes households into poverty or debt, with high out-of-pocket expenditure as its primary driver; this is the mechanism that makes health a social-justice question rather than only a service-delivery question, because a single hospitalisation can undo years of asset accumulation, and it is the bridge that connects this topic to the poverty and nutrition owner. C. The regulatory instruments recorded by the owners are the Clinical Establishments Act, 2010, a Central Act providing for the registration and regulation of clinical establishments which States must adopt or replace with equivalent legislation, price capping of the kind exercised by the national pharmaceutical pricing authority over devices, and standard treatment guidelines that promote rational care and cost control; each carries a stated enforcement condition, because State adoption of the Central Act varies and some States have not adopted or notified equivalent rules, and adoption of treatment guidelines varies across public and empanelled private providers, so a recommendation to regulate must name the adoption gap it is closing. D. Primary care is first-contact preventive, promotive and basic curative care delivered at the wellness centre and the primary health centre, secondary care is specialist outpatient and inpatient care at the community health centre and the district hospital, and tertiary care is super-specialty and advanced procedural care at medical colleges and referral centres; the flagship programme is deliberately split along this line, with the first pillar addressing primary care and the second addressing secondary and tertiary care, so confusing the pillars is

simultaneously a factual error and a conceptual one.

Answer: B. Explanation: Catastrophic health expenditure is health spending that exceeds a threshold share of household income or consumption, commonly stated in the range of ten to twenty-five per cent, and it pushes households into poverty or debt, with high out-of-pocket expenditure as its primary driver; this is the mechanism that makes health a social-justice question rather than only a service-delivery question, because a single hospitalisation can undo years of asset accumulation, and it is the bridge that connects this topic to the poverty and nutrition owner. The remaining options belong to different chronology, actor or analytical categories.

Q35. Which option preserves the source-bounded meaning of Catastrophic health expenditure as the impoverishment mechanism?

A. Marketisation refers to the dominance of private providers in curative care, often unregulated or under-regulated, and the owners record its consequences as high costs, unnecessary procedures, catastrophic spending and inequitable access; this is the exact concern raised by the 2024 demand, so an answer to it must define the phenomenon and its harms before proposing measures, and must avoid the two symmetrical errors of denying the private sector's real capacity contribution and of treating regulation as automatic once a statute exists. B. The regulatory instruments recorded by the owners are the Clinical Establishments Act, 2010, a Central Act providing for the registration and regulation of clinical establishments which States must adopt or replace with equivalent legislation, price capping of the kind exercised by the national pharmaceutical pricing authority over devices, and standard treatment guidelines that promote rational care and cost control; each carries a stated enforcement condition, because State adoption of the Central Act varies and some States have not adopted or notified equivalent rules, and adoption of treatment guidelines varies across public and empanelled private providers, so a recommendation to regulate must name the adoption gap it is closing. C. Catastrophic health expenditure is health spending that exceeds a threshold share of household income or consumption, commonly stated in the range of ten to twenty-five per cent, and it pushes households into poverty or debt, with high out-of-pocket expenditure as its primary driver; this is the mechanism that makes health a social-justice question rather than only a service-delivery question, because a single hospitalisation can undo years of asset accumulation, and it is the bridge that connects this topic to the poverty and nutrition owner. D. Social health insurance pools contribution-linked cover, of which the employees' state insurance system is the Indian example, whereas tax-funded programmes draw on general revenue, which is how both the health mission and the hospitalisation pillar are financed, and the owners record that India uses a mixed and fragmented architecture rather than one comprehensive national pool; the standing objective-question trap is to describe the hospitalisation pillar as contribution-based social health insurance, which reverses the two financing families.

Answer: C. Explanation: Catastrophic health expenditure is health spending that exceeds a threshold share of household income or consumption, commonly stated in the range of ten to twenty-five per cent, and it pushes households into poverty or debt, with high out-of-pocket expenditure as its primary driver; this is the mechanism that makes health a social-justice question rather than only a service-delivery question, because a single hospitalisation can undo years of asset accumulation, and it is the bridge that connects this topic to the poverty and nutrition owner. The remaining options belong to different chronology, actor or analytical categories.

Q36. Which statement avoids a close-option trap about Catastrophic health expenditure as the impoverishment mechanism?

A. Social health insurance pools contribution-linked cover, of which the employees' state insurance system is the Indian example, whereas tax-funded programmes draw on general revenue, which is how both the health mission and the hospitalisation pillar are financed, and the owners record that India uses a mixed and fragmented architecture rather than one comprehensive national pool; the standing objective-question trap is to describe the hospitalisation pillar as contribution-based social health insurance, which reverses the two financing families. B. The financing chain runs from revenue collection through risk pooling and strategic purchasing to service provision, and the owners record the decisive consequence that insurance without public primary care leaves prevention, medicines and outpatient out-of-pocket expenditure unresolved; this is the analytical core of any question about financial protection, because hospitalisation cover addresses the catastrophic tail while the recurring outpatient and medicine burden, which is where most household health spending actually occurs, is addressed only by funded primary provision. C. The

regulatory instruments recorded by the owners are the Clinical Establishments Act, 2010, a Central Act providing for the registration and regulation of clinical establishments which States must adopt or replace with equivalent legislation, price capping of the kind exercised by the national pharmaceutical pricing authority over devices, and standard treatment guidelines that promote rational care and cost control; each carries a stated enforcement condition, because State adoption of the Central Act varies and some States have not adopted or notified equivalent rules, and adoption of treatment guidelines varies across public and empanelled private providers, so a recommendation to regulate must name the adoption gap it is closing. D. Catastrophic health expenditure is health spending that exceeds a threshold share of household income or consumption, commonly stated in the range of ten to twenty-five per cent, and it pushes households into poverty or debt, with high out-of-pocket expenditure as its primary driver; this is the mechanism that makes health a social-justice question rather than only a service-delivery question, because a single hospitalisation can undo years of asset accumulation, and it is the bridge that connects this topic to the poverty and nutrition owner.

Answer: D. Explanation: Catastrophic health expenditure is health spending that exceeds a threshold share of household income or consumption, commonly stated in the range of ten to twenty-five per cent, and it pushes households into poverty or debt, with high out-of-pocket expenditure as its primary driver; this is the mechanism that makes health a social-justice question rather than only a service-delivery question, because a single hospitalisation can undo years of asset accumulation, and it is the bridge that connects this topic to the poverty and nutrition owner. The remaining options belong to different chronology, actor or analytical categories.

Q37. Which statement correctly identifies The tiered public delivery structure?

A. The public health system is tiered from the sub-centre through the primary health centre and the community health centre to the sub-district and district hospital, and this hierarchy is the referral spine on which the first pillar's upgrade and the second pillar's empanelment both sit; naming the tier at which a failure occurs is what converts a general complaint about public hospitals into a diagnosis, because a vacancy at the sub-centre, a diagnostic gap at the primary health centre and a specialist shortage at the community health centre call for three different corrections. B. Primary care is first-contact preventive, promotive and basic curative care delivered at the wellness centre and the primary health centre, secondary care is specialist outpatient and inpatient care at the community health centre and the district hospital, and tertiary care is super-specialty and advanced procedural care at medical colleges and referral centres; the flagship programme is deliberately split along this line, with the first pillar addressing primary care and the second addressing secondary and tertiary care, so confusing the pillars is simultaneously a factual error and a conceptual one. C. The regulatory instruments recorded by the owners are the Clinical Establishments Act, 2010, a Central Act providing for the registration and regulation of clinical establishments which States must adopt or replace with equivalent legislation, price capping of the kind exercised by the national pharmaceutical pricing authority over devices, and standard treatment guidelines that promote rational care and cost control; each carries a stated enforcement condition, because State adoption of the Central Act varies and some States have not adopted or notified equivalent rules, and adoption of treatment guidelines varies across public and empanelled private providers, so a recommendation to regulate must name the adoption gap it is closing. D. Marketisation refers to the dominance of private providers in curative care, often unregulated or under-regulated, and the owners record its consequences as high costs, unnecessary procedures, catastrophic spending and inequitable access; this is the exact concern raised by the 2024 demand, so an answer to it must define the phenomenon and its harms before proposing measures, and must avoid the two symmetrical errors of denying the private sector's real capacity contribution and of treating regulation as automatic once a statute exists.

Answer: A. Explanation: The public health system is tiered from the sub-centre through the primary health centre and the community health centre to the sub-district and district hospital, and this hierarchy is the referral spine on which the first pillar's upgrade and the second pillar's empanelment both sit; naming the tier at which a failure occurs is what converts a general complaint about public hospitals into a diagnosis, because a vacancy at the sub-centre, a diagnostic gap at the primary health centre and a specialist shortage at the community health centre call for three different corrections. The remaining options belong to different chronology, actor or analytical categories.

Q38. Which chronology card should be filed under The tiered public delivery structure?

A. Social health insurance pools contribution-linked cover, of which the employees' state insurance system is the Indian example, whereas tax-funded programmes draw on general revenue, which is how both the health mission and the hospitalisation pillar are financed, and the owners record that India uses a mixed and fragmented architecture rather than one comprehensive national pool; the standing objective-question trap is to describe the hospitalisation pillar as contribution-based social health insurance, which reverses the two financing families. B. The public health system is tiered from the sub-centre through the primary health centre and the community health centre to the sub-district and district hospital, and this hierarchy is the referral spine on which the first pillar's upgrade and the second pillar's empanelment both sit; naming the tier at which a failure occurs is what converts a general complaint about public hospitals into a diagnosis, because a vacancy at the sub-centre, a diagnostic gap at the primary health centre and a specialist shortage at the community health centre call for three different corrections. C. The regulatory instruments recorded by the owners are the Clinical Establishments Act, 2010, a Central Act providing for the registration and regulation of clinical establishments which States must adopt or replace with equivalent legislation, price capping of the kind exercised by the national pharmaceutical pricing authority over devices, and standard treatment guidelines that promote rational care and cost control; each carries a stated enforcement condition, because State adoption of the Central Act varies and some States have not adopted or notified equivalent rules, and adoption of treatment guidelines varies across public and empanelled private providers, so a recommendation to regulate must name the adoption gap it is closing. D. Marketisation refers to the dominance of private providers in curative care, often unregulated or under-regulated, and the owners record its consequences as high costs, unnecessary procedures, catastrophic spending and inequitable access; this is the exact concern raised by the 2024 demand, so an answer to it must define the phenomenon and its harms before proposing measures, and must avoid the two symmetrical errors of denying the private sector's real capacity contribution and of treating regulation as automatic once a statute exists.

Answer: B. Explanation: The public health system is tiered from the sub-centre through the primary health centre and the community health centre to the sub-district and district hospital, and this hierarchy is the referral spine on which the first pillar's upgrade and the second pillar's empanelment both sit; naming the tier at which a failure occurs is what converts a general complaint about public hospitals into a diagnosis, because a vacancy at the sub-centre, a diagnostic gap at the primary health centre and a specialist shortage at the community health centre call for three different corrections. The remaining options belong to different chronology, actor or analytical categories.

Q39. Which option preserves the source-bounded meaning of The tiered public delivery structure?

A. Social health insurance pools contribution-linked cover, of which the employees' state insurance system is the Indian example, whereas tax-funded programmes draw on general revenue, which is how both the health mission and the hospitalisation pillar are financed, and the owners record that India uses a mixed and fragmented architecture rather than one comprehensive national pool; the standing objective-question trap is to describe the hospitalisation pillar as contribution-based social health insurance, which reverses the two financing families. B. The financing chain runs from revenue collection through risk pooling and strategic purchasing to service provision, and the owners record the decisive consequence that insurance without public primary care leaves prevention, medicines and outpatient out-of-pocket expenditure unresolved; this is the analytical core of any question about financial protection, because hospitalisation cover addresses the catastrophic tail while the recurring outpatient and medicine burden, which is where most household health spending actually occurs, is addressed only by funded primary provision. C. The public health system is tiered from the sub-centre through the primary health centre and the community health centre to the sub-district and district hospital, and this hierarchy is the referral spine on which the first pillar's upgrade and the second pillar's empanelment both sit; naming the tier at which a failure occurs is what converts a general complaint about public hospitals into a diagnosis, because a vacancy at the sub-centre, a diagnostic gap at the primary health centre and a specialist shortage at the community health centre call for three different corrections. D. The regulatory instruments recorded by the owners are the Clinical Establishments Act, 2010, a Central Act providing for the registration and regulation of clinical establishments which States must adopt or replace with equivalent legislation, price capping of the kind exercised by the national pharmaceutical pricing authority over devices, and standard treatment guidelines that promote rational care and cost control; each carries a stated enforcement condition, because State adoption of the Central Act varies and some States have not adopted or notified equivalent rules, and

adoption of treatment guidelines varies across public and empanelled private providers, so a recommendation to regulate must name the adoption gap it is closing.

Answer: C. Explanation: The public health system is tiered from the sub-centre through the primary health centre and the community health centre to the sub-district and district hospital, and this hierarchy is the referral spine on which the first pillar's upgrade and the second pillar's empanelment both sit; naming the tier at which a failure occurs is what converts a general complaint about public hospitals into a diagnosis, because a vacancy at the sub-centre, a diagnostic gap at the primary health centre and a specialist shortage at the community health centre call for three different corrections. The remaining options belong to different chronology, actor or analytical categories.

Q40. Which statement avoids a close-option trap about The tiered public delivery structure?

A. The owners route community health through the Alma-Ata commitments of equity, participation, inter-sectoral action and appropriate technology, and map them onto accredited social health activists, auxiliary nurse midwives, village health, sanitation and nutrition committees and the upgraded wellness centres; this mapping is what allows a community-health demand to be answered with named Indian institutions rather than with an abstract restatement of health for all, and it also explains why inter-sectoral determinants such as water, sanitation and nutrition sit inside a health answer rather than outside it. B. Social health insurance pools contribution-linked cover, of which the employees' state insurance system is the Indian example, whereas tax-funded programmes draw on general revenue, which is how both the health mission and the hospitalisation pillar are financed, and the owners record that India uses a mixed and fragmented architecture rather than one comprehensive national pool; the standing objective-question trap is to describe the hospitalisation pillar as contribution-based social health insurance, which reverses the two financing families. C. The financing chain runs from revenue collection through risk pooling and strategic purchasing to service provision, and the owners record the decisive consequence that insurance without public primary care leaves prevention, medicines and outpatient out-of-pocket expenditure unresolved; this is the analytical core of any question about financial protection, because hospitalisation cover addresses the catastrophic tail while the recurring outpatient and medicine burden, which is where most household health spending actually occurs, is addressed only by funded primary provision. D. The public health system is tiered from the sub-centre through the primary health centre and the community health centre to the sub-district and district hospital, and this hierarchy is the referral spine on which the first pillar's upgrade and the second pillar's empanelment both sit; naming the tier at which a failure occurs is what converts a general complaint about public hospitals into a diagnosis, because a vacancy at the sub-centre, a diagnostic gap at the primary health centre and a specialist shortage at the community health centre call for three different corrections.

Answer: D. Explanation: The public health system is tiered from the sub-centre through the primary health centre and the community health centre to the sub-district and district hospital, and this hierarchy is the referral spine on which the first pillar's upgrade and the second pillar's empanelment both sit; naming the tier at which a failure occurs is what converts a general complaint about public hospitals into a diagnosis, because a vacancy at the sub-centre, a diagnostic gap at the primary health centre and a specialist shortage at the community health centre call for three different corrections. The remaining options belong to different chronology, actor or analytical categories.

Q41. Which statement correctly identifies Primary, secondary and tertiary care?

A. Primary care is first-contact preventive, promotive and basic curative care delivered at the wellness centre and the primary health centre, secondary care is specialist outpatient and inpatient care at the community health centre and the district hospital, and tertiary care is super-specialty and advanced procedural care at medical colleges and referral centres; the flagship programme is deliberately split along this line, with the first pillar addressing primary care and the second addressing secondary and tertiary care, so confusing the pillars is simultaneously a factual error and a conceptual one. B. Social health insurance pools contribution-linked cover, of which the employees' state insurance system is the Indian example, whereas tax-funded programmes draw on general revenue, which is how both the health mission and the hospitalisation pillar are financed, and the owners record that India uses a mixed and fragmented architecture rather than one comprehensive national pool; the standing objective-question trap is to describe the hospitalisation pillar as contribution-based social health insurance, which reverses the two financing families. C. Marketisation refers to the dominance of private providers in curative care, often unregulated or under-regulated, and the owners record its consequences as high costs, unnecessary procedures, catastrophic

spending and inequitable access; this is the exact concern raised by the 2024 demand, so an answer to it must define the phenomenon and its harms before proposing measures, and must avoid the two symmetrical errors of denying the private sector's real capacity contribution and of treating regulation as automatic once a statute exists. D. The regulatory instruments recorded by the owners are the Clinical Establishments Act, 2010, a Central Act providing for the registration and regulation of clinical establishments which States must adopt or replace with equivalent legislation, price capping of the kind exercised by the national pharmaceutical pricing authority over devices, and standard treatment guidelines that promote rational care and cost control; each carries a stated enforcement condition, because State adoption of the Central Act varies and some States have not adopted or notified equivalent rules, and adoption of treatment guidelines varies across public and empanelled private providers, so a recommendation to regulate must name the adoption gap it is closing.

Answer: A. Explanation: Primary care is first-contact preventive, promotive and basic curative care delivered at the wellness centre and the primary health centre, secondary care is specialist outpatient and inpatient care at the community health centre and the district hospital, and tertiary care is super-specialty and advanced procedural care at medical colleges and referral centres; the flagship programme is deliberately split along this line, with the first pillar addressing primary care and the second addressing secondary and tertiary care, so confusing the pillars is simultaneously a factual error and a conceptual one. The remaining options belong to different chronology, actor or analytical categories.

Q42. Which chronology card should be filed under Primary, secondary and tertiary care?

A. Social health insurance pools contribution-linked cover, of which the employees' state insurance system is the Indian example, whereas tax-funded programmes draw on general revenue, which is how both the health mission and the hospitalisation pillar are financed, and the owners record that India uses a mixed and fragmented architecture rather than one comprehensive national pool; the standing objective-question trap is to describe the hospitalisation pillar as contribution-based social health insurance, which reverses the two financing families. B. Primary care is first-contact preventive, promotive and basic curative care delivered at the wellness centre and the primary health centre, secondary care is specialist outpatient and inpatient care at the community health centre and the district hospital, and tertiary care is super-specialty and advanced procedural care at medical colleges and referral centres; the flagship programme is deliberately split along this line, with the first pillar addressing primary care and the second addressing secondary and tertiary care, so confusing the pillars is simultaneously a factual error and a conceptual one. C. The regulatory instruments recorded by the owners are the Clinical Establishments Act, 2010, a Central Act providing for the registration and regulation of clinical establishments which States must adopt or replace with equivalent legislation, price capping of the kind exercised by the national pharmaceutical pricing authority over devices, and standard treatment guidelines that promote rational care and cost control; each carries a stated enforcement condition, because State adoption of the Central Act varies and some States have not adopted or notified equivalent rules, and adoption of treatment guidelines varies across public and empanelled private providers, so a recommendation to regulate must name the adoption gap it is closing. D. The financing chain runs from revenue collection through risk pooling and strategic purchasing to service provision, and the owners record the decisive consequence that insurance without public primary care leaves prevention, medicines and outpatient out-of-pocket expenditure unresolved; this is the analytical core of any question about financial protection, because hospitalisation cover addresses the catastrophic tail while the recurring outpatient and medicine burden, which is where most household health spending actually occurs, is addressed only by funded primary provision.

Answer: B. Explanation: Primary care is first-contact preventive, promotive and basic curative care delivered at the wellness centre and the primary health centre, secondary care is specialist outpatient and inpatient care at the community health centre and the district hospital, and tertiary care is super-specialty and advanced procedural care at medical colleges and referral centres; the flagship programme is deliberately split along this line, with the first pillar addressing primary care and the second addressing secondary and tertiary care, so confusing the pillars is simultaneously a factual error and a conceptual one. The remaining options belong to different chronology, actor or analytical categories.

Q43. Which option preserves the source-bounded meaning of Primary, secondary and tertiary care?

A. The owners route community health through the Alma-Ata commitments of equity, participation, inter-sectoral action and appropriate technology, and map them onto accredited social health activists, auxiliary nurse midwives, village health, sanitation and nutrition committees and the upgraded wellness centres; this mapping is what allows a community-health demand to be answered with named Indian institutions rather than with an abstract restatement of health for all, and it also explains why inter-sectoral determinants such as water, sanitation and nutrition sit inside a health answer rather than outside it. B. The financing chain runs from revenue collection through risk pooling and strategic purchasing to service provision, and the owners record the decisive consequence that insurance without public primary care leaves prevention, medicines and outpatient out-of-pocket expenditure unresolved; this is the analytical core of any question about financial protection, because hospitalisation cover addresses the catastrophic tail while the recurring outpatient and medicine burden, which is where most household health spending actually occurs, is addressed only by funded primary provision. C. Primary care is first-contact preventive, promotive and basic curative care delivered at the wellness centre and the primary health centre, secondary care is specialist outpatient and inpatient care at the community health centre and the district hospital, and tertiary care is super-specialty and advanced procedural care at medical colleges and referral centres; the flagship programme is deliberately split along this line, with the first pillar addressing primary care and the second addressing secondary and tertiary care, so confusing the pillars is simultaneously a factual error and a conceptual one. D. Social health insurance pools contribution-linked cover, of which the employees' state insurance system is the Indian example, whereas tax-funded programmes draw on general revenue, which is how both the health mission and the hospitalisation pillar are financed, and the owners record that India uses a mixed and fragmented architecture rather than one comprehensive national pool; the standing objective-question trap is to describe the hospitalisation pillar as contribution-based social health insurance, which reverses the two financing families.

Answer: C. Explanation: Primary care is first-contact preventive, promotive and basic curative care delivered at the wellness centre and the primary health centre, secondary care is specialist outpatient and inpatient care at the community health centre and the district hospital, and tertiary care is super-specialty and advanced procedural care at medical colleges and referral centres; the flagship programme is deliberately split along this line, with the first pillar addressing primary care and the second addressing secondary and tertiary care, so confusing the pillars is simultaneously a factual error and a conceptual one. The remaining options belong to different chronology, actor or analytical categories.

Q44. Which statement avoids a close-option trap about Primary, secondary and tertiary care?

A. The owners record that the health mission, free drugs and diagnostics initiatives and the health-infrastructure mission strengthen systems while uneven State capacity and workforce vacancies limit outcomes, and that Union financing and standards depend on State delivery capacity because health is delivered by the States; the answer consequence is that a national financing announcement is a necessary but not sufficient condition, and that a graded verdict on any national health programme must be conditioned on the delivery capacity of the State in question. B. The owners route community health through the Alma-Ata commitments of equity, participation, inter-sectoral action and appropriate technology, and map them onto accredited social health activists, auxiliary nurse midwives, village health, sanitation and nutrition committees and the upgraded wellness centres; this mapping is what allows a community-health demand to be answered with named Indian institutions rather than with an abstract restatement of health for all, and it also explains why inter-sectoral determinants such as water, sanitation and nutrition sit inside a health answer rather than outside it. C. The financing chain runs from revenue collection through risk pooling and strategic purchasing to service provision, and the owners record the decisive consequence that insurance without public primary care leaves prevention, medicines and outpatient out-of-pocket expenditure unresolved; this is the analytical core of any question about financial protection, because hospitalisation cover addresses the catastrophic tail while the recurring outpatient and medicine burden, which is where most household health spending actually occurs, is addressed only by funded primary provision. D. Primary care is first-contact preventive, promotive and basic curative care delivered at the wellness centre and the primary health centre, secondary care is specialist outpatient and inpatient care at the community health centre and the district hospital, and tertiary care is super-specialty and advanced procedural care at medical colleges and referral centres; the flagship programme is deliberately split along this line, with the first pillar addressing primary care and the second addressing secondary and tertiary care, so confusing the pillars is simultaneously a factual error and a conceptual one.

Answer: D. Explanation: Primary care is first-contact preventive, promotive and basic curative care delivered at the wellness centre and the primary health centre, secondary care is specialist outpatient and inpatient care at the community health centre and the district hospital, and tertiary care is super-specialty and advanced procedural care at medical colleges and referral centres; the flagship programme is deliberately split along this line, with the first pillar addressing primary care and the second addressing secondary and tertiary care, so confusing the pillars is simultaneously a factual error and a conceptual one. The remaining options belong to different chronology, actor or analytical categories.

Q45. Which statement correctly identifies Marketisation of health care and what it produces?

A. Marketisation refers to the dominance of private providers in curative care, often unregulated or under-regulated, and the owners record its consequences as high costs, unnecessary procedures, catastrophic spending and inequitable access; this is the exact concern raised by the 2024 demand, so an answer to it must define the phenomenon and its harms before proposing measures, and must avoid the two symmetrical errors of denying the private sector's real capacity contribution and of treating regulation as automatic once a statute exists. B. The regulatory instruments recorded by the owners are the Clinical Establishments Act, 2010, a Central Act providing for the registration and regulation of clinical establishments which States must adopt or replace with equivalent legislation, price capping of the kind exercised by the national pharmaceutical pricing authority over devices, and standard treatment guidelines that promote rational care and cost control; each carries a stated enforcement condition, because State adoption of the Central Act varies and some States have not adopted or notified equivalent rules, and adoption of treatment guidelines varies across public and empanelled private providers, so a recommendation to regulate must name the adoption gap it is closing. C. The financing chain runs from revenue collection through risk pooling and strategic purchasing to service provision, and the owners record the decisive consequence that insurance without public primary care leaves prevention, medicines and outpatient out-of-pocket expenditure unresolved; this is the analytical core of any question about financial protection, because hospitalisation cover addresses the catastrophic tail while the recurring outpatient and medicine burden, which is where most household health spending actually occurs, is addressed only by funded primary provision. D. Social health insurance pools contribution-linked cover, of which the employees' state insurance system is the Indian example, whereas tax-funded programmes draw on general revenue, which is how both the health mission and the hospitalisation pillar are financed, and the owners record that India uses a mixed and fragmented architecture rather than one comprehensive national pool; the standing objective-question trap is to describe the hospitalisation pillar as contribution-based social health insurance, which reverses the two financing families.

Answer: A. Explanation: Marketisation refers to the dominance of private providers in curative care, often unregulated or under-regulated, and the owners record its consequences as high costs, unnecessary procedures, catastrophic spending and inequitable access; this is the exact concern raised by the 2024 demand, so an answer to it must define the phenomenon and its harms before proposing measures, and must avoid the two symmetrical errors of denying the private sector's real capacity contribution and of treating regulation as automatic once a statute exists. The remaining options belong to different chronology, actor or analytical categories.

Q46. Which chronology card should be filed under Marketisation of health care and what it produces?

A. The financing chain runs from revenue collection through risk pooling and strategic purchasing to service provision, and the owners record the decisive consequence that insurance without public primary care leaves prevention, medicines and outpatient out-of-pocket expenditure unresolved; this is the analytical core of any question about financial protection, because hospitalisation cover addresses the catastrophic tail while the recurring outpatient and medicine burden, which is where most household health spending actually occurs, is addressed only by funded primary provision. B. Marketisation refers to the dominance of private providers in curative care, often unregulated or under-regulated, and the owners record its consequences as high costs, unnecessary procedures, catastrophic spending and inequitable access; this is the exact concern raised by the 2024 demand, so an answer to it must define the phenomenon and its harms before proposing measures, and must avoid the two symmetrical errors of denying the private sector's real capacity contribution and of treating regulation as automatic once a statute exists. C. Social health insurance pools contribution-linked cover, of which the employees' state insurance system is the Indian example, whereas tax-funded programmes draw on general revenue, which is how both the health mission and the

hospitalisation pillar are financed, and the owners record that India uses a mixed and fragmented architecture rather than one comprehensive national pool; the standing objective-question trap is to describe the hospitalisation pillar as contribution-based social health insurance, which reverses the two financing families. D. The owners route community health through the Alma-Ata commitments of equity, participation, inter-sectoral action and appropriate technology, and map them onto accredited social health activists, auxiliary nurse midwives, village health, sanitation and nutrition committees and the upgraded wellness centres; this mapping is what allows a community-health demand to be answered with named Indian institutions rather than with an abstract restatement of health for all, and it also explains why inter-sectoral determinants such as water, sanitation and nutrition sit inside a health answer rather than outside it.

Answer: B. Explanation: Marketisation refers to the dominance of private providers in curative care, often unregulated or under-regulated, and the owners record its consequences as high costs, unnecessary procedures, catastrophic spending and inequitable access; this is the exact concern raised by the 2024 demand, so an answer to it must define the phenomenon and its harms before proposing measures, and must avoid the two symmetrical errors of denying the private sector's real capacity contribution and of treating regulation as automatic once a statute exists. The remaining options belong to different chronology, actor or analytical categories.

Q47. Which option preserves the source-bounded meaning of Marketisation of health care and what it produces?

A. The owners route community health through the Alma-Ata commitments of equity, participation, inter-sectoral action and appropriate technology, and map them onto accredited social health activists, auxiliary nurse midwives, village health, sanitation and nutrition committees and the upgraded wellness centres; this mapping is what allows a community-health demand to be answered with named Indian institutions rather than with an abstract restatement of health for all, and it also explains why inter-sectoral determinants such as water, sanitation and nutrition sit inside a health answer rather than outside it. B. The owners record that the health mission, free drugs and diagnostics initiatives and the health-infrastructure mission strengthen systems while uneven State capacity and workforce vacancies limit outcomes, and that Union financing and standards depend on State delivery capacity because health is delivered by the States; the answer consequence is that a national financing announcement is a necessary but not sufficient condition, and that a graded verdict on any national health programme must be conditioned on the delivery capacity of the State in question. C. Marketisation refers to the dominance of private providers in curative care, often unregulated or under-regulated, and the owners record its consequences as high costs, unnecessary procedures, catastrophic spending and inequitable access; this is the exact concern raised by the 2024 demand, so an answer to it must define the phenomenon and its harms before proposing measures, and must avoid the two symmetrical errors of denying the private sector's real capacity contribution and of treating regulation as automatic once a statute exists. D. The financing chain runs from revenue collection through risk pooling and strategic purchasing to service provision, and the owners record the decisive consequence that insurance without public primary care leaves prevention, medicines and outpatient out-of-pocket expenditure unresolved; this is the analytical core of any question about financial protection, because hospitalisation cover addresses the catastrophic tail while the recurring outpatient and medicine burden, which is where most household health spending actually occurs, is addressed only by funded primary provision.

Answer: C. Explanation: Marketisation refers to the dominance of private providers in curative care, often unregulated or under-regulated, and the owners record its consequences as high costs, unnecessary procedures, catastrophic spending and inequitable access; this is the exact concern raised by the 2024 demand, so an answer to it must define the phenomenon and its harms before proposing measures, and must avoid the two symmetrical errors of denying the private sector's real capacity contribution and of treating regulation as automatic once a statute exists. The remaining options belong to different chronology, actor or analytical categories.

Q48. Which statement avoids a close-option trap about Marketisation of health care and what it produces?

A. The owners record that the health mission, free drugs and diagnostics initiatives and the health-infrastructure mission strengthen systems while uneven State capacity and workforce vacancies limit outcomes, and that Union financing and standards depend on State delivery capacity because health is delivered by the States; the answer consequence is that a national financing announcement is a necessary but not sufficient condition, and that a graded verdict on any national health programme must be conditioned on the delivery capacity of the State in question. B. The owners route community health through the Alma-Ata commitments of equity, participation, inter-sectoral action and appropriate technology, and map them onto accredited social health activists, auxiliary nurse midwives, village health, sanitation and nutrition committees and the upgraded wellness centres; this mapping is what allows a community-health demand to be answered with named Indian institutions rather than with an abstract restatement of health for all, and it also explains why inter-sectoral determinants such as water, sanitation and nutrition sit inside a health answer rather than outside it. C. The Mental Healthcare Act, 2017 supplies a rights framework and the national tele-mental-health service expands access, but the owners record expressly that neither replaces district-level professionals and continuity of care; this is the folder's standard illustration that a statutory right and a technology channel together still leave a human-resource gap, and it is directly usable in any answer that asks whether a legal entitlement has been converted into a service. D. Marketisation refers to the dominance of private providers in curative care, often unregulated or under-regulated, and the owners record its consequences as high costs, unnecessary procedures, catastrophic spending and inequitable access; this is the exact concern raised by the 2024 demand, so an answer to it must define the phenomenon and its harms before proposing measures, and must avoid the two symmetrical errors of denying the private sector's real capacity contribution and of treating regulation as automatic once a statute exists.

Answer: D. Explanation: Marketisation refers to the dominance of private providers in curative care, often unregulated or under-regulated, and the owners record its consequences as high costs, unnecessary procedures, catastrophic spending and inequitable access; this is the exact concern raised by the 2024 demand, so an answer to it must define the phenomenon and its harms before proposing measures, and must avoid the two symmetrical errors of denying the private sector's real capacity contribution and of treating regulation as automatic once a statute exists. The remaining options belong to different chronology, actor or analytical categories.

Q49. Which statement correctly identifies The regulatory toolkit and its enforcement condition?

A. The regulatory instruments recorded by the owners are the Clinical Establishments Act, 2010, a Central Act providing for the registration and regulation of clinical establishments which States must adopt or replace with equivalent legislation, price capping of the kind exercised by the national pharmaceutical pricing authority over devices, and standard treatment guidelines that promote rational care and cost control; each carries a stated enforcement condition, because State adoption of the Central Act varies and some States have not adopted or notified equivalent rules, and adoption of treatment guidelines varies across public and empanelled private providers, so a recommendation to regulate must name the adoption gap it is closing. B. The owners route community health through the Alma-Ata commitments of equity, participation, inter-sectoral action and appropriate technology, and map them onto accredited social health activists, auxiliary nurse midwives, village health, sanitation and nutrition committees and the upgraded wellness centres; this mapping is what allows a community-health demand to be answered with named Indian institutions rather than with an abstract restatement of health for all, and it also explains why inter-sectoral determinants such as water, sanitation and nutrition sit inside a health answer rather than outside it. C. The financing chain runs from revenue collection through risk pooling and strategic purchasing to service provision, and the owners record the decisive consequence that insurance without public primary care leaves prevention, medicines and outpatient out-of-pocket expenditure unresolved; this is the analytical core of any question about financial protection, because hospitalisation cover addresses the catastrophic tail while the recurring outpatient and medicine burden, which is where most household health spending actually occurs, is addressed only by funded primary provision. D. Social health insurance pools contribution-linked cover, of which the employees' state insurance system is the Indian example, whereas tax-funded programmes draw on general revenue, which is how both the health mission and the hospitalisation pillar are financed, and the owners record that India uses a mixed and fragmented architecture rather than one comprehensive national pool; the standing objective-question trap is to describe the hospitalisation pillar as contribution-based social health insurance, which reverses the two financing families.

Answer: A. Explanation: The regulatory instruments recorded by the owners are the Clinical Establishments Act, 2010, a Central Act providing for the registration and regulation of clinical establishments which States must adopt or replace with equivalent legislation, price capping of the kind exercised by the national pharmaceutical pricing authority over devices, and standard treatment guidelines that promote rational care and cost control; each carries a stated enforcement condition, because State adoption of the Central Act varies and some States have not adopted or notified equivalent rules, and adoption of treatment guidelines varies across public and empanelled private providers, so a recommendation to regulate must name the adoption gap it is closing. The remaining options belong to different chronology, actor or analytical categories.

Q50. Which chronology card should be filed under The regulatory toolkit and its enforcement condition?

A. The financing chain runs from revenue collection through risk pooling and strategic purchasing to service provision, and the owners record the decisive consequence that insurance without public primary care leaves prevention, medicines and outpatient out-of-pocket expenditure unresolved; this is the analytical core of any question about financial protection, because hospitalisation cover addresses the catastrophic tail while the recurring outpatient and medicine burden, which is where most household health spending actually occurs, is addressed only by funded primary provision. B. The regulatory instruments recorded by the owners are the Clinical Establishments Act, 2010, a Central Act providing for the registration and regulation of clinical establishments which States must adopt or replace with equivalent legislation, price capping of the kind exercised by the national pharmaceutical pricing authority over devices, and standard treatment guidelines that promote rational care and cost control; each carries a stated enforcement condition, because State adoption of the Central Act varies and some States have not adopted or notified equivalent rules, and adoption of treatment guidelines varies across public and empanelled private providers, so a recommendation to regulate must name the adoption gap it is closing. C. The owners record that the health mission, free drugs and diagnostics initiatives and the health-infrastructure mission strengthen systems while uneven State capacity and workforce vacancies limit outcomes, and that Union financing and standards depend on State delivery capacity because health is delivered by the States; the answer consequence is that a national financing announcement is a necessary but not sufficient condition, and that a graded verdict on any national health programme must be conditioned on the delivery capacity of the State in question. D. The owners route community health through the Alma-Ata commitments of equity, participation, inter-sectoral action and appropriate technology, and map them onto accredited social health activists, auxiliary nurse midwives, village health, sanitation and nutrition committees and the upgraded wellness centres; this mapping is what allows a community-health demand to be answered with named Indian institutions rather than with an abstract restatement of health for all, and it also explains why inter-sectoral determinants such as water, sanitation and nutrition sit inside a health answer rather than outside it.

Answer: B. Explanation: The regulatory instruments recorded by the owners are the Clinical Establishments Act, 2010, a Central Act providing for the registration and regulation of clinical establishments which States must adopt or replace with equivalent legislation, price capping of the kind exercised by the national pharmaceutical pricing authority over devices, and standard treatment guidelines that promote rational care and cost control; each carries a stated enforcement condition, because State adoption of the Central Act varies and some States have not adopted or notified equivalent rules, and adoption of treatment guidelines varies across public and empanelled private providers, so a recommendation to regulate must name the adoption gap it is closing. The remaining options belong to different chronology, actor or analytical categories.

Q51. Which option preserves the source-bounded meaning of The regulatory toolkit and its enforcement condition?

A. The owners record that the health mission, free drugs and diagnostics initiatives and the health-infrastructure mission strengthen systems while uneven State capacity and workforce vacancies limit outcomes, and that Union financing and standards depend on State delivery capacity because health is delivered by the States; the answer consequence is that a national financing announcement is a necessary but not sufficient condition, and that a graded verdict on any national health programme must be conditioned on the delivery capacity of the State in question. B. The owners route community health through the Alma-Ata commitments of equity, participation, inter-sectoral action and appropriate technology, and map them onto accredited social health activists, auxiliary nurse midwives, village

health, sanitation and nutrition committees and the upgraded wellness centres; this mapping is what allows a community-health demand to be answered with named Indian institutions rather than with an abstract restatement of health for all, and it also explains why inter-sectoral determinants such as water, sanitation and nutrition sit inside a health answer rather than outside it. C. The regulatory instruments recorded by the owners are the Clinical Establishments Act, 2010, a Central Act providing for the registration and regulation of clinical establishments which States must adopt or replace with equivalent legislation, price capping of the kind exercised by the national pharmaceutical pricing authority over devices, and standard treatment guidelines that promote rational care and cost control; each carries a stated enforcement condition, because State adoption of the Central Act varies and some States have not adopted or notified equivalent rules, and adoption of treatment guidelines varies across public and empanelled private providers, so a recommendation to regulate must name the adoption gap it is closing. D. The Mental Healthcare Act, 2017 supplies a rights framework and the national tele-mental-health service expands access, but the owners record expressly that neither replaces district-level professionals and continuity of care; this is the folder's standard illustration that a statutory right and a technology channel together still leave a human-resource gap, and it is directly usable in any answer that asks whether a legal entitlement has been converted into a service.

Answer: C. Explanation: The regulatory instruments recorded by the owners are the Clinical Establishments Act, 2010, a Central Act providing for the registration and regulation of clinical establishments which States must adopt or replace with equivalent legislation, price capping of the kind exercised by the national pharmaceutical pricing authority over devices, and standard treatment guidelines that promote rational care and cost control; each carries a stated enforcement condition, because State adoption of the Central Act varies and some States have not adopted or notified equivalent rules, and adoption of treatment guidelines varies across public and empanelled private providers, so a recommendation to regulate must name the adoption gap it is closing. The remaining options belong to different chronology, actor or analytical categories.

Q52. Which statement avoids a close-option trap about The regulatory toolkit and its enforcement condition?

A. The health mission is a centrally sponsored scheme with Centre-State cost sharing that supports accredited social health activists and auxiliary nurse midwives, free drugs and diagnostics, mobile medical units and maternal-health support of the kind delivered through the maternity protection scheme, while State health agencies implement the hospitalisation pillar at State level and may add State-funded top-up schemes; naming this two-layer arrangement is what allows an answer to explain why identical national schemes produce unequal outcomes across States. B. The owners record that the health mission, free drugs and diagnostics initiatives and the health-infrastructure mission strengthen systems while uneven State capacity and workforce vacancies limit outcomes, and that Union financing and standards depend on State delivery capacity because health is delivered by the States; the answer consequence is that a national financing announcement is a necessary but not sufficient condition, and that a graded verdict on any national health programme must be conditioned on the delivery capacity of the State in question. C. The Mental Healthcare Act, 2017 supplies a rights framework and the national tele-mental-health service expands access, but the owners record expressly that neither replaces district-level professionals and continuity of care; this is the folder's standard illustration that a statutory right and a technology channel together still leave a human-resource gap, and it is directly usable in any answer that asks whether a legal entitlement has been converted into a service. D. The regulatory instruments recorded by the owners are the Clinical Establishments Act, 2010, a Central Act providing for the registration and regulation of clinical establishments which States must adopt or replace with equivalent legislation, price capping of the kind exercised by the national pharmaceutical pricing authority over devices, and standard treatment guidelines that promote rational care and cost control; each carries a stated enforcement condition, because State adoption of the Central Act varies and some States have not adopted or notified equivalent rules, and adoption of treatment guidelines varies across public and empanelled private providers, so a recommendation to regulate must name the adoption gap it is closing.

Answer: D. Explanation: The regulatory instruments recorded by the owners are the Clinical Establishments Act, 2010, a Central Act providing for the registration and regulation of clinical establishments which States must adopt or replace with equivalent legislation, price capping of the kind exercised by the national pharmaceutical pricing authority over devices, and standard treatment guidelines that promote rational care and cost control; each carries a stated enforcement condition, because State adoption of the Central Act varies and some States have not adopted or notified equivalent rules, and adoption of treatment guidelines

varies across public and empanelled private providers, so a recommendation to regulate must name the adoption gap it is closing. The remaining options belong to different chronology, actor or analytical categories.

Q53. Which statement correctly identifies Social health insurance against tax-funded provision?

A. Social health insurance pools contribution-linked cover, of which the employees' state insurance system is the Indian example, whereas tax-funded programmes draw on general revenue, which is how both the health mission and the hospitalisation pillar are financed, and the owners record that India uses a mixed and fragmented architecture rather than one comprehensive national pool; the standing objective-question trap is to describe the hospitalisation pillar as contribution-based social health insurance, which reverses the two financing families. B. The owners route community health through the Alma-Ata commitments of equity, participation, inter-sectoral action and appropriate technology, and map them onto accredited social health activists, auxiliary nurse midwives, village health, sanitation and nutrition committees and the upgraded wellness centres; this mapping is what allows a community-health demand to be answered with named Indian institutions rather than with an abstract restatement of health for all, and it also explains why inter-sectoral determinants such as water, sanitation and nutrition sit inside a health answer rather than outside it. C. The financing chain runs from revenue collection through risk pooling and strategic purchasing to service provision, and the owners record the decisive consequence that insurance without public primary care leaves prevention, medicines and outpatient out-of-pocket expenditure unresolved; this is the analytical core of any question about financial protection, because hospitalisation cover addresses the catastrophic tail while the recurring outpatient and medicine burden, which is where most household health spending actually occurs, is addressed only by funded primary provision. D. The owners record that the health mission, free drugs and diagnostics initiatives and the health-infrastructure mission strengthen systems while uneven State capacity and workforce vacancies limit outcomes, and that Union financing and standards depend on State delivery capacity because health is delivered by the States; the answer consequence is that a national financing announcement is a necessary but not sufficient condition, and that a graded verdict on any national health programme must be conditioned on the delivery capacity of the State in question.

Answer: A. Explanation: Social health insurance pools contribution-linked cover, of which the employees' state insurance system is the Indian example, whereas tax-funded programmes draw on general revenue, which is how both the health mission and the hospitalisation pillar are financed, and the owners record that India uses a mixed and fragmented architecture rather than one comprehensive national pool; the standing objective-question trap is to describe the hospitalisation pillar as contribution-based social health insurance, which reverses the two financing families. The remaining options belong to different chronology, actor or analytical categories.

Q54. Which chronology card should be filed under Social health insurance against tax-funded provision?

A. The Mental Healthcare Act, 2017 supplies a rights framework and the national tele-mental-health service expands access, but the owners record expressly that neither replaces district-level professionals and continuity of care; this is the folder's standard illustration that a statutory right and a technology channel together still leave a human-resource gap, and it is directly usable in any answer that asks whether a legal entitlement has been converted into a service. B. Social health insurance pools contribution-linked cover, of which the employees' state insurance system is the Indian example, whereas tax-funded programmes draw on general revenue, which is how both the health mission and the hospitalisation pillar are financed, and the owners record that India uses a mixed and fragmented architecture rather than one comprehensive national pool; the standing objective-question trap is to describe the hospitalisation pillar as contribution-based social health insurance, which reverses the two financing families. C. The owners record that the health mission, free drugs and diagnostics initiatives and the health-infrastructure mission strengthen systems while uneven State capacity and workforce vacancies limit outcomes, and that Union financing and standards depend on State delivery capacity because health is delivered by the States; the answer consequence is that a national financing announcement is a necessary but not sufficient condition, and that a graded verdict on any national health programme must be conditioned on the delivery capacity of the State in question. D. The owners route community health through the Alma-Ata commitments of equity, participation, inter-sectoral action and appropriate technology, and map them onto accredited social health activists, auxiliary nurse midwives, village health, sanitation and nutrition committees and the upgraded wellness centres; this mapping is what allows a community-health demand to be answered with named

Indian institutions rather than with an abstract restatement of health for all, and it also explains why inter-sectoral determinants such as water, sanitation and nutrition sit inside a health answer rather than outside it.

Answer: B. Explanation: Social health insurance pools contribution-linked cover, of which the employees' state insurance system is the Indian example, whereas tax-funded programmes draw on general revenue, which is how both the health mission and the hospitalisation pillar are financed, and the owners record that India uses a mixed and fragmented architecture rather than one comprehensive national pool; the standing objective-question trap is to describe the hospitalisation pillar as contribution-based social health insurance, which reverses the two financing families. The remaining options belong to different chronology, actor or analytical categories.

Q55. Which option preserves the source-bounded meaning of Social health insurance against tax-funded provision?

A. The Mental Healthcare Act, 2017 supplies a rights framework and the national tele-mental-health service expands access, but the owners record expressly that neither replaces district-level professionals and continuity of care; this is the folder's standard illustration that a statutory right and a technology channel together still leave a human-resource gap, and it is directly usable in any answer that asks whether a legal entitlement has been converted into a service. B. The owners record that the health mission, free drugs and diagnostics initiatives and the health-infrastructure mission strengthen systems while uneven State capacity and workforce vacancies limit outcomes, and that Union financing and standards depend on State delivery capacity because health is delivered by the States; the answer consequence is that a national financing announcement is a necessary but not sufficient condition, and that a graded verdict on any national health programme must be conditioned on the delivery capacity of the State in question. C. Social health insurance pools contribution-linked cover, of which the employees' state insurance system is the Indian example, whereas tax-funded programmes draw on general revenue, which is how both the health mission and the hospitalisation pillar are financed, and the owners record that India uses a mixed and fragmented architecture rather than one comprehensive national pool; the standing objective-question trap is to describe the hospitalisation pillar as contribution-based social health insurance, which reverses the two financing families. D. The health mission is a centrally sponsored scheme with Centre-State cost sharing that supports accredited social health activists and auxiliary nurse midwives, free drugs and diagnostics, mobile medical units and maternal-health support of the kind delivered through the maternity protection scheme, while State health agencies implement the hospitalisation pillar at State level and may add State-funded top-up schemes; naming this two-layer arrangement is what allows an answer to explain why identical national schemes produce unequal outcomes across States.

Answer: C. Explanation: Social health insurance pools contribution-linked cover, of which the employees' state insurance system is the Indian example, whereas tax-funded programmes draw on general revenue, which is how both the health mission and the hospitalisation pillar are financed, and the owners record that India uses a mixed and fragmented architecture rather than one comprehensive national pool; the standing objective-question trap is to describe the hospitalisation pillar as contribution-based social health insurance, which reverses the two financing families. The remaining options belong to different chronology, actor or analytical categories.

Q56. Which statement avoids a close-option trap about Social health insurance against tax-funded provision?

A. The health mission is a centrally sponsored scheme with Centre-State cost sharing that supports accredited social health activists and auxiliary nurse midwives, free drugs and diagnostics, mobile medical units and maternal-health support of the kind delivered through the maternity protection scheme, while State health agencies implement the hospitalisation pillar at State level and may add State-funded top-up schemes; naming this two-layer arrangement is what allows an answer to explain why identical national schemes produce unequal outcomes across States. B. The owners record that cumulative card counts and cumulative claim counts are dashboard outputs and are not by themselves a measure of service quality or of financial protection, so this package asserts no card total, claim total, empanelment count, wellness-centre count, expenditure share beyond the dated national health accounts sequence, or state-level rank. On ownership, four General Studies Mains demands are routed to this topic in the audited ledgers, from 2018, 2020, 2021 and 2024, with the 2020 geriatric and maternal-care demand recorded as cross-cutting ownership shared with the senior-citizens topic, and three objective demands are routed, namely the 2023 maternal-health scheme question and the 2023 public-health-system question whose official keys the ledgers record

as unavailable locally, and the 2024 maternal-health scheme question for which the ledger records that the official Set-A key is available locally but expressly states that no answer is recorded or inferred; this package therefore records no option for any of the three. C. The Mental Healthcare Act, 2017 supplies a rights framework and the national tele-mental-health service expands access, but the owners record expressly that neither replaces district-level professionals and continuity of care; this is the folder's standard illustration that a statutory right and a technology channel together still leave a human-resource gap, and it is directly usable in any answer that asks whether a legal entitlement has been converted into a service. D. Social health insurance pools contribution-linked cover, of which the employees' state insurance system is the Indian example, whereas tax-funded programmes draw on general revenue, which is how both the health mission and the hospitalisation pillar are financed, and the owners record that India uses a mixed and fragmented architecture rather than one comprehensive national pool; the standing objective-question trap is to describe the hospitalisation pillar as contribution-based social health insurance, which reverses the two financing families.

Answer: D. Explanation: Social health insurance pools contribution-linked cover, of which the employees' state insurance system is the Indian example, whereas tax-funded programmes draw on general revenue, which is how both the health mission and the hospitalisation pillar are financed, and the owners record that India uses a mixed and fragmented architecture rather than one comprehensive national pool; the standing objective-question trap is to describe the hospitalisation pillar as contribution-based social health insurance, which reverses the two financing families. The remaining options belong to different chronology, actor or analytical categories.

Q57. Which statement correctly identifies The health financing chain and what insurance cannot buy?

A. The financing chain runs from revenue collection through risk pooling and strategic purchasing to service provision, and the owners record the decisive consequence that insurance without public primary care leaves prevention, medicines and outpatient out-of-pocket expenditure unresolved; this is the analytical core of any question about financial protection, because hospitalisation cover addresses the catastrophic tail while the recurring outpatient and medicine burden, which is where most household health spending actually occurs, is addressed only by funded primary provision. B. The owners record that the health mission, free drugs and diagnostics initiatives and the health-infrastructure mission strengthen systems while uneven State capacity and workforce vacancies limit outcomes, and that Union financing and standards depend on State delivery capacity because health is delivered by the States; the answer consequence is that a national financing announcement is a necessary but not sufficient condition, and that a graded verdict on any national health programme must be conditioned on the delivery capacity of the State in question. C. The owners route community health through the Alma-Ata commitments of equity, participation, inter-sectoral action and appropriate technology, and map them onto accredited social health activists, auxiliary nurse midwives, village health, sanitation and nutrition committees and the upgraded wellness centres; this mapping is what allows a community-health demand to be answered with named Indian institutions rather than with an abstract restatement of health for all, and it also explains why inter-sectoral determinants such as water, sanitation and nutrition sit inside a health answer rather than outside it. D. The Mental Healthcare Act, 2017 supplies a rights framework and the national tele-mental-health service expands access, but the owners record expressly that neither replaces district-level professionals and continuity of care; this is the folder's standard illustration that a statutory right and a technology channel together still leave a human-resource gap, and it is directly usable in any answer that asks whether a legal entitlement has been converted into a service.

Answer: A. Explanation: The financing chain runs from revenue collection through risk pooling and strategic purchasing to service provision, and the owners record the decisive consequence that insurance without public primary care leaves prevention, medicines and outpatient out-of-pocket expenditure unresolved; this is the analytical core of any question about financial protection, because hospitalisation cover addresses the catastrophic tail while the recurring outpatient and medicine burden, which is where most household health spending actually occurs, is addressed only by funded primary provision. The remaining options belong to different chronology, actor or analytical categories.

Q58. Which chronology card should be filed under The health financing chain and what insurance cannot buy?

A. The Mental Healthcare Act, 2017 supplies a rights framework and the national tele-mental-health service expands access, but the owners record expressly that neither replaces district-level professionals and continuity of care; this is the folder's standard illustration that a statutory right and a technology channel together still leave a human-resource gap, and it is directly usable in any answer that asks whether a legal entitlement has been converted into a service. B. The financing chain runs from revenue collection through risk pooling and strategic purchasing to service provision, and the owners record the decisive consequence that insurance without public primary care leaves prevention, medicines and outpatient out-of-pocket expenditure unresolved; this is the analytical core of any question about financial protection, because hospitalisation cover addresses the catastrophic tail while the recurring outpatient and medicine burden, which is where most household health spending actually occurs, is addressed only by funded primary provision. C. The health mission is a centrally sponsored scheme with Centre-State cost sharing that supports accredited social health activists and auxiliary nurse midwives, free drugs and diagnostics, mobile medical units and maternal-health support of the kind delivered through the maternity protection scheme, while State health agencies implement the hospitalisation pillar at State level and may add State-funded top-up schemes; naming this two-layer arrangement is what allows an answer to explain why identical national schemes produce unequal outcomes across States. D. The owners record that the health mission, free drugs and diagnostics initiatives and the health-infrastructure mission strengthen systems while uneven State capacity and workforce vacancies limit outcomes, and that Union financing and standards depend on State delivery capacity because health is delivered by the States; the answer consequence is that a national financing announcement is a necessary but not sufficient condition, and that a graded verdict on any national health programme must be conditioned on the delivery capacity of the State in question.

Answer: B. Explanation: The financing chain runs from revenue collection through risk pooling and strategic purchasing to service provision, and the owners record the decisive consequence that insurance without public primary care leaves prevention, medicines and outpatient out-of-pocket expenditure unresolved; this is the analytical core of any question about financial protection, because hospitalisation cover addresses the catastrophic tail while the recurring outpatient and medicine burden, which is where most household health spending actually occurs, is addressed only by funded primary provision. The remaining options belong to different chronology, actor or analytical categories.

Q59. Which option preserves the source-bounded meaning of The health financing chain and what insurance cannot buy?

A. The owners record that cumulative card counts and cumulative claim counts are dashboard outputs and are not by themselves a measure of service quality or of financial protection, so this package asserts no card total, claim total, empanelment count, wellness-centre count, expenditure share beyond the dated national health accounts sequence, or state-level rank. On ownership, four General Studies Mains demands are routed to this topic in the audited ledgers, from 2018, 2020, 2021 and 2024, with the 2020 geriatric and maternal-care demand recorded as cross-cutting ownership shared with the senior-citizens topic, and three objective demands are routed, namely the 2023 maternal-health scheme question and the 2023 public-health-system question whose official keys the ledgers record as unavailable locally, and the 2024 maternal-health scheme question for which the ledger records that the official Set-A key is available locally but expressly states that no answer is recorded or inferred; this package therefore records no option for any of the three. B. The health mission is a centrally sponsored scheme with Centre-State cost sharing that supports accredited social health activists and auxiliary nurse midwives, free drugs and diagnostics, mobile medical units and maternal-health support of the kind delivered through the maternity protection scheme, while State health agencies implement the hospitalisation pillar at State level and may add State-funded top-up schemes; naming this two-layer arrangement is what allows an answer to explain why identical national schemes produce unequal outcomes across States. C. The financing chain runs from revenue collection through risk pooling and strategic purchasing to service provision, and the owners record the decisive consequence that insurance without public primary care leaves prevention, medicines and outpatient out-of-pocket expenditure unresolved; this is the analytical core of any question about financial protection, because hospitalisation cover addresses the catastrophic tail while the recurring outpatient and medicine burden, which is where most household health spending actually occurs, is addressed only by funded primary provision. D. The Mental Healthcare Act, 2017 supplies a rights framework and the national tele-mental-health service expands access, but the owners record expressly that neither

replaces district-level professionals and continuity of care; this is the folder's standard illustration that a statutory right and a technology channel together still leave a human-resource gap, and it is directly usable in any answer that asks whether a legal entitlement has been converted into a service.

Answer: C. Explanation: The financing chain runs from revenue collection through risk pooling and strategic purchasing to service provision, and the owners record the decisive consequence that insurance without public primary care leaves prevention, medicines and outpatient out-of-pocket expenditure unresolved; this is the analytical core of any question about financial protection, because hospitalisation cover addresses the catastrophic tail while the recurring outpatient and medicine burden, which is where most household health spending actually occurs, is addressed only by funded primary provision. The remaining options belong to different chronology, actor or analytical categories.

Q60. Which statement avoids a close-option trap about The health financing chain and what insurance cannot buy?

A. The health mission is a centrally sponsored scheme with Centre-State cost sharing that supports accredited social health activists and auxiliary nurse midwives, free drugs and diagnostics, mobile medical units and maternal-health support of the kind delivered through the maternity protection scheme, while State health agencies implement the hospitalisation pillar at State level and may add State-funded top-up schemes; naming this two-layer arrangement is what allows an answer to explain why identical national schemes produce unequal outcomes across States. B. This topic owns the health half of the capability argument, namely the universal-health-coverage standard, the policy vision, the mission that funds public delivery, the two pillars of the flagship health programme, the tiered public infrastructure and the financial-protection measure by which the whole system is judged, and its distinctive feature is the densest direct-question record in the folder: four General Studies Mains demands from 2018, 2020, 2021 and 2024 and three objective demands from 2023 and 2024 are routed to it in the audited ledgers; the macro economics of health financing belongs to the Economy owner, the elderly-care share of the 2020 demand is jointly owned with the senior-citizens topic, and this topic argues coverage, provision and protection. C. The owners record that cumulative card counts and cumulative claim counts are dashboard outputs and are not by themselves a measure of service quality or of financial protection, so this package asserts no card total, claim total, empanelment count, wellness-centre count, expenditure share beyond the dated national health accounts sequence, or state-level rank. On ownership, four General Studies Mains demands are routed to this topic in the audited ledgers, from 2018, 2020, 2021 and 2024, with the 2020 geriatric and maternal-care demand recorded as cross-cutting ownership shared with the senior-citizens topic, and three objective demands are routed, namely the 2023 maternal-health scheme question and the 2023 public-health-system question whose official keys the ledgers record as unavailable locally, and the 2024 maternal-health scheme question for which the ledger records that the official Set-A key is available locally but expressly states that no answer is recorded or inferred; this package therefore records no option for any of the three. D. The financing chain runs from revenue collection through risk pooling and strategic purchasing to service provision, and the owners record the decisive consequence that insurance without public primary care leaves prevention, medicines and outpatient out-of-pocket expenditure unresolved; this is the analytical core of any question about financial protection, because hospitalisation cover addresses the catastrophic tail while the recurring outpatient and medicine burden, which is where most household health spending actually occurs, is addressed only by funded primary provision.

Answer: D. Explanation: The financing chain runs from revenue collection through risk pooling and strategic purchasing to service provision, and the owners record the decisive consequence that insurance without public primary care leaves prevention, medicines and outpatient out-of-pocket expenditure unresolved; this is the analytical core of any question about financial protection, because hospitalisation cover addresses the catastrophic tail while the recurring outpatient and medicine burden, which is where most household health spending actually occurs, is addressed only by funded primary provision. The remaining options belong to different chronology, actor or analytical categories.

Q61. Which statement correctly identifies Community health and the Alma-Ata inheritance?

A. The owners route community health through the Alma-Ata commitments of equity, participation, inter-sectoral action and appropriate technology, and map them onto accredited social health activists, auxiliary nurse midwives, village health, sanitation and nutrition committees and the upgraded wellness centres; this mapping is what allows a community-health demand to be answered with named Indian institutions rather than with an abstract restatement of health for all, and it also explains why inter-sectoral determinants such as water, sanitation and nutrition sit inside a health answer rather than outside it. B. The Mental Healthcare Act, 2017 supplies a rights framework and the national tele-mental-health service expands access, but the owners record expressly that neither replaces district-level professionals and continuity of care; this is the folder's standard illustration that a statutory right and a technology channel together still leave a human-resource gap, and it is directly usable in any answer that asks whether a legal entitlement has been converted into a service. C. The health mission is a centrally sponsored scheme with Centre-State cost sharing that supports accredited social health activists and auxiliary nurse midwives, free drugs and diagnostics, mobile medical units and maternal-health support of the kind delivered through the maternity protection scheme, while State health agencies implement the hospitalisation pillar at State level and may add State-funded top-up schemes; naming this two-layer arrangement is what allows an answer to explain why identical national schemes produce unequal outcomes across States. D. The owners record that the health mission, free drugs and diagnostics initiatives and the health-infrastructure mission strengthen systems while uneven State capacity and workforce vacancies limit outcomes, and that Union financing and standards depend on State delivery capacity because health is delivered by the States; the answer consequence is that a national financing announcement is a necessary but not sufficient condition, and that a graded verdict on any national health programme must be conditioned on the delivery capacity of the State in question.

Answer: A. Explanation: The owners route community health through the Alma-Ata commitments of equity, participation, inter-sectoral action and appropriate technology, and map them onto accredited social health activists, auxiliary nurse midwives, village health, sanitation and nutrition committees and the upgraded wellness centres; this mapping is what allows a community-health demand to be answered with named Indian institutions rather than with an abstract restatement of health for all, and it also explains why inter-sectoral determinants such as water, sanitation and nutrition sit inside a health answer rather than outside it. The remaining options belong to different chronology, actor or analytical categories.

Q62. Which chronology card should be filed under Community health and the Alma-Ata inheritance?

A. The Mental Healthcare Act, 2017 supplies a rights framework and the national tele-mental-health service expands access, but the owners record expressly that neither replaces district-level professionals and continuity of care; this is the folder's standard illustration that a statutory right and a technology channel together still leave a human-resource gap, and it is directly usable in any answer that asks whether a legal entitlement has been converted into a service. B. The owners route community health through the Alma-Ata commitments of equity, participation, inter-sectoral action and appropriate technology, and map them onto accredited social health activists, auxiliary nurse midwives, village health, sanitation and nutrition committees and the upgraded wellness centres; this mapping is what allows a community-health demand to be answered with named Indian institutions rather than with an abstract restatement of health for all, and it also explains why inter-sectoral determinants such as water, sanitation and nutrition sit inside a health answer rather than outside it. C. The health mission is a centrally sponsored scheme with Centre-State cost sharing that supports accredited social health activists and auxiliary nurse midwives, free drugs and diagnostics, mobile medical units and maternal-health support of the kind delivered through the maternity protection scheme, while State health agencies implement the hospitalisation pillar at State level and may add State-funded top-up schemes; naming this two-layer arrangement is what allows an answer to explain why identical national schemes produce unequal outcomes across States. D. The owners record that cumulative card counts and cumulative claim counts are dashboard outputs and are not by themselves a measure of service quality or of financial protection, so this package asserts no card total, claim total, empanelment count, wellness-centre count, expenditure share beyond the dated national health accounts sequence, or state-level rank. On ownership, four General Studies Mains demands are routed to this topic in the audited ledgers, from 2018, 2020, 2021 and 2024, with the 2020 geriatric and maternal-care demand recorded as cross-cutting ownership shared with the senior-citizens topic, and three objective demands are routed, namely the 2023 maternal-health scheme question and the 2023 public-health-system question whose official keys the ledgers record as unavailable locally, and the 2024 maternal-health scheme question for which the ledger

records that the official Set-A key is available locally but expressly states that no answer is recorded or inferred; this package therefore records no option for any of the three.

Answer: B. Explanation: The owners route community health through the Alma-Ata commitments of equity, participation, inter-sectoral action and appropriate technology, and map them onto accredited social health activists, auxiliary nurse midwives, village health, sanitation and nutrition committees and the upgraded wellness centres; this mapping is what allows a community-health demand to be answered with named Indian institutions rather than with an abstract restatement of health for all, and it also explains why inter-sectoral determinants such as water, sanitation and nutrition sit inside a health answer rather than outside it. The remaining options belong to different chronology, actor or analytical categories.

Q63. Which option preserves the source-bounded meaning of Community health and the Alma-Ata inheritance?

A. This topic owns the health half of the capability argument, namely the universal-health-coverage standard, the policy vision, the mission that funds public delivery, the two pillars of the flagship health programme, the tiered public infrastructure and the financial-protection measure by which the whole system is judged, and its distinctive feature is the densest direct-question record in the folder: four General Studies Mains demands from 2018, 2020, 2021 and 2024 and three objective demands from 2023 and 2024 are routed to it in the audited ledgers; the macro economics of health financing belongs to the Economy owner, the elderly-care share of the 2020 demand is jointly owned with the senior-citizens topic, and this topic argues coverage, provision and protection. B. The health mission is a centrally sponsored scheme with Centre-State cost sharing that supports accredited social health activists and auxiliary nurse midwives, free drugs and diagnostics, mobile medical units and maternal-health support of the kind delivered through the maternity protection scheme, while State health agencies implement the hospitalisation pillar at State level and may add State-funded top-up schemes; naming this two-layer arrangement is what allows an answer to explain why identical national schemes produce unequal outcomes across States. C. The owners route community health through the Alma-Ata commitments of equity, participation, inter-sectoral action and appropriate technology, and map them onto accredited social health activists, auxiliary nurse midwives, village health, sanitation and nutrition committees and the upgraded wellness centres; this mapping is what allows a community-health demand to be answered with named Indian institutions rather than with an abstract restatement of health for all, and it also explains why inter-sectoral determinants such as water, sanitation and nutrition sit inside a health answer rather than outside it. D. The owners record that cumulative card counts and cumulative claim counts are dashboard outputs and are not by themselves a measure of service quality or of financial protection, so this package asserts no card total, claim total, empanelment count, wellness-centre count, expenditure share beyond the dated national health accounts sequence, or state-level rank. On ownership, four General Studies Mains demands are routed to this topic in the audited ledgers, from 2018, 2020, 2021 and 2024, with the 2020 geriatric and maternal-care demand recorded as cross-cutting ownership shared with the senior-citizens topic, and three objective demands are routed, namely the 2023 maternal-health scheme question and the 2023 public-health-system question whose official keys the ledgers record as unavailable locally, and the 2024 maternal-health scheme question for which the ledger records that the official Set-A key is available locally but expressly states that no answer is recorded or inferred; this package therefore records no option for any of the three.

Answer: C. Explanation: The owners route community health through the Alma-Ata commitments of equity, participation, inter-sectoral action and appropriate technology, and map them onto accredited social health activists, auxiliary nurse midwives, village health, sanitation and nutrition committees and the upgraded wellness centres; this mapping is what allows a community-health demand to be answered with named Indian institutions rather than with an abstract restatement of health for all, and it also explains why inter-sectoral determinants such as water, sanitation and nutrition sit inside a health answer rather than outside it. The remaining options belong to different chronology, actor or analytical categories.

Q64. Which statement avoids a close-option trap about Community health and the Alma-Ata inheritance?

A. This topic owns the health half of the capability argument, namely the universal-health-coverage standard, the policy vision, the mission that funds public delivery, the two pillars of the flagship health programme, the tiered public infrastructure and the financial-protection measure by which the whole system is judged, and its distinctive feature is the densest direct-question record in the folder: four General Studies Mains demands from 2018, 2020, 2021 and 2024 and three objective demands from 2023 and 2024 are routed to it in the audited ledgers; the macro economics of health financing belongs to the Economy owner, the elderly-care share of the 2020 demand is jointly owned with the senior-citizens topic, and this topic argues coverage, provision and protection. B. Universal health coverage means ensuring that all people have access to the promotive, preventive, curative and rehabilitative health services they need, of sufficient quality, without suffering financial hardship, and it is analysed on three dimensions that the World Health Organization presents as a cube: population, which is the breadth of who is covered, services, which is the depth of what is covered, and financial protection, which is the height of how little the user pays at the point of service; the analytical value of the cube is that a system can expand on one dimension while contracting on another, so an answer that reports only beneficiary numbers has measured one edge of a three-dimensional claim. C. The owners record that cumulative card counts and cumulative claim counts are dashboard outputs and are not by themselves a measure of service quality or of financial protection, so this package asserts no card total, claim total, empanelment count, wellness-centre count, expenditure share beyond the dated national health accounts sequence, or state-level rank. On ownership, four General Studies Mains demands are routed to this topic in the audited ledgers, from 2018, 2020, 2021 and 2024, with the 2020 geriatric and maternal-care demand recorded as cross-cutting ownership shared with the senior-citizens topic, and three objective demands are routed, namely the 2023 maternal-health scheme question and the 2023 public-health-system question whose official keys the ledgers record as unavailable locally, and the 2024 maternal-health scheme question for which the ledger records that the official Set-A key is available locally but expressly states that no answer is recorded or inferred; this package therefore records no option for any of the three. D. The owners route community health through the Alma-Ata commitments of equity, participation, inter-sectoral action and appropriate technology, and map them onto accredited social health activists, auxiliary nurse midwives, village health, sanitation and nutrition committees and the upgraded wellness centres; this mapping is what allows a community-health demand to be answered with named Indian institutions rather than with an abstract restatement of health for all, and it also explains why inter-sectoral determinants such as water, sanitation and nutrition sit inside a health answer rather than outside it.

Answer: D. Explanation: The owners route community health through the Alma-Ata commitments of equity, participation, inter-sectoral action and appropriate technology, and map them onto accredited social health activists, auxiliary nurse midwives, village health, sanitation and nutrition committees and the upgraded wellness centres; this mapping is what allows a community-health demand to be answered with named Indian institutions rather than with an abstract restatement of health for all, and it also explains why inter-sectoral determinants such as water, sanitation and nutrition sit inside a health answer rather than outside it. The remaining options belong to different chronology, actor or analytical categories.

Q65. Which statement correctly identifies System capacity, federalism and the workforce constraint?

A. The owners record that the health mission, free drugs and diagnostics initiatives and the health-infrastructure mission strengthen systems while uneven State capacity and workforce vacancies limit outcomes, and that Union financing and standards depend on State delivery capacity because health is delivered by the States; the answer consequence is that a national financing announcement is a necessary but not sufficient condition, and that a graded verdict on any national health programme must be conditioned on the delivery capacity of the State in question. B. The Mental Healthcare Act, 2017 supplies a rights framework and the national tele-mental-health service expands access, but the owners record expressly that neither replaces district-level professionals and continuity of care; this is the folder's standard illustration that a statutory right and a technology channel together still leave a human-resource gap, and it is directly usable in any answer that asks whether a legal entitlement has been converted into a service. C. The health mission is a centrally sponsored scheme with Centre-State cost sharing that supports accredited social health activists and auxiliary nurse midwives, free drugs and diagnostics, mobile medical units and maternal-health support of the kind delivered through the maternity protection scheme, while State health agencies implement the hospitalisation pillar at State level and may add State-funded top-up schemes; naming this two-layer arrangement is

what allows an answer to explain why identical national schemes produce unequal outcomes across States. D. The owners record that cumulative card counts and cumulative claim counts are dashboard outputs and are not by themselves a measure of service quality or of financial protection, so this package asserts no card total, claim total, empanelment count, wellness-centre count, expenditure share beyond the dated national health accounts sequence, or state-level rank. On ownership, four General Studies Mains demands are routed to this topic in the audited ledgers, from 2018, 2020, 2021 and 2024, with the 2020 geriatric and maternal-care demand recorded as cross-cutting ownership shared with the senior-citizens topic, and three objective demands are routed, namely the 2023 maternal-health scheme question and the 2023 public-health-system question whose official keys the ledgers record as unavailable locally, and the 2024 maternal-health scheme question for which the ledger records that the official Set-A key is available locally but expressly states that no answer is recorded or inferred; this package therefore records no option for any of the three.

Answer: A. Explanation: The owners record that the health mission, free drugs and diagnostics initiatives and the health-infrastructure mission strengthen systems while uneven State capacity and workforce vacancies limit outcomes, and that Union financing and standards depend on State delivery capacity because health is delivered by the States; the answer consequence is that a national financing announcement is a necessary but not sufficient condition, and that a graded verdict on any national health programme must be conditioned on the delivery capacity of the State in question. The remaining options belong to different chronology, actor or analytical categories.

Q66. Which chronology card should be filed under System capacity, federalism and the workforce constraint?

A. This topic owns the health half of the capability argument, namely the universal-health-coverage standard, the policy vision, the mission that funds public delivery, the two pillars of the flagship health programme, the tiered public infrastructure and the financial-protection measure by which the whole system is judged, and its distinctive feature is the densest direct-question record in the folder: four General Studies Mains demands from 2018, 2020, 2021 and 2024 and three objective demands from 2023 and 2024 are routed to it in the audited ledgers; the macro economics of health financing belongs to the Economy owner, the elderly-care share of the 2020 demand is jointly owned with the senior-citizens topic, and this topic argues coverage, provision and protection. B. The owners record that the health mission, free drugs and diagnostics initiatives and the health-infrastructure mission strengthen systems while uneven State capacity and workforce vacancies limit outcomes, and that Union financing and standards depend on State delivery capacity because health is delivered by the States; the answer consequence is that a national financing announcement is a necessary but not sufficient condition, and that a graded verdict on any national health programme must be conditioned on the delivery capacity of the State in question. C. The owners record that cumulative card counts and cumulative claim counts are dashboard outputs and are not by themselves a measure of service quality or of financial protection, so this package asserts no card total, claim total, empanelment count, wellness-centre count, expenditure share beyond the dated national health accounts sequence, or state-level rank. On ownership, four General Studies Mains demands are routed to this topic in the audited ledgers, from 2018, 2020, 2021 and 2024, with the 2020 geriatric and maternal-care demand recorded as cross-cutting ownership shared with the senior-citizens topic, and three objective demands are routed, namely the 2023 maternal-health scheme question and the 2023 public-health-system question whose official keys the ledgers record as unavailable locally, and the 2024 maternal-health scheme question for which the ledger records that the official Set-A key is available locally but expressly states that no answer is recorded or inferred; this package therefore records no option for any of the three. D. The health mission is a centrally sponsored scheme with Centre-State cost sharing that supports accredited social health activists and auxiliary nurse midwives, free drugs and diagnostics, mobile medical units and maternal-health support of the kind delivered through the maternity protection scheme, while State health agencies implement the hospitalisation pillar at State level and may add State-funded top-up schemes; naming this two-layer arrangement is what allows an answer to explain why identical national schemes produce unequal outcomes across States.

Answer: B. Explanation: The owners record that the health mission, free drugs and diagnostics initiatives and the health-infrastructure mission strengthen systems while uneven State capacity and workforce vacancies limit outcomes, and that Union financing and standards depend on State delivery capacity because health is delivered by the States; the answer consequence is that a national financing announcement is a necessary but not sufficient condition, and that a graded verdict on any national health programme must be conditioned on the delivery capacity of the State in question. The remaining options belong to different chronology, actor

Q67. Which option preserves the source-bounded meaning of System capacity, federalism and the workforce constraint?

A. Universal health coverage means ensuring that all people have access to the promotive, preventive, curative and rehabilitative health services they need, of sufficient quality, without suffering financial hardship, and it is analysed on three dimensions that the World Health Organization presents as a cube: population, which is the breadth of who is covered, services, which is the depth of what is covered, and financial protection, which is the height of how little the user pays at the point of service; the analytical value of the cube is that a system can expand on one dimension while contracting on another, so an answer that reports only beneficiary numbers has measured one edge of a three-dimensional claim. B. This topic owns the health half of the capability argument, namely the universal-health-coverage standard, the policy vision, the mission that funds public delivery, the two pillars of the flagship health programme, the tiered public infrastructure and the financial-protection measure by which the whole system is judged, and its distinctive feature is the densest direct-question record in the folder: four General Studies Mains demands from 2018, 2020, 2021 and 2024 and three objective demands from 2023 and 2024 are routed to it in the audited ledgers; the macro economics of health financing belongs to the Economy owner, the elderly-care share of the 2020 demand is jointly owned with the senior-citizens topic, and this topic argues coverage, provision and protection. C. The owners record that the health mission, free drugs and diagnostics initiatives and the health-infrastructure mission strengthen systems while uneven State capacity and workforce vacancies limit outcomes, and that Union financing and standards depend on State delivery capacity because health is delivered by the States; the answer consequence is that a national financing announcement is a necessary but not sufficient condition, and that a graded verdict on any national health programme must be conditioned on the delivery capacity of the State in question. D. The owners record that cumulative card counts and cumulative claim counts are dashboard outputs and are not by themselves a measure of service quality or of financial protection, so this package asserts no card total, claim total, empanelment count, wellness-centre count, expenditure share beyond the dated national health accounts sequence, or state-level rank. On ownership, four General Studies Mains demands are routed to this topic in the audited ledgers, from 2018, 2020, 2021 and 2024, with the 2020 geriatric and maternal-care demand recorded as cross-cutting ownership shared with the senior-citizens topic, and three objective demands are routed, namely the 2023 maternal-health scheme question and the 2023 public-health-system question whose official keys the ledgers record as unavailable locally, and the 2024 maternal-health scheme question for which the ledger records that the official Set-A key is available locally but expressly states that no answer is recorded or inferred; this package therefore records no option for any of the three.

Answer: C. Explanation: The owners record that the health mission, free drugs and diagnostics initiatives and the health-infrastructure mission strengthen systems while uneven State capacity and workforce vacancies limit outcomes, and that Union financing and standards depend on State delivery capacity because health is delivered by the States; the answer consequence is that a national financing announcement is a necessary but not sufficient condition, and that a graded verdict on any national health programme must be conditioned on the delivery capacity of the State in question. The remaining options belong to different chronology, actor or analytical categories.

Q68. Which statement avoids a close-option trap about System capacity, federalism and the workforce constraint?

A. Universal health coverage means ensuring that all people have access to the promotive, preventive, curative and rehabilitative health services they need, of sufficient quality, without suffering financial hardship, and it is analysed on three dimensions that the World Health Organization presents as a cube: population, which is the breadth of who is covered, services, which is the depth of what is covered, and financial protection, which is the height of how little the user pays at the point of service; the analytical value of the cube is that a system can expand on one dimension while contracting on another, so an answer that reports only beneficiary numbers has measured one edge of a three-dimensional claim. B. This topic owns the health half of the capability argument, namely the universal-health-coverage standard, the policy vision, the mission that funds public delivery, the two pillars of the flagship health programme, the tiered public infrastructure and the financial-protection measure by which the whole system is judged, and its distinctive feature is the densest direct-question record in the folder: four General Studies Mains demands from 2018, 2020, 2021 and 2024 and three objective demands from 2023 and 2024 are routed to it in

the audited ledgers; the macro economics of health financing belongs to the Economy owner, the elderly-care share of the 2020 demand is jointly owned with the senior-citizens topic, and this topic argues coverage, provision and protection. C. The National Health Policy, 2017 is India's guiding health-policy document, and the owners record its goals as raising public health expenditure to two and a half per cent of gross domestic product, strengthening primary care, achieving universal health coverage and reducing out-of-pocket expenditure; the examinable discipline is that it is a policy document and not an enacted statute, so its targets are commitments rather than justiciable entitlements, and describing them as legally binding is a status error of exactly the kind this folder is built to prevent. D. The owners record that the health mission, free drugs and diagnostics initiatives and the health-infrastructure mission strengthen systems while uneven State capacity and workforce vacancies limit outcomes, and that Union financing and standards depend on State delivery capacity because health is delivered by the States; the answer consequence is that a national financing announcement is a necessary but not sufficient condition, and that a graded verdict on any national health programme must be conditioned on the delivery capacity of the State in question.

Answer: D. Explanation: The owners record that the health mission, free drugs and diagnostics initiatives and the health-infrastructure mission strengthen systems while uneven State capacity and workforce vacancies limit outcomes, and that Union financing and standards depend on State delivery capacity because health is delivered by the States; the answer consequence is that a national financing announcement is a necessary but not sufficient condition, and that a graded verdict on any national health programme must be conditioned on the delivery capacity of the State in question. The remaining options belong to different chronology, actor or analytical categories.

Q69. Which statement correctly identifies Mental health as a rights framework with a capacity gap?

A. The Mental Healthcare Act, 2017 supplies a rights framework and the national tele-mental-health service expands access, but the owners record expressly that neither replaces district-level professionals and continuity of care; this is the folder's standard illustration that a statutory right and a technology channel together still leave a human-resource gap, and it is directly usable in any answer that asks whether a legal entitlement has been converted into a service. B. The health mission is a centrally sponsored scheme with Centre-State cost sharing that supports accredited social health activists and auxiliary nurse midwives, free drugs and diagnostics, mobile medical units and maternal-health support of the kind delivered through the maternity protection scheme, while State health agencies implement the hospitalisation pillar at State level and may add State-funded top-up schemes; naming this two-layer arrangement is what allows an answer to explain why identical national schemes produce unequal outcomes across States. C. The owners record that cumulative card counts and cumulative claim counts are dashboard outputs and are not by themselves a measure of service quality or of financial protection, so this package asserts no card total, claim total, empanelment count, wellness-centre count, expenditure share beyond the dated national health accounts sequence, or state-level rank. On ownership, four General Studies Mains demands are routed to this topic in the audited ledgers, from 2018, 2020, 2021 and 2024, with the 2020 geriatric and maternal-care demand recorded as cross-cutting ownership shared with the senior-citizens topic, and three objective demands are routed, namely the 2023 maternal-health scheme question and the 2023 public-health-system question whose official keys the ledgers record as unavailable locally, and the 2024 maternal-health scheme question for which the ledger records that the official Set-A key is available locally but expressly states that no answer is recorded or inferred; this package therefore records no option for any of the three. D. This topic owns the health half of the capability argument, namely the universal-health-coverage standard, the policy vision, the mission that funds public delivery, the two pillars of the flagship health programme, the tiered public infrastructure and the financial-protection measure by which the whole system is judged, and its distinctive feature is the densest direct-question record in the folder: four General Studies Mains demands from 2018, 2020, 2021 and 2024 and three objective demands from 2023 and 2024 are routed to it in the audited ledgers; the macro economics of health financing belongs to the Economy owner, the elderly-care share of the 2020 demand is jointly owned with the senior-citizens topic, and this topic argues coverage, provision and protection.

Answer: A. Explanation: The Mental Healthcare Act, 2017 supplies a rights framework and the national tele-mental-health service expands access, but the owners record expressly that neither replaces district-level professionals and continuity of care; this is the folder's standard illustration that a statutory right and a technology channel together still leave a human-resource gap, and it is directly usable in any answer that asks whether a legal entitlement has been converted into a service. The remaining options

belong to different chronology, actor or analytical categories.

Q70. Which chronology card should be filed under Mental health as a rights framework with a capacity gap?

A. The owners record that cumulative card counts and cumulative claim counts are dashboard outputs and are not by themselves a measure of service quality or of financial protection, so this package asserts no card total, claim total, empanelment count, wellness-centre count, expenditure share beyond the dated national health accounts sequence, or state-level rank. On ownership, four General Studies Mains demands are routed to this topic in the audited ledgers, from 2018, 2020, 2021 and 2024, with the 2020 geriatric and maternal-care demand recorded as cross-cutting ownership shared with the senior-citizens topic, and three objective demands are routed, namely the 2023 maternal-health scheme question and the 2023 public-health-system question whose official keys the ledgers record as unavailable locally, and the 2024 maternal-health scheme question for which the ledger records that the official Set-A key is available locally but expressly states that no answer is recorded or inferred; this package therefore records no option for any of the three. B. The Mental Healthcare Act, 2017 supplies a rights framework and the national tele-mental-health service expands access, but the owners record expressly that neither replaces district-level professionals and continuity of care; this is the folder's standard illustration that a statutory right and a technology channel together still leave a human-resource gap, and it is directly usable in any answer that asks whether a legal entitlement has been converted into a service. C. This topic owns the health half of the capability argument, namely the universal-health-coverage standard, the policy vision, the mission that funds public delivery, the two pillars of the flagship health programme, the tiered public infrastructure and the financial-protection measure by which the whole system is judged, and its distinctive feature is the densest direct-question record in the folder: four General Studies Mains demands from 2018, 2020, 2021 and 2024 and three objective demands from 2023 and 2024 are routed to it in the audited ledgers; the macro economics of health financing belongs to the Economy owner, the elderly-care share of the 2020 demand is jointly owned with the senior-citizens topic, and this topic argues coverage, provision and protection. D. Universal health coverage means ensuring that all people have access to the promotive, preventive, curative and rehabilitative health services they need, of sufficient quality, without suffering financial hardship, and it is analysed on three dimensions that the World Health Organization presents as a cube: population, which is the breadth of who is covered, services, which is the depth of what is covered, and financial protection, which is the height of how little the user pays at the point of service; the analytical value of the cube is that a system can expand on one dimension while contracting on another, so an answer that reports only beneficiary numbers has measured one edge of a three-dimensional claim.

Answer: B. Explanation: The Mental Healthcare Act, 2017 supplies a rights framework and the national tele-mental-health service expands access, but the owners record expressly that neither replaces district-level professionals and continuity of care; this is the folder's standard illustration that a statutory right and a technology channel together still leave a human-resource gap, and it is directly usable in any answer that asks whether a legal entitlement has been converted into a service. The remaining options belong to different chronology, actor or analytical categories.

Q71. Which option preserves the source-bounded meaning of Mental health as a rights framework with a capacity gap?

A. The National Health Policy, 2017 is India's guiding health-policy document, and the owners record its goals as raising public health expenditure to two and a half per cent of gross domestic product, strengthening primary care, achieving universal health coverage and reducing out-of-pocket expenditure; the examinable discipline is that it is a policy document and not an enacted statute, so its targets are commitments rather than justiciable entitlements, and describing them as legally binding is a status error of exactly the kind this folder is built to prevent. B. Universal health coverage means ensuring that all people have access to the promotive, preventive, curative and rehabilitative health services they need, of sufficient quality, without suffering financial hardship, and it is analysed on three dimensions that the World Health Organization presents as a cube: population, which is the breadth of who is covered, services, which is the depth of what is covered, and financial protection, which is the height of how little the user pays at the point of service; the analytical value of the cube is that a system can expand on one dimension while contracting on another, so an answer that reports only beneficiary numbers has measured one edge of a three-dimensional claim. C. The Mental Healthcare Act, 2017 supplies a rights framework and the national tele-mental-health service expands access, but the owners record expressly that neither replaces district-level professionals and continuity of care; this is

the folder's standard illustration that a statutory right and a technology channel together still leave a human-resource gap, and it is directly usable in any answer that asks whether a legal entitlement has been converted into a service. D. This topic owns the health half of the capability argument, namely the universal-health-coverage standard, the policy vision, the mission that funds public delivery, the two pillars of the flagship health programme, the tiered public infrastructure and the financial-protection measure by which the whole system is judged, and its distinctive feature is the densest direct-question record in the folder: four General Studies Mains demands from 2018, 2020, 2021 and 2024 and three objective demands from 2023 and 2024 are routed to it in the audited ledgers; the macro economics of health financing belongs to the Economy owner, the elderly-care share of the 2020 demand is jointly owned with the senior-citizens topic, and this topic argues coverage, provision and protection.

Answer: C. Explanation: The Mental Healthcare Act, 2017 supplies a rights framework and the national tele-mental-health service expands access, but the owners record expressly that neither replaces district-level professionals and continuity of care; this is the folder's standard illustration that a statutory right and a technology channel together still leave a human-resource gap, and it is directly usable in any answer that asks whether a legal entitlement has been converted into a service. The remaining options belong to different chronology, actor or analytical categories.

Q72. Which statement avoids a close-option trap about Mental health as a rights framework with a capacity gap?

A. The National Health Policy, 2017 is India's guiding health-policy document, and the owners record its goals as raising public health expenditure to two and a half per cent of gross domestic product, strengthening primary care, achieving universal health coverage and reducing out-of-pocket expenditure; the examinable discipline is that it is a policy document and not an enacted statute, so its targets are commitments rather than justiciable entitlements, and describing them as legally binding is a status error of exactly the kind this folder is built to prevent. B. The National Health Mission's own official page, read on 2 September 2026, records that the National Rural Health Mission was launched by the Prime Minister on 12 April 2005 to provide accessible, affordable and quality health care to the rural population and especially to vulnerable groups, and that the Union Cabinet, by its decision dated 1 May 2013, approved the launch of the National Urban Health Mission as a sub-mission of an over-arching National Health Mission with the National Rural Health Mission being the other sub-mission; the same page records special focus on the Empowered Action Group States, the North Eastern States, Jammu and Kashmir and Himachal Pradesh, a thrust on a fully functional, community-owned, decentralised health delivery system with inter-sectoral convergence acting on determinants such as water, sanitation, education, nutrition and social and gender equality, and measurement of facilities against the Indian Public Health Standards, so the mission is urban as well as rural and treating it as rural-only is a factual error. C. Universal health coverage means ensuring that all people have access to the promotive, preventive, curative and rehabilitative health services they need, of sufficient quality, without suffering financial hardship, and it is analysed on three dimensions that the World Health Organization presents as a cube: population, which is the breadth of who is covered, services, which is the depth of what is covered, and financial protection, which is the height of how little the user pays at the point of service; the analytical value of the cube is that a system can expand on one dimension while contracting on another, so an answer that reports only beneficiary numbers has measured one edge of a three-dimensional claim. D. The Mental Healthcare Act, 2017 supplies a rights framework and the national tele-mental-health service expands access, but the owners record expressly that neither replaces district-level professionals and continuity of care; this is the folder's standard illustration that a statutory right and a technology channel together still leave a human-resource gap, and it is directly usable in any answer that asks whether a legal entitlement has been converted into a service.

Answer: D. Explanation: The Mental Healthcare Act, 2017 supplies a rights framework and the national tele-mental-health service expands access, but the owners record expressly that neither replaces district-level professionals and continuity of care; this is the folder's standard illustration that a statutory right and a technology channel together still leave a human-resource gap, and it is directly usable in any answer that asks whether a legal entitlement has been converted into a service. The remaining options belong to different chronology, actor or analytical categories.

Q73. Which statement correctly identifies Delivery instruments and the State implementing layer?

A. The health mission is a centrally sponsored scheme with Centre-State cost sharing that supports accredited social health activists and auxiliary nurse midwives, free drugs and diagnostics, mobile medical units and maternal-health support of the kind delivered through the maternity protection scheme, while State health agencies implement the hospitalisation pillar at State level and may add State-funded top-up schemes; naming this two-layer arrangement is what allows an answer to explain why identical national schemes produce unequal outcomes across States. B. The owners record that cumulative card counts and cumulative claim counts are dashboard outputs and are not by themselves a measure of service quality or of financial protection, so this package asserts no card total, claim total, empanelment count, wellness-centre count, expenditure share beyond the dated national health accounts sequence, or state-level rank. On ownership, four General Studies Mains demands are routed to this topic in the audited ledgers, from 2018, 2020, 2021 and 2024, with the 2020 geriatric and maternal-care demand recorded as cross-cutting ownership shared with the senior-citizens topic, and three objective demands are routed, namely the 2023 maternal-health scheme question and the 2023 public-health-system question whose official keys the ledgers record as unavailable locally, and the 2024 maternal-health scheme question for which the ledger records that the official Set-A key is available locally but expressly states that no answer is recorded or inferred; this package therefore records no option for any of the three. C. Universal health coverage means ensuring that all people have access to the promotive, preventive, curative and rehabilitative health services they need, of sufficient quality, without suffering financial hardship, and it is analysed on three dimensions that the World Health Organization presents as a cube: population, which is the breadth of who is covered, services, which is the depth of what is covered, and financial protection, which is the height of how little the user pays at the point of service; the analytical value of the cube is that a system can expand on one dimension while contracting on another, so an answer that reports only beneficiary numbers has measured one edge of a three-dimensional claim. D. This topic owns the health half of the capability argument, namely the universal-health-coverage standard, the policy vision, the mission that funds public delivery, the two pillars of the flagship health programme, the tiered public infrastructure and the financial-protection measure by which the whole system is judged, and its distinctive feature is the densest direct-question record in the folder: four General Studies Mains demands from 2018, 2020, 2021 and 2024 and three objective demands from 2023 and 2024 are routed to it in the audited ledgers; the macro economics of health financing belongs to the Economy owner, the elderly-care share of the 2020 demand is jointly owned with the senior-citizens topic, and this topic argues coverage, provision and protection.

Answer: A. Explanation: The health mission is a centrally sponsored scheme with Centre-State cost sharing that supports accredited social health activists and auxiliary nurse midwives, free drugs and diagnostics, mobile medical units and maternal-health support of the kind delivered through the maternity protection scheme, while State health agencies implement the hospitalisation pillar at State level and may add State-funded top-up schemes; naming this two-layer arrangement is what allows an answer to explain why identical national schemes produce unequal outcomes across States. The remaining options belong to different chronology, actor or analytical categories.

Q74. Which chronology card should be filed under Delivery instruments and the State implementing layer?

A. Universal health coverage means ensuring that all people have access to the promotive, preventive, curative and rehabilitative health services they need, of sufficient quality, without suffering financial hardship, and it is analysed on three dimensions that the World Health Organization presents as a cube: population, which is the breadth of who is covered, services, which is the depth of what is covered, and financial protection, which is the height of how little the user pays at the point of service; the analytical value of the cube is that a system can expand on one dimension while contracting on another, so an answer that reports only beneficiary numbers has measured one edge of a three-dimensional claim. B. The health mission is a centrally sponsored scheme with Centre-State cost sharing that supports accredited social health activists and auxiliary nurse midwives, free drugs and diagnostics, mobile medical units and maternal-health support of the kind delivered through the maternity protection scheme, while State health agencies implement the hospitalisation pillar at State level and may add State-funded top-up schemes; naming this two-layer arrangement is what allows an answer to explain why identical national schemes produce unequal outcomes across States. C. The National Health Policy, 2017 is India's guiding health-policy document, and the owners record its goals as raising public health expenditure to two and a half per cent of gross domestic product, strengthening primary care, achieving universal health coverage and reducing out-of-pocket expenditure; the examinable discipline

is that it is a policy document and not an enacted statute, so its targets are commitments rather than justiciable entitlements, and describing them as legally binding is a status error of exactly the kind this folder is built to prevent. D. This topic owns the health half of the capability argument, namely the universal-health-coverage standard, the policy vision, the mission that funds public delivery, the two pillars of the flagship health programme, the tiered public infrastructure and the financial-protection measure by which the whole system is judged, and its distinctive feature is the densest direct-question record in the folder: four General Studies Mains demands from 2018, 2020, 2021 and 2024 and three objective demands from 2023 and 2024 are routed to it in the audited ledgers; the macro economics of health financing belongs to the Economy owner, the elderly-care share of the 2020 demand is jointly owned with the senior-citizens topic, and this topic argues coverage, provision and protection.

Answer: B. Explanation: The health mission is a centrally sponsored scheme with Centre-State cost sharing that supports accredited social health activists and auxiliary nurse midwives, free drugs and diagnostics, mobile medical units and maternal-health support of the kind delivered through the maternity protection scheme, while State health agencies implement the hospitalisation pillar at State level and may add State-funded top-up schemes; naming this two-layer arrangement is what allows an answer to explain why identical national schemes produce unequal outcomes across States. The remaining options belong to different chronology, actor or analytical categories.

Q75. Which option preserves the source-bounded meaning of Delivery instruments and the State implementing layer?

A. The National Health Mission's own official page, read on 2 September 2026, records that the National Rural Health Mission was launched by the Prime Minister on 12 April 2005 to provide accessible, affordable and quality health care to the rural population and especially to vulnerable groups, and that the Union Cabinet, by its decision dated 1 May 2013, approved the launch of the National Urban Health Mission as a sub-mission of an over-arching National Health Mission with the National Rural Health Mission being the other sub-mission; the same page records special focus on the Empowered Action Group States, the North Eastern States, Jammu and Kashmir and Himachal Pradesh, a thrust on a fully functional, community-owned, decentralised health delivery system with inter-sectoral convergence acting on determinants such as water, sanitation, education, nutrition and social and gender equality, and measurement of facilities against the Indian Public Health Standards, so the mission is urban as well as rural and treating it as rural-only is a factual error. B. The National Health Policy, 2017 is India's guiding health-policy document, and the owners record its goals as raising public health expenditure to two and a half per cent of gross domestic product, strengthening primary care, achieving universal health coverage and reducing out-of-pocket expenditure; the examinable discipline is that it is a policy document and not an enacted statute, so its targets are commitments rather than justiciable entitlements, and describing them as legally binding is a status error of exactly the kind this folder is built to prevent. C. The health mission is a centrally sponsored scheme with Centre-State cost sharing that supports accredited social health activists and auxiliary nurse midwives, free drugs and diagnostics, mobile medical units and maternal-health support of the kind delivered through the maternity protection scheme, while State health agencies implement the hospitalisation pillar at State level and may add State-funded top-up schemes; naming this two-layer arrangement is what allows an answer to explain why identical national schemes produce unequal outcomes across States. D. Universal health coverage means ensuring that all people have access to the promotive, preventive, curative and rehabilitative health services they need, of sufficient quality, without suffering financial hardship, and it is analysed on three dimensions that the World Health Organization presents as a cube: population, which is the breadth of who is covered, services, which is the depth of what is covered, and financial protection, which is the height of how little the user pays at the point of service; the analytical value of the cube is that a system can expand on one dimension while contracting on another, so an answer that reports only beneficiary numbers has measured one edge of a three-dimensional claim.

Answer: C. Explanation: The health mission is a centrally sponsored scheme with Centre-State cost sharing that supports accredited social health activists and auxiliary nurse midwives, free drugs and diagnostics, mobile medical units and maternal-health support of the kind delivered through the maternity protection scheme, while State health agencies implement the hospitalisation pillar at State level and may add State-funded top-up schemes; naming this two-layer arrangement is what allows an answer to explain why identical national schemes produce unequal outcomes across States. The remaining options belong to different chronology, actor or analytical categories.

Q76. Which statement avoids a close-option trap about Delivery instruments and the State implementing layer?

A. The National Health Policy, 2017 is India's guiding health-policy document, and the owners record its goals as raising public health expenditure to two and a half per cent of gross domestic product, strengthening primary care, achieving universal health coverage and reducing out-of-pocket expenditure; the examinable discipline is that it is a policy document and not an enacted statute, so its targets are commitments rather than justiciable entitlements, and describing them as legally binding is a status error of exactly the kind this folder is built to prevent. B. The first pillar of Ayushman Bharat upgrades sub-centres and primary health centres into Health and Wellness Centres, now carried as Ayushman Arogya Mandirs, which deliver comprehensive primary health care through twelve service packages that extend beyond reproductive and child health to non-communicable diseases, mental health, ear, nose and throat care, ophthalmology, oral health, geriatric and palliative care and emergency care, staffed by a community health officer or mid-level health provider; the design intent is to shift care-seeking from hospitals to a local first-contact layer, which is why describing this pillar as reproductive and child health alone understates it. C. The National Health Mission's own official page, read on 2 September 2026, records that the National Rural Health Mission was launched by the Prime Minister on 12 April 2005 to provide accessible, affordable and quality health care to the rural population and especially to vulnerable groups, and that the Union Cabinet, by its decision dated 1 May 2013, approved the launch of the National Urban Health Mission as a sub-mission of an over-arching National Health Mission with the National Rural Health Mission being the other sub-mission; the same page records special focus on the Empowered Action Group States, the North Eastern States, Jammu and Kashmir and Himachal Pradesh, a thrust on a fully functional, community-owned, decentralised health delivery system with inter-sectoral convergence acting on determinants such as water, sanitation, education, nutrition and social and gender equality, and measurement of facilities against the Indian Public Health Standards, so the mission is urban as well as rural and treating it as rural-only is a factual error. D. The health mission is a centrally sponsored scheme with Centre-State cost sharing that supports accredited social health activists and auxiliary nurse midwives, free drugs and diagnostics, mobile medical units and maternal-health support of the kind delivered through the maternity protection scheme, while State health agencies implement the hospitalisation pillar at State level and may add State-funded top-up schemes; naming this two-layer arrangement is what allows an answer to explain why identical national schemes produce unequal outcomes across States.

Answer: D. Explanation: The health mission is a centrally sponsored scheme with Centre-State cost sharing that supports accredited social health activists and auxiliary nurse midwives, free drugs and diagnostics, mobile medical units and maternal-health support of the kind delivered through the maternity protection scheme, while State health agencies implement the hospitalisation pillar at State level and may add State-funded top-up schemes; naming this two-layer arrangement is what allows an answer to explain why identical national schemes produce unequal outcomes across States. The remaining options belong to different chronology, actor or analytical categories.

Q77. Which statement correctly identifies The dashboard-output boundary and honest question ownership?

A. The owners record that cumulative card counts and cumulative claim counts are dashboard outputs and are not by themselves a measure of service quality or of financial protection, so this package asserts no card total, claim total, empanelment count, wellness-centre count, expenditure share beyond the dated national health accounts sequence, or state-level rank. On ownership, four General Studies Mains demands are routed to this topic in the audited ledgers, from 2018, 2020, 2021 and 2024, with the 2020 geriatric and maternal-care demand recorded as cross-cutting ownership shared with the senior-citizens topic, and three objective demands are routed, namely the 2023 maternal-health scheme question and the 2023 public-health-system question whose official keys the ledgers record as unavailable locally, and the 2024 maternal-health scheme question for which the ledger records that the official Set-A key is available locally but expressly states that no answer is recorded or inferred; this package therefore records no option for any of the three. B. The National Health Policy, 2017 is India's guiding health-policy document, and the owners record its goals as raising public health expenditure to two and a half per cent of gross domestic product, strengthening primary care, achieving universal health coverage and reducing out-of-pocket expenditure; the examinable discipline is that it is a policy document and not an enacted statute, so its targets are commitments rather than justiciable entitlements, and describing them as legally binding is a status error of exactly the kind this folder is built to prevent. C. This topic owns the health half of the capability argument, namely the universal-health-coverage standard, the policy vision, the mission that funds public delivery, the two pillars of the flagship health programme, the

tiered public infrastructure and the financial-protection measure by which the whole system is judged, and its distinctive feature is the densest direct-question record in the folder: four General Studies Mains demands from 2018, 2020, 2021 and 2024 and three objective demands from 2023 and 2024 are routed to it in the audited ledgers; the macro economics of health financing belongs to the Economy owner, the elderly-care share of the 2020 demand is jointly owned with the senior-citizens topic, and this topic argues coverage, provision and protection. D. Universal health coverage means ensuring that all people have access to the promotive, preventive, curative and rehabilitative health services they need, of sufficient quality, without suffering financial hardship, and it is analysed on three dimensions that the World Health Organization presents as a cube: population, which is the breadth of who is covered, services, which is the depth of what is covered, and financial protection, which is the height of how little the user pays at the point of service; the analytical value of the cube is that a system can expand on one dimension while contracting on another, so an answer that reports only beneficiary numbers has measured one edge of a three-dimensional claim.

Answer: A. Explanation: The owners record that cumulative card counts and cumulative claim counts are dashboard outputs and are not by themselves a measure of service quality or of financial protection, so this package asserts no card total, claim total, empanelment count, wellness-centre count, expenditure share beyond the dated national health accounts sequence, or state-level rank. On ownership, four General Studies Mains demands are routed to this topic in the audited ledgers, from 2018, 2020, 2021 and 2024, with the 2020 geriatric and maternal-care demand recorded as cross-cutting ownership shared with the senior-citizens topic, and three objective demands are routed, namely the 2023 maternal-health scheme question and the 2023 public-health-system question whose official keys the ledgers record as unavailable locally, and the 2024 maternal-health scheme question for which the ledger records that the official Set-A key is available locally but expressly states that no answer is recorded or inferred; this package therefore records no option for any of the three. The remaining options belong to different chronology, actor or analytical categories.

Q78. Which chronology card should be filed under The dashboard-output boundary and honest question ownership?

A. The National Health Policy, 2017 is India's guiding health-policy document, and the owners record its goals as raising public health expenditure to two and a half per cent of gross domestic product, strengthening primary care, achieving universal health coverage and reducing out-of-pocket expenditure; the examinable discipline is that it is a policy document and not an enacted statute, so its targets are commitments rather than justiciable entitlements, and describing them as legally binding is a status error of exactly the kind this folder is built to prevent. B. The owners record that cumulative card counts and cumulative claim counts are dashboard outputs and are not by themselves a measure of service quality or of financial protection, so this package asserts no card total, claim total, empanelment count, wellness-centre count, expenditure share beyond the dated national health accounts sequence, or state-level rank. On ownership, four General Studies Mains demands are routed to this topic in the audited ledgers, from 2018, 2020, 2021 and 2024, with the 2020 geriatric and maternal-care demand recorded as cross-cutting ownership shared with the senior-citizens topic, and three objective demands are routed, namely the 2023 maternal-health scheme question and the 2023 public-health-system question whose official keys the ledgers record as unavailable locally, and the 2024 maternal-health scheme question for which the ledger records that the official Set-A key is available locally but expressly states that no answer is recorded or inferred; this package therefore records no option for any of the three. C. The National Health Mission's own official page, read on 2 September 2026, records that the National Rural Health Mission was launched by the Prime Minister on 12 April 2005 to provide accessible, affordable and quality health care to the rural population and especially to vulnerable groups, and that the Union Cabinet, by its decision dated 1 May 2013, approved the launch of the National Urban Health Mission as a sub-mission of an over-arching National Health Mission with the National Rural Health Mission being the other sub-mission; the same page records special focus on the Empowered Action Group States, the North Eastern States, Jammu and Kashmir and Himachal Pradesh, a thrust on a fully functional, community-owned, decentralised health delivery system with inter-sectoral convergence acting on determinants such as water, sanitation, education, nutrition and social and gender equality, and measurement of facilities against the Indian Public Health Standards, so the mission is urban as well as rural and treating it as rural-only is a factual error. D. Universal health coverage means ensuring that all people have access to the promotive, preventive, curative and rehabilitative health services they need, of sufficient quality, without suffering financial hardship, and it is analysed on three dimensions that the World Health Organization presents as a cube: population, which is the breadth of who is covered, services, which is the depth of what is

covered, and financial protection, which is the height of how little the user pays at the point of service; the analytical value of the cube is that a system can expand on one dimension while contracting on another, so an answer that reports only beneficiary numbers has measured one edge of a three-dimensional claim.

Answer: B. Explanation: The owners record that cumulative card counts and cumulative claim counts are dashboard outputs and are not by themselves a measure of service quality or of financial protection, so this package asserts no card total, claim total, empanelment count, wellness-centre count, expenditure share beyond the dated national health accounts sequence, or state-level rank. On ownership, four General Studies Mains demands are routed to this topic in the audited ledgers, from 2018, 2020, 2021 and 2024, with the 2020 geriatric and maternal-care demand recorded as cross-cutting ownership shared with the senior-citizens topic, and three objective demands are routed, namely the 2023 maternal-health scheme question and the 2023 public-health-system question whose official keys the ledgers record as unavailable locally, and the 2024 maternal-health scheme question for which the ledger records that the official Set-A key is available locally but expressly states that no answer is recorded or inferred; this package therefore records no option for any of the three. The remaining options belong to different chronology, actor or analytical categories.

Q79. Which option preserves the source-bounded meaning of The dashboard-output boundary and honest question ownership?

A. The first pillar of Ayushman Bharat upgrades sub-centres and primary health centres into Health and Wellness Centres, now carried as Ayushman Arogya Mandirs, which deliver comprehensive primary health care through twelve service packages that extend beyond reproductive and child health to non-communicable diseases, mental health, ear, nose and throat care, ophthalmology, oral health, geriatric and palliative care and emergency care, staffed by a community health officer or mid-level health provider; the design intent is to shift care-seeking from hospitals to a local first-contact layer, which is why describing this pillar as reproductive and child health alone understates it. B. The National Health Policy, 2017 is India's guiding health-policy document, and the owners record its goals as raising public health expenditure to two and a half per cent of gross domestic product, strengthening primary care, achieving universal health coverage and reducing out-of-pocket expenditure; the examinable discipline is that it is a policy document and not an enacted statute, so its targets are commitments rather than justiciable entitlements, and describing them as legally binding is a status error of exactly the kind this folder is built to prevent. C. The owners record that cumulative card counts and cumulative claim counts are dashboard outputs and are not by themselves a measure of service quality or of financial protection, so this package asserts no card total, claim total, empanelment count, wellness-centre count, expenditure share beyond the dated national health accounts sequence, or state-level rank. On ownership, four General Studies Mains demands are routed to this topic in the audited ledgers, from 2018, 2020, 2021 and 2024, with the 2020 geriatric and maternal-care demand recorded as cross-cutting ownership shared with the senior-citizens topic, and three objective demands are routed, namely the 2023 maternal-health scheme question and the 2023 public-health-system question whose official keys the ledgers record as unavailable locally, and the 2024 maternal-health scheme question for which the ledger records that the official Set-A key is available locally but expressly states that no answer is recorded or inferred; this package therefore records no option for any of the three. D. The National Health Mission's own official page, read on 2 September 2026, records that the National Rural Health Mission was launched by the Prime Minister on 12 April 2005 to provide accessible, affordable and quality health care to the rural population and especially to vulnerable groups, and that the Union Cabinet, by its decision dated 1 May 2013, approved the launch of the National Urban Health Mission as a sub-mission of an over-arching National Health Mission with the National Rural Health Mission being the other sub-mission; the same page records special focus on the Empowered Action Group States, the North Eastern States, Jammu and Kashmir and Himachal Pradesh, a thrust on a fully functional, community-owned, decentralised health delivery system with inter-sectoral convergence acting on determinants such as water, sanitation, education, nutrition and social and gender equality, and measurement of facilities against the Indian Public Health Standards, so the mission is urban as well as rural and treating it as rural-only is a factual error.

Answer: C. Explanation: The owners record that cumulative card counts and cumulative claim counts are dashboard outputs and are not by themselves a measure of service quality or of financial protection, so this package asserts no card total, claim total, empanelment count, wellness-centre count, expenditure share beyond the dated national health accounts sequence, or state-level rank. On ownership, four General Studies Mains demands are routed to this topic in the audited ledgers, from 2018, 2020, 2021 and 2024, with the 2020 geriatric and maternal-care demand recorded as cross-cutting ownership shared with the senior-citizens

topic, and three objective demands are routed, namely the 2023 maternal-health scheme question and the 2023 public-health-system question whose official keys the ledgers record as unavailable locally, and the 2024 maternal-health scheme question for which the ledger records that the official Set-A key is available locally but expressly states that no answer is recorded or inferred; this package therefore records no option for any of the three. The remaining options belong to different chronology, actor or analytical categories.

Q80. Which statement avoids a close-option trap about The dashboard-output boundary and honest question ownership?

A. The second pillar, Pradhan Mantri Jan Arogya Yojana, provides cashless secondary and tertiary hospitalisation cover of up to five lakh rupees per family per year at empanelled public and private hospitals for its core eligible households, which are identified through the deprivation criteria of the socio-economic and caste census, and the owners separately record a distinct expansion under which all citizens aged seventy years and above can enrol regardless of income or that census status; the discipline required is exact, because the core scheme is eligibility-based while the age-based expansion is universal only for the seventy-plus group, so an age-specific expansion must never be written up as universal coverage for every age. B. The first pillar of Ayushman Bharat upgrades sub-centres and primary health centres into Health and Wellness Centres, now carried as Ayushman Arogya Mandirs, which deliver comprehensive primary health care through twelve service packages that extend beyond reproductive and child health to non-communicable diseases, mental health, ear, nose and throat care, ophthalmology, oral health, geriatric and palliative care and emergency care, staffed by a community health officer or mid-level health provider; the design intent is to shift care-seeking from hospitals to a local first-contact layer, which is why describing this pillar as reproductive and child health alone understates it. C. The National Health Mission's own official page, read on 2 September 2026, records that the National Rural Health Mission was launched by the Prime Minister on 12 April 2005 to provide accessible, affordable and quality health care to the rural population and especially to vulnerable groups, and that the Union Cabinet, by its decision dated 1 May 2013, approved the launch of the National Urban Health Mission as a sub-mission of an over-arching National Health Mission with the National Rural Health Mission being the other sub-mission; the same page records special focus on the Empowered Action Group States, the North Eastern States, Jammu and Kashmir and Himachal Pradesh, a thrust on a fully functional, community-owned, decentralised health delivery system with inter-sectoral convergence acting on determinants such as water, sanitation, education, nutrition and social and gender equality, and measurement of facilities against the Indian Public Health Standards, so the mission is urban as well as rural and treating it as rural-only is a factual error. D. The owners record that cumulative card counts and cumulative claim counts are dashboard outputs and are not by themselves a measure of service quality or of financial protection, so this package asserts no card total, claim total, empanelment count, wellness-centre count, expenditure share beyond the dated national health accounts sequence, or state-level rank. On ownership, four General Studies Mains demands are routed to this topic in the audited ledgers, from 2018, 2020, 2021 and 2024, with the 2020 geriatric and maternal-care demand recorded as cross-cutting ownership shared with the senior-citizens topic, and three objective demands are routed, namely the 2023 maternal-health scheme question and the 2023 public-health-system question whose official keys the ledgers record as unavailable locally, and the 2024 maternal-health scheme question for which the ledger records that the official Set-A key is available locally but expressly states that no answer is recorded or inferred; this package therefore records no option for any of the three.

Answer: D. Explanation: The owners record that cumulative card counts and cumulative claim counts are dashboard outputs and are not by themselves a measure of service quality or of financial protection, so this package asserts no card total, claim total, empanelment count, wellness-centre count, expenditure share beyond the dated national health accounts sequence, or state-level rank. On ownership, four General Studies Mains demands are routed to this topic in the audited ledgers, from 2018, 2020, 2021 and 2024, with the 2020 geriatric and maternal-care demand recorded as cross-cutting ownership shared with the senior-citizens topic, and three objective demands are routed, namely the 2023 maternal-health scheme question and the 2023 public-health-system question whose official keys the ledgers record as unavailable locally, and the 2024 maternal-health scheme question for which the ledger records that the official Set-A key is available locally but expressly states that no answer is recorded or inferred; this package therefore records no option for any of the three. The remaining options belong to different chronology, actor or analytical categories.

PYQS AND ANSWER PRACTICE

VERIFIED PYQ OWNERSHIP AUDIT

Four General Studies Mains demands and three objective demands are routed to this topic in the audited routing ledgers, and each Mains demand is reproduced below as a demand card with its printed year, paper, question number, directive, marks and word limit exactly as the ledgers record them: 2018 General Studies Paper II Question 7, 2020 General Studies Paper II Question 6, 2021 General Studies Paper II Question 6 and 2024 General Studies Paper II Question 17. The 2020 demand is recorded in the ledger as cross-cutting Core ownership shared with the senior-citizens topic, and that shared ownership is stated openly here rather than being claimed exclusively. Three objective demands are also routed, namely 2023 Prelims General Studies Paper I Question 51 on the maternal and neonatal mortality and institutional-delivery scheme, 2023 Prelims General Studies Paper I Question 75 on the curative, preventive and decentralised character of the Indian public health system, and 2024 Prelims General Studies Paper I Question 99 on the maternal-health abhiyan; the ledger records the official 2018-2023 Prelims keys as not held locally, and for the 2024 question it records that the official Set-A key is available locally while expressly stating that no answer is recorded or inferred, so this package records no option, key or inferred answer for any of the three and converts none of them into a solved question. The locally held OCR-searchable official General Studies papers were read only to confirm the printed wording of the routed Mains demands; no question was invented from them and no marking scheme or page precision was imported. Every model solution below is original work written to the repository's examiner-grade standard using the claim, named evidence, analysis and qualification pattern.

Demand decoding: The directive **answer** requires a direct position on "VERIFIED PYQ OWNERSHIP AUDIT", every clause, constitutional/statutory/policy distinction, rights-holder and institution map, eligibility/exclusion and delivery chain, intersectional and regional variation, named Indian evidence, accountability and qualified conclusion.

Detailed examiner-grade model answer:

Introduction and thesis: The answer must resolve the Social Justice demand in "VERIFIED PYQ OWNERSHIP AUDIT".

Analytical body:

- 1. Claim and named evidence:** Define the vulnerable group, deprivation, right or policy concept and distinguish the nearest legal and measured category. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction.
Qualification: State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
- 2. Claim and named evidence:** Identify the constitutional, statutory, judicial or executive basis and the competent Union, state or local institution. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction.
Qualification: State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
- 3. Claim and named evidence:** Map rights-holder, household/community context, duty-bearer, frontline worker, provider and accountability forum. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction.
Qualification: State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
- 4. Claim and named evidence:** Trace identification, eligibility, documentation, finance, access, portability, grievance, appeal and correction. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction.
Qualification: State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
- 5. Claim and named evidence:** Use a named Indian law, judgment, scheme, institution or official dataset with source, date, denominator and status. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction.
Qualification: State internal group and regional variation, legal/operative status, denominator, fiscal or causal

limit, accessibility, dignity, privacy, portability or residual exclusion.

6. **Claim and named evidence:** Test intersectionality, regional variation, stigma, accessibility, privacy, fiscal adequacy, exclusion and causal limits. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction.

Qualification: State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.

Counter-position / limit: A constitutional promise, prohibition, reservation provision, scheme catalogue, allocation, registration count or headline statistic cannot alone establish accessible implementation, adequate benefit, dignity, remedy or attributable outcome; test eligibility, institutions, frontline capacity, intersectionality, regional variation, grievance and evidence status.

Qualified conclusion: The answer must resolve the Social Justice demand in "VERIFIED PYQ OWNERSHIP AUDIT".

Executable exam-length answer / compression plan: For 15 marks, spend one-sixth of the time decoding the directive and drawing right/need -> institution -> eligibility -> delivery -> outcome -> remedy; state a thesis; write four to seven claim -> named evidence -> analysis -> qualification points; reserve the final minute for dignity, accessibility, portability, grievance, denominator, fiscal and causal limits.

Why this earns marks: The answer obeys the directive, explains mechanisms rather than listing schemes, uses named India-centric evidence and preserves legal, institutional, group, eligibility, implementation, fiscal, data and outcome distinctions.

How to improve this answer: For "VERIFIED PYQ OWNERSHIP AUDIT", replace the weakest scheme-catalogue point with one named rights-holder, duty-bearer, eligibility bottleneck, safeguard, grievance route, process/outcome indicator and source-status qualification.

OWNER PYQ LEDGER EXTRACTS

9. PYQ application

- **FACT: 2024 Q17 (15 marks):** Structure the answer as:

1. Define marketisation and its adverse impacts (high costs, unnecessary procedures, catastrophic OOPe, inequitable access).

2. State the constitutional and policy basis for state intervention (DPSP Art 47, NHP 2017).

3. List measures to enhance grassroots reach:

- Expand HWC/AAM network for comprehensive primary care.
- Strengthen NHM support (free drugs, diagnostics, ASHA/ANM workforce).
- PM-JAY financial protection to shift catastrophic burden away from households.

- Regulatory tools: Clinical Establishments Act enforcement, price capping (e.g., NPPA for devices), standard treatment guidelines.

- Public-private partnerships with accountability safeguards.

4. Conclude with the UHC imperative: state must remain the primary guarantor of health security, not a residual provider.

Recent PYQ Integration (2024-2025)

Status: 2024-2025 question-level PYQ demand is integrated into this owner. **Provenance:** Audited local official-paper routing ledgers: _PYQ-ROUTING-MAINS-GS1-GS2-ESSAY-2024-2025.md, _PYQ-ROUTING-PRELIMS-2024-2025.md. **Answer-key rule:** The official 2024-2025 Prelims Set-A keys are present in the repository and CSAT Set-A keys are supplied; even so, no option or answer is recorded or inferred in this integration.

- **Years represented:** 2024
- **Paper(s):** GS-II, Prelims GS-I
- **Routed question demands:** 2

Year	Paper	Q	PYQ demand (neutral rendering)	Directive / format	Source status	Owner requirement
2024	GS-II	17	State's role against marketisation of public healthcare	Suggest · 15 marks · 250 words	Routed to owning topic	Prepare context, core dimensions, evidence/examples, counterpoint and a concise conclusion.
2024	Prelims GS-I	99	Pradhan Mantri Surakshit Matritva Abhiyan (maternal health)	Objective question; official Set-A key available locally, answer not inferred	Key available locally (official Set-A answer key present); answer not recorded here	Cover the named fact/concept and its likely statement-level distinctions.

What this owner must now support

- State's role against marketisation of public healthcare
- Pradhan Mantri Surakshit Matritva Abhiyan (maternal health)

This block integrates the 2024-2025 examinable demand and paper metadata. It is kept separate from the 2018-2023 block and does not convert an unkeyed/answer-free objective question into a solved answer.

Historical PYQ Integration (2018-2023)

Status: Question-level PYQ demand is integrated into this owner. **Provenance:** Audited local official-paper routing ledgers: _PYQ-ROUTING-MAINS-GS1-GS2-ESSAY-2018-2023.md, _PYQ-ROUTING-PRELIMS-2018-2023.md. **Answer-key rule:** The official 2018-2023 Prelims/CSAT keys are not held locally; no option or answer has been inferred.

- **Years represented:** 2018, 2020, 2021, 2023
- **Paper(s):** GS-II, Prelims GS-I
- **Routed question demands:** 5

Year	Paper	Q	PYQ demand (neutral rendering)	Directive / format	Source status	Owner requirement
2018	GS-II	7	Community-level healthcare intervention and Health for All	Explain · 10 marks · 150 words	Core route supersedes older Advanced ownership	Prepare context, core dimensions, evidence/examples, counterpoint and a concise conclusion.
2020	GS-II	6	Health policies for geriatric and maternal care	Discuss · 10 marks · 150 words	Cross-cutting Core ownership	Prepare context, core dimensions, evidence/examples, counterpoint and a concise conclusion.
2021	GS-II	6	Primary health structure as a precondition for sustainable development	Analyze · 10 marks · 150 words	Core route supersedes older Advanced ownership	Prepare context, core dimensions, evidence/examples, counterpoint and a concise conclusion.
2023	Prelims GS-I	51	Janani Suraksha Yojana maternal neonatal mortality institutional delivery	Objective question; official key unavailable locally	Routed; key unavailable locally	Cover the named fact/concept and its likely statement-level distinctions.
2023	Prelims GS-I	75	India public health system curative preventive decentralized approach	Objective question; official key unavailable locally	Routed; key unavailable locally	Cover the named fact/concept and its likely statement-level distinctions.

What this owner must now support

- Community-level healthcare intervention and Health for All
- Health policies for geriatric and maternal care
- Primary health structure as a precondition for sustainable development
- Janani Suraksha Yojana maternal neonatal mortality institutional delivery
- India public health system curative preventive decentralized approach

The table integrates the examinable demand and paper metadata. It does not turn an unkeyed objective question into a solved answer, and it does not claim that lexical presence alone proves full conceptual sufficiency.

3. Detailed causal chain: marketisation and state response (2024 PYQ Q17)

1. **Public-provision constraint:** NHP 2017 sets a policy aspiration of raising public health expenditure to 2.5% of GDP; an aspiration is not proof that the target has been reached or that state-level primary care is adequately staffed.
2. **Mixed provider landscape:** Where public primary care is unavailable or perceived as low quality, households turn to private providers. The relevant Mains issue is quality, price and regulation, not a single undated private-sector share.
3. **High OOPE:** National Health Accounts reported household OOPE at 47.1% in 2019-20, 44.4% in 2020-21 and 39.4% in 2021-22. The decline is progress, while the remaining share still signals incomplete financial protection.
4. **Catastrophic and impoverishing spending:** The risk is concentrated among households needing medicines, diagnostics or repeated care; cite the survey/study and threshold before asserting a poverty-impact percentage.
5. **State's corrective measures (Q17 answer structure):**
 - Expand HWC/AAM for comprehensive primary care - reduces need for hospital-level care-seeking.
 - Strengthen NHM (free drugs, diagnostics, workforce) - improves public-facility attractiveness.
 - PM-JAY for secondary/tertiary cover - shifts hospitalisation cost from households to public pool.
 - Regulatory enforcement: Clinical Establishments Act, price caps (NPPA for devices/stents), STGs, quality accreditation.
 - Public-private partnerships with accountability: empanel private providers under PM-JAY with rate caps and audit.

10. PYQ-based analytical application

- FACT: **2024 Q17 (15 marks):** A strong answer should:
 1. Define marketisation and its adverse impacts (OOPE, catastrophic expenditure, inequitable access, supplier-induced demand).
 2. Cite the constitutional and policy mandate (Art 47, NHP 2017).
 3. Present a multi-pronged state response:
 - Supply-side: HWC/AAM expansion, NHM strengthening, HR augmentation.
 - Demand-side: PM-JAY financial protection.
 - Regulation: Clinical Establishments Act, price caps, STGs, quality accreditation.
 - Data and accountability: NHA monitoring, social audit, grievance

redress.

4. Conclude with the UHC imperative: state as guarantor, not residual provider.

Historical PYQ Integration (2018-2023)

Status: Question-level PYQ demand is integrated into this owner. **Provenance:** Audited local official-paper routing ledgers: _PYQ-ROUTING-MAINS-GS1-GS2-ESSAY-2018-2023.md.

- **Years represented:** 2018, 2020, 2021
- **Paper(s):** GS-II
- **Routed question demands:** 3

Year	Paper	Q	PYQ demand (neutral rendering)	Directive / format	Source status	Owner requirement
2018	GS-II	7	Community-level healthcare intervention and Health for All	Explain · 10 marks · 150 words	Routed to owning topic	Prepare context, core dimensions, evidence/examples, counterpoint and a concise conclusion.
2020	GS-II	6	Health policies for geriatric and maternal care	Discuss · 10 marks · 150 words	Cross-cutting; the question names two distinct care domains	Prepare context, core dimensions, evidence/examples, counterpoint and a concise conclusion.
2021	GS-II	6	Primary health structure as a precondition for sustainable development	Analyze · 10 marks · 150 words	Routed to owning topic	Prepare context, core dimensions, evidence/examples, counterpoint and a concise conclusion.

What this owner must now support

- Community-level healthcare intervention and Health for All
- Health policies for geriatric and maternal care
- Primary health structure as a precondition for sustainable development

The table integrates the examinable demand and paper metadata. It does not turn an unkeyed objective question into a solved answer, and it does not claim that lexical presence alone proves full conceptual sufficiency.

PYQ DEMAND CARD 1 - 2018 General Studies Paper II Question 7

Demand: Community-level healthcare intervention and Health for All. Explain, 10 marks, 150 words, exactly as recorded in the audited 2018-2023 Mains routing ledger.

Status: Routed to this owner in the audited 2018-2023 Mains routing ledger, where the Core route is recorded as superseding the older Advanced ownership. No official answer key exists for a Mains demand and none is claimed.

Model solution: Claim: health for all is achieved at the community level or not at all, because the determinants that decide health outcomes are produced outside the hospital. Named evidence and example: the Alma-Ata commitments of equity, participation, inter-sectoral action and appropriate technology; their Indian mapping onto accredited social health activists, auxiliary nurse midwives, village health, sanitation and nutrition committees and the upgraded wellness centres; the twelve comprehensive primary health care service packages delivered by a community health officer or mid-level health provider; and the national health mission's own recorded thrust, read on its official page on 2 September 2026, on a fully functional, community-owned, decentralised delivery system with inter-sectoral convergence on water, sanitation, education, nutrition and social and gender equality. Analysis: community-level intervention works because it converts an episodic curative contact into continuity, detects non-communicable disease and undernutrition before they become hospitalisations, and gives the village a participatory claim on the facility, which is why the same national financing produces different outcomes where the frontline layer is staffed and supervised.

Qualification: the community layer is not a substitute for referral capacity, and no cumulative wellness-centre or worker count is asserted here because such counts are dashboard outputs rather than evidence of service quality. Why this earns marks: it explains a mechanism, names Indian institutions against a stated international standard and ends with a bounded claim.

Demand decoding: The directive **answer** requires a direct position on “PYQ DEMAND CARD 1 - 2018 General Studies Paper II Question 7”, every clause, constitutional/statutory/policy distinction, rights-holder and institution map, eligibility/exclusion and delivery chain, intersectional and regional variation, named Indian evidence, accountability and qualified conclusion.

Detailed examiner-grade model answer:

Introduction and thesis: Claim: health for all is achieved at the community level or not at all, because the determinants that decide health outcomes are produced outside the hospital. Named evidence and example: the Alma-Ata commitments of equity, participation, inter-sectoral action and appropriate technology; their Indian mapping onto accredited social health activists, auxiliary nurse midwives, village health, sanitation and nutrition committees and the upgraded wellness centres; the twelve comprehensive primary health care service packages delivered by a community health officer or mid-level health provider; and the national health mission's own recorded thrust, read on its official page on 2 September 2026, on a fully functional, community-owned, decentralised delivery system with inter-sectoral convergence on water, sanitation, education, nutrition and social and gender equality. Analysis: community-level intervention works because it converts an episodic curative contact into continuity, detects non-communicable disease and undernutrition before they become hospitalisations, and gives the village a participatory claim on the facility, which is why the same national financing produces different outcomes where the frontline layer is staffed and supervised. **Qualification:** the community layer is not a substitute for referral capacity, and no cumulative wellness-centre or worker count is asserted here because such counts are dashboard outputs rather than evidence of service quality. Why this earns marks: it explains a mechanism, names Indian institutions against a stated international standard and ends with a bounded claim.

Analytical body:

1. **Claim and named evidence:** PYQ DEMAND CARD 1 - 2018 General Studies Paper II Question 7 **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
2. **Claim and named evidence:** Demand: Community-level healthcare intervention and Health for All. Explain, 10 marks, 150 words, exactly as recorded in the audited 2018-2023 Mains routing ledger. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.

Counter-position / limit: A constitutional promise, prohibition, reservation provision, scheme catalogue, allocation, registration count or headline statistic cannot alone establish accessible implementation, adequate benefit, dignity, remedy or attributable outcome; test eligibility, institutions, frontline capacity, intersectionality, regional variation, grievance and evidence status.

Qualified conclusion: Claim: health for all is achieved at the community level or not at all, because the determinants that decide health outcomes are produced outside the hospital. Named evidence and example: the Alma-Ata commitments of equity, participation, inter-sectoral action and appropriate technology; their Indian mapping onto accredited social health activists, auxiliary nurse midwives, village health, sanitation and nutrition committees and the upgraded wellness centres; the twelve comprehensive primary health care service packages delivered by a community health officer or mid-level health provider; and the national health mission's own recorded thrust, read on its official page on 2 September 2026, on a fully functional, community-owned, decentralised delivery system with inter-sectoral convergence on water, sanitation, education, nutrition and social and gender equality. Analysis: community-level intervention works because it converts an episodic curative contact into continuity, detects non-communicable disease and undernutrition before they become hospitalisations, and gives the village a participatory claim on the facility, which is why the same national financing produces different outcomes where the frontline layer is staffed and supervised. **Qualification:** the community layer is not a substitute for referral capacity, and

no cumulative wellness-centre or worker count is asserted here because such counts are dashboard outputs rather than evidence of service quality. Why this earns marks: it explains a mechanism, names Indian institutions against a stated international standard and ends with a bounded claim.

Executable exam-length answer / compression plan: For 15 marks, spend one-sixth of the time decoding the directive and drawing right/need -> institution -> eligibility -> delivery -> outcome -> remedy; state a thesis; write four to seven claim -> named evidence -> analysis -> qualification points; reserve the final minute for dignity, accessibility, portability, grievance, denominator, fiscal and causal limits.

How to improve this answer: For “PYQ DEMAND CARD 1 - 2018 General Studies Paper II Question 7”, replace the weakest scheme-catalogue point with one named rights-holder, duty-bearer, eligibility bottleneck, safeguard, grievance route, process/outcome indicator and source-status qualification.

PYQ DEMAND CARD 2 - 2020 General Studies Paper II Question 6

Demand: Health policies for geriatric and maternal care. Discuss, 10 marks, 150 words, exactly as recorded in the audited 2018-2023 Mains routing ledger, where ownership is recorded as cross-cutting between this topic and the senior-citizens topic.

Status: Routed to this owner in the audited 2018-2023 Mains routing ledger as cross-cutting Core ownership shared with the senior-citizens topic, and that shared ownership is stated rather than claimed exclusively. No official answer key exists for a Mains demand and none is claimed.

Model solution: Claim: geriatric and maternal care are the two life-cycle ends at which India's mixed health architecture is most exposed, because both need continuity rather than a single episode of treatment. Named evidence and example: the twelve comprehensive primary health care packages, which expressly include geriatric and palliative care alongside maternal services; the health mission's support for frontline workers, free drugs and diagnostics, mobile medical units and maternal-health protection; the hospitalisation pillar's age-based route allowing every citizen aged seventy and above to enrol regardless of income, distinct from its core eligibility-based route; and the tiered referral spine from the sub-centre to the district hospital. Analysis: maternal care is protected by a dense frontline and referral chain, so its failures are chiefly tier and transport failures, whereas geriatric care depends on chronic-disease management, medicines and palliative support that sit largely outside hospitalisation cover, which is why the outpatient and medicine burden is the binding constraint at the older end. Qualification: the age-based enrolment route must not be described as universal cover for every age, and no claim, card or facility count is cited as evidence of quality. Why this earns marks: it discusses two policy fields through one structural lens and separates two enrolment routes precisely.

Demand decoding: The directive **answer** requires a direct position on “PYQ DEMAND CARD 2 - 2020 General Studies Paper II Question 6”, every clause, constitutional/statutory/policy distinction, rights-holder and institution map, eligibility/exclusion and delivery chain, intersectional and regional variation, named Indian evidence, accountability and qualified conclusion.

Detailed examiner-grade model answer:

Introduction and thesis: Claim: geriatric and maternal care are the two life-cycle ends at which India's mixed health architecture is most exposed, because both need continuity rather than a single episode of treatment. Named evidence and example: the twelve comprehensive primary health care packages, which expressly include geriatric and palliative care alongside maternal services; the health mission's support for frontline workers, free drugs and diagnostics, mobile medical units and maternal-health protection; the hospitalisation pillar's age-based route allowing every citizen aged seventy and above to enrol regardless of income, distinct from its core eligibility-based route; and the tiered referral spine from the sub-centre to the district hospital. Analysis: maternal care is protected by a dense frontline and referral chain, so its failures are chiefly tier and transport failures, whereas geriatric care depends on chronic-disease management, medicines and palliative support that sit largely outside hospitalisation cover, which is why the outpatient and medicine burden is the binding constraint at the older end. Qualification: the age-based enrolment route must not be described as universal cover for every age, and no claim, card or facility count is cited as evidence of quality. Why this earns marks: it discusses two policy fields through one structural lens and separates two enrolment routes precisely.

Analytical body:

1. **Claim and named evidence:** PYQ DEMAND CARD 2 - 2020 General Studies Paper II Question 6 **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.

Counter-position / limit: A constitutional promise, prohibition, reservation provision, scheme catalogue, allocation, registration count or headline statistic cannot alone establish accessible implementation, adequate benefit, dignity, remedy or attributable outcome; test eligibility, institutions, frontline capacity, intersectionality, regional variation, grievance and evidence status.

Qualified conclusion: Claim: geriatric and maternal care are the two life-cycle ends at which India's mixed health architecture is most exposed, because both need continuity rather than a single episode of treatment. Named evidence and example: the twelve comprehensive primary health care packages, which expressly include geriatric and palliative care alongside maternal services; the health mission's support for frontline workers, free drugs and diagnostics, mobile medical units and maternal-health protection; the hospitalisation pillar's age-based route allowing every citizen aged seventy and above to enrol regardless of income, distinct from its core eligibility-based route; and the tiered referral spine from the sub-centre to the district hospital. Analysis: maternal care is protected by a dense frontline and referral chain, so its failures are chiefly tier and transport failures, whereas geriatric care depends on chronic-disease management, medicines and palliative support that sit largely outside hospitalisation cover, which is why the outpatient and medicine burden is the binding constraint at the older end. Qualification: the age-based enrolment route must not be described as universal cover for every age, and no claim, card or facility count is cited as evidence of quality. Why this earns marks: it discusses two policy fields through one structural lens and separates two enrolment routes precisely.

Executable exam-length answer / compression plan: For 15 marks, spend one-sixth of the time decoding the directive and drawing right/need -> institution -> eligibility -> delivery -> outcome -> remedy; state a thesis; write four to seven claim -> named evidence -> analysis -> qualification points; reserve the final minute for dignity, accessibility, portability, grievance, denominator, fiscal and causal limits.

How to improve this answer: For "PYQ DEMAND CARD 2 - 2020 General Studies Paper II Question 6", replace the weakest scheme-catalogue point with one named rights-holder, duty-bearer, eligibility bottleneck, safeguard, grievance route, process/outcome indicator and source-status qualification.

PYQ DEMAND CARD 3 - 2021 General Studies Paper II Question 6

Demand: Primary health structure as a precondition for sustainable development. Analyze, 10 marks, 150 words, exactly as recorded in the audited 2018-2023 Mains routing ledger.

Status: Routed to this owner in the audited 2018-2023 Mains routing ledger, where the Core route is recorded as superseding the older Advanced ownership. No official answer key exists for a Mains demand and none is claimed.

Model solution: Claim: a funded primary structure is the precondition because it is the only layer that produces prevention, early detection and continuity, which are the inputs sustainable development actually consumes. Named evidence and example: the universal health coverage cube with its population, service and financial-protection dimensions; the policy commitment to raise public health expenditure to two and a half per cent of gross domestic product and to build a primary-care-led model; the financing chain from revenue collection through risk pooling and strategic purchasing to service provision; and the national health accounts sequence recording out-of-pocket expenditure at 47.1 per cent of total health expenditure in 2019-20, 44.4 per cent in 2020-21 and 39.4 per cent in 2021-22. Analysis: hospital-led systems purchase treatment after the fact and leave the recurring outpatient and medicine burden with the household, so financial protection improves only as the primary layer absorbs first contact, and the same layer carries the inter-sectoral determinants of water, sanitation and nutrition on which every other development goal rests. Qualification: the expenditure sequence is a dated national aggregate that says nothing about a particular household, and the policy target is a commitment rather than a justiciable entitlement. Why this earns marks: it analyses a precondition through a financing mechanism with dated evidence and an explicit status caveat.

Demand decoding: The directive **answer** requires a direct position on "PYQ DEMAND CARD 3 - 2021 General Studies Paper II Question 6", every clause, constitutional/statutory/policy distinction, rights-holder and institution map, eligibility/exclusion and delivery chain, intersectional and regional variation, named Indian evidence, accountability

and qualified conclusion.

Detailed examiner-grade model answer:

Introduction and thesis: Claim: a funded primary structure is the precondition because it is the only layer that produces prevention, early detection and continuity, which are the inputs sustainable development actually consumes. Named evidence and example: the universal health coverage cube with its population, service and financial-protection dimensions; the policy commitment to raise public health expenditure to two and a half per cent of gross domestic product and to build a primary-care-led model; the financing chain from revenue collection through risk pooling and strategic purchasing to service provision; and the national health accounts sequence recording out-of-pocket expenditure at 47.1 per cent of total health expenditure in 2019-20, 44.4 per cent in 2020-21 and 39.4 per cent in 2021-22. Analysis: hospital-led systems purchase treatment after the fact and leave the recurring outpatient and medicine burden with the household, so financial protection improves only as the primary layer absorbs first contact, and the same layer carries the inter-sectoral determinants of water, sanitation and nutrition on which every other development goal rests. Qualification: the expenditure sequence is a dated national aggregate that says nothing about a particular household, and the policy target is a commitment rather than a justiciable entitlement. Why this earns marks: it analyses a precondition through a financing mechanism with dated evidence and an explicit status caveat.

Analytical body:

1. **Claim and named evidence:** PYQ DEMAND CARD 3 - 2021 General Studies Paper II Question 6 **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
2. **Claim and named evidence:** Demand: Primary health structure as a precondition for sustainable development. Analyze, 10 marks, 150 words, exactly as recorded in the audited 2018-2023 Mains routing ledger. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.

Counter-position / limit: A constitutional promise, prohibition, reservation provision, scheme catalogue, allocation, registration count or headline statistic cannot alone establish accessible implementation, adequate benefit, dignity, remedy or attributable outcome; test eligibility, institutions, frontline capacity, intersectionality, regional variation, grievance and evidence status.

Qualified conclusion: Claim: a funded primary structure is the precondition because it is the only layer that produces prevention, early detection and continuity, which are the inputs sustainable development actually consumes. Named evidence and example: the universal health coverage cube with its population, service and financial-protection dimensions; the policy commitment to raise public health expenditure to two and a half per cent of gross domestic product and to build a primary-care-led model; the financing chain from revenue collection through risk pooling and strategic purchasing to service provision; and the national health accounts sequence recording out-of-pocket expenditure at 47.1 per cent of total health expenditure in 2019-20, 44.4 per cent in 2020-21 and 39.4 per cent in 2021-22. Analysis: hospital-led systems purchase treatment after the fact and leave the recurring outpatient and medicine burden with the household, so financial protection improves only as the primary layer absorbs first contact, and the same layer carries the inter-sectoral determinants of water, sanitation and nutrition on which every other development goal rests. Qualification: the expenditure sequence is a dated national aggregate that says nothing about a particular household, and the policy target is a commitment rather than a justiciable entitlement. Why this earns marks: it analyses a precondition through a financing mechanism with dated evidence and an explicit status caveat.

Executable exam-length answer / compression plan: For 15 marks, spend one-sixth of the time decoding the directive and drawing right/need -> institution -> eligibility -> delivery -> outcome -> remedy; state a thesis; write four to seven claim -> named evidence -> analysis -> qualification points; reserve the final minute for dignity, accessibility, portability, grievance, denominator, fiscal and causal limits.

How to improve this answer: For "PYQ DEMAND CARD 3 - 2021 General Studies Paper II Question 6", replace the weakest scheme-catalogue point with one named rights-holder, duty-bearer, eligibility bottleneck, safeguard, grievance route, process/outcome indicator and source-status qualification.

PYQ DEMAND CARD 4 - 2024 General Studies Paper II Question 17

Demand: In a crucial domain like the public healthcare system the Indian State should play a vital role to contain the adverse impact of marketisation of the system. Suggest some measures through which the State can enhance the reach of public healthcare at the grassroots level, 15 marks, 250 words, exactly as recorded in the audited 2024-2025 Mains routing ledger and reproduced in the Basic owner.

Status: Routed to this owner in the audited 2024-2025 Mains routing ledger. No official answer key exists for a Mains demand and none is claimed.

Model solution: Claim: marketisation is contained by provision and regulation together, and grassroots reach is a provision problem before it is a regulation problem. Named evidence and example: the recorded harms of marketisation, namely high costs, unnecessary procedures, catastrophic spending and inequitable access; the constitutional and policy basis for State intervention in the Directive Principle on public health and in the National Health Policy, 2017; the upgraded wellness centres delivering twelve comprehensive primary health care packages through a community health officer or mid-level health provider; the health mission's support for frontline workers, free drugs and diagnostics and mobile medical units, with its urban arm created by the Union Cabinet decision dated 1 May 2013; the hospitalisation pillar's cover of up to five lakh rupees per family per year; and the regulatory instruments of the Clinical Establishments Act, 2010, device price capping by the pharmaceutical pricing authority and standard treatment guidelines. Analysis: expanding the first-contact layer moves care-seeking away from the unregulated curative market, free drugs and diagnostics remove the two costs that push households into private provision, and financial protection shifts the catastrophic tail off the household, while regulation works only where the State has adopted the central Act or notified equivalent rules, so each measure must be paired with the adoption or staffing gap it closes. Qualification: the private sector's capacity contribution should not be denied, insurance cannot answer outpatient and medicine costs, and no cumulative card, claim or facility count is offered as proof of quality. Why this earns marks: it suggests measures that are institutionally specific, each attached to a named gap, and closes with a graded verdict rather than a wish list.

Demand decoding: The directive **answer** requires a direct position on "PYQ DEMAND CARD 4 - 2024 General Studies Paper II Question 17", every clause, constitutional/statutory/policy distinction, rights-holder and institution map, eligibility/exclusion and delivery chain, intersectional and regional variation, named Indian evidence, accountability and qualified conclusion.

Detailed examiner-grade model answer:

Introduction and thesis: Claim: marketisation is contained by provision and regulation together, and grassroots reach is a provision problem before it is a regulation problem. Named evidence and example: the recorded harms of marketisation, namely high costs, unnecessary procedures, catastrophic spending and inequitable access; the constitutional and policy basis for State intervention in the Directive Principle on public health and in the National Health Policy, 2017; the upgraded wellness centres delivering twelve comprehensive primary health care packages through a community health officer or mid-level health provider; the health mission's support for frontline workers, free drugs and diagnostics and mobile medical units, with its urban arm created by the Union Cabinet decision dated 1 May 2013; the hospitalisation pillar's cover of up to five lakh rupees per family per year; and the regulatory instruments of the Clinical Establishments Act, 2010, device price capping by the pharmaceutical pricing authority and standard treatment guidelines. Analysis: expanding the first-contact layer moves care-seeking away from the unregulated curative market, free drugs and diagnostics remove the two costs that push households into private provision, and financial protection shifts the catastrophic tail off the household, while regulation works only where the State has adopted the central Act or notified equivalent rules, so each measure must be paired with the adoption or staffing gap it closes. Qualification: the private sector's capacity contribution should not be denied, insurance cannot answer outpatient and medicine costs, and no cumulative card, claim or facility count is offered as proof of quality. Why this earns marks: it suggests measures that are institutionally specific, each attached to a named gap, and closes with a graded verdict rather than a wish list.

Analytical body:

1. **Claim and named evidence:** PYQ DEMAND CARD 4 - 2024 General Studies Paper II Question 17 **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or

residual exclusion.

2. **Claim and named evidence:** Status: Routed to this owner in the audited 2024-2025 Mains routing ledger. No official answer key exists for a Mains demand and none is claimed. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.

Counter-position / limit: A constitutional promise, prohibition, reservation provision, scheme catalogue, allocation, registration count or headline statistic cannot alone establish accessible implementation, adequate benefit, dignity, remedy or attributable outcome; test eligibility, institutions, frontline capacity, intersectionality, regional variation, grievance and evidence status.

Qualified conclusion: Claim: marketisation is contained by provision and regulation together, and grassroots reach is a provision problem before it is a regulation problem. Named evidence and example: the recorded harms of marketisation, namely high costs, unnecessary procedures, catastrophic spending and inequitable access; the constitutional and policy basis for State intervention in the Directive Principle on public health and in the National Health Policy, 2017; the upgraded wellness centres delivering twelve comprehensive primary health care packages through a community health officer or mid-level health provider; the health mission's support for frontline workers, free drugs and diagnostics and mobile medical units, with its urban arm created by the Union Cabinet decision dated 1 May 2013; the hospitalisation pillar's cover of up to five lakh rupees per family per year; and the regulatory instruments of the Clinical Establishments Act, 2010, device price capping by the pharmaceutical pricing authority and standard treatment guidelines. Analysis: expanding the first-contact layer moves care-seeking away from the unregulated curative market, free drugs and diagnostics remove the two costs that push households into private provision, and financial protection shifts the catastrophic tail off the household, while regulation works only where the State has adopted the central Act or notified equivalent rules, so each measure must be paired with the adoption or staffing gap it closes. Qualification: the private sector's capacity contribution should not be denied, insurance cannot answer outpatient and medicine costs, and no cumulative card, claim or facility count is offered as proof of quality. Why this earns marks: it suggests measures that are institutionally specific, each attached to a named gap, and closes with a graded verdict rather than a wish list.

Executable exam-length answer / compression plan: For 15 marks, spend one-sixth of the time decoding the directive and drawing right/need -> institution -> eligibility -> delivery -> outcome -> remedy; state a thesis; write four to seven claim -> named evidence -> analysis -> qualification points; reserve the final minute for dignity, accessibility, portability, grievance, denominator, fiscal and causal limits.

How to improve this answer: For "PYQ DEMAND CARD 4 - 2024 General Studies Paper II Question 17", replace the weakest scheme-catalogue point with one named rights-holder, duty-bearer, eligibility bottleneck, safeguard, grievance route, process/outcome indicator and source-status qualification.

ORIGINAL MAINS 1 - 10 MARKS

Question: Distinguish the two pillars of Ayushman Bharat and explain how they jointly serve universal health coverage. Answer in about 150 words.

Model thesis: The two pillars address different rungs of the care hierarchy and different edges of the coverage cube, so the answer must define each precisely, connect them through the referral spine and close by naming the dimension that remains weakest.

Claim -> named evidence -> analysis -> qualification:

- The first pillar of Ayushman Bharat upgrades sub-centres and primary health centres into Health and Wellness Centres, now carried as Ayushman Arogya Mandirs, which deliver comprehensive primary health care through twelve service packages that extend beyond reproductive and child health to non-communicable diseases, mental health, ear, nose and throat care, ophthalmology, oral health, geriatric and palliative care and emergency care, staffed by a community health officer or mid-level health provider; the design intent is to shift care-seeking from hospitals to a local first-contact layer, which is why describing this pillar as reproductive and child health alone understates it.
- The second pillar, Pradhan Mantri Jan Arogya Yojana, provides cashless secondary and tertiary hospitalisation cover of up to five lakh rupees per family per year at empanelled public and private hospitals for its core eligible

households, which are identified through the deprivation criteria of the socio-economic and caste census, and the owners separately record a distinct expansion under which all citizens aged seventy years and above can enrol regardless of income or that census status; the discipline required is exact, because the core scheme is eligibility-based while the age-based expansion is universal only for the seventy-plus group, so an age-specific expansion must never be written up as universal coverage for every age.

- Universal health coverage means ensuring that all people have access to the promotive, preventive, curative and rehabilitative health services they need, of sufficient quality, without suffering financial hardship, and it is analysed on three dimensions that the World Health Organization presents as a cube: population, which is the breadth of who is covered, services, which is the depth of what is covered, and financial protection, which is the height of how little the user pays at the point of service; the analytical value of the cube is that a system can expand on one dimension while contracting on another, so an answer that reports only beneficiary numbers has measured one edge of a three-dimensional claim.
- The financing chain runs from revenue collection through risk pooling and strategic purchasing to service provision, and the owners record the decisive consequence that insurance without public primary care leaves prevention, medicines and outpatient out-of-pocket expenditure unresolved; this is the analytical core of any question about financial protection, because hospitalisation cover addresses the catastrophic tail while the recurring outpatient and medicine burden, which is where most household health spending actually occurs, is addressed only by funded primary provision.

Qualified conclusion: The two pillars address different rungs of the care hierarchy and different edges of the coverage cube, so the answer must define each precisely, connect them through the referral spine and close by naming the dimension that remains weakest.

Demand decoding: The directive **explain** requires a direct position on “Distinguish the two pillars of Ayushman Bharat and explain how they jointly serve universal...”, every clause, constitutional/statutory/policy distinction, rights-holder and institution map, eligibility/exclusion and delivery chain, intersectional and regional variation, named Indian evidence, accountability and qualified conclusion.

Detailed examiner-grade model answer:

Introduction and thesis: The two pillars address different rungs of the care hierarchy and different edges of the coverage cube, so the answer must define each precisely, connect them through the referral spine and close by naming the dimension that remains weakest.

Analytical body:

1. **Claim and named evidence:** The first pillar of Ayushman Bharat upgrades sub-centres and primary health centres into Health and Wellness Centres, now carried as Ayushman Arogya Mandirs, which deliver comprehensive primary health care through twelve service packages that extend beyond reproductive and child health to non-communicable diseases, mental health, ear, nose and throat care, ophthalmology, oral health, geriatric and palliative care and emergency care, staffed by a community health officer or mid-level health provider; the design intent is to shift care-seeking from hospitals to a local first-contact layer, which is why describing this pillar as reproductive and child health alone understates it. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
2. **Claim and named evidence:** The second pillar, Pradhan Mantri Jan Arogya Yojana, provides cashless secondary and tertiary hospitalisation cover of up to five lakh rupees per family per year at empanelled public and private hospitals for its core eligible households, which are identified through the deprivation criteria of the socio-economic and caste census, and the owners separately record a distinct expansion under which all citizens aged seventy years and above can enrol regardless of income or that census status; the discipline required is exact, because the core scheme is eligibility-based while the age-based expansion is universal only for the seventy-plus group, so an age-specific expansion must never be written up as universal coverage for every age. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.

3. **Claim and named evidence:** Universal health coverage means ensuring that all people have access to the promotive, preventive, curative and rehabilitative health services they need, of sufficient quality, without suffering financial hardship, and it is analysed on three dimensions that the World Health Organization presents as a cube: population, which is the breadth of who is covered, services, which is the depth of what is covered, and financial protection, which is the height of how little the user pays at the point of service; the analytical value of the cube is that a system can expand on one dimension while contracting on another, so an answer that reports only beneficiary numbers has measured one edge of a three-dimensional claim. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
4. **Claim and named evidence:** The financing chain runs from revenue collection through risk pooling and strategic purchasing to service provision, and the owners record the decisive consequence that insurance without public primary care leaves prevention, medicines and outpatient out-of-pocket expenditure unresolved; this is the analytical core of any question about financial protection, because hospitalisation cover addresses the catastrophic tail while the recurring outpatient and medicine burden, which is where most household health spending actually occurs, is addressed only by funded primary provision. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.

Counter-position / limit: A constitutional promise, prohibition, reservation provision, scheme catalogue, allocation, registration count or headline statistic cannot alone establish accessible implementation, adequate benefit, dignity, remedy or attributable outcome; test eligibility, institutions, frontline capacity, intersectionality, regional variation, grievance and evidence status.

Qualified conclusion: The two pillars address different rungs of the care hierarchy and different edges of the coverage cube, so the answer must define each precisely, connect them through the referral spine and close by naming the dimension that remains weakest.

Executable exam-length answer / compression plan: For 10 marks, spend one-sixth of the time decoding the directive and drawing right/need -> institution -> eligibility -> delivery -> outcome -> remedy; state a thesis; write four to seven claim -> named evidence -> analysis -> qualification points; reserve the final minute for dignity, accessibility, portability, grievance, denominator, fiscal and causal limits.

Why this earns marks: The answer obeys the directive, explains mechanisms rather than listing schemes, uses named India-centric evidence and preserves legal, institutional, group, eligibility, implementation, fiscal, data and outcome distinctions.

How to improve this answer: For “Distinguish the two pillars of Ayushman Bharat and explain how they jointly serve universal...”, replace the weakest scheme-catalogue point with one named rights-holder, duty-bearer, eligibility bottleneck, safeguard, grievance route, process/outcome indicator and source-status qualification.

ORIGINAL MAINS 2 - 10 MARKS

Question: Analyse why a functioning primary health structure is a precondition for sustainable development. Answer in about 150 words.

Model thesis: Primary care is where prevention, early detection and continuity are produced, so the answer must show that hospital-led systems buy treatment without buying health, and must anchor the claim in named Indian primary-care institutions.

Claim -> named evidence -> analysis -> qualification:

- The owners route community health through the Alma-Ata commitments of equity, participation, inter-sectoral action and appropriate technology, and map them onto accredited social health activists, auxiliary nurse midwives, village health, sanitation and nutrition committees and the upgraded wellness centres; this mapping is what allows a community-health demand to be answered with named Indian institutions rather than with an abstract restatement of health for all, and it also explains why inter-sectoral determinants such as water, sanitation and nutrition sit inside a health answer rather than outside it.

- The first pillar of Ayushman Bharat upgrades sub-centres and primary health centres into Health and Wellness Centres, now carried as Ayushman Arogya Mandirs, which deliver comprehensive primary health care through twelve service packages that extend beyond reproductive and child health to non-communicable diseases, mental health, ear, nose and throat care, ophthalmology, oral health, geriatric and palliative care and emergency care, staffed by a community health officer or mid-level health provider; the design intent is to shift care-seeking from hospitals to a local first-contact layer, which is why describing this pillar as reproductive and child health alone understates it.
- The public health system is tiered from the sub-centre through the primary health centre and the community health centre to the sub-district and district hospital, and this hierarchy is the referral spine on which the first pillar's upgrade and the second pillar's empanelment both sit; naming the tier at which a failure occurs is what converts a general complaint about public hospitals into a diagnosis, because a vacancy at the sub-centre, a diagnostic gap at the primary health centre and a specialist shortage at the community health centre call for three different corrections.
- The owners record that the health mission, free drugs and diagnostics initiatives and the health-infrastructure mission strengthen systems while uneven State capacity and workforce vacancies limit outcomes, and that Union financing and standards depend on State delivery capacity because health is delivered by the States; the answer consequence is that a national financing announcement is a necessary but not sufficient condition, and that a graded verdict on any national health programme must be conditioned on the delivery capacity of the State in question.

Qualified conclusion: Primary care is where prevention, early detection and continuity are produced, so the answer must show that hospital-led systems buy treatment without buying health, and must anchor the claim in named Indian primary-care institutions.

Demand decoding: The directive **analyse** requires a direct position on "Analyse why a functioning primary health structure is a precondition for sustainable...", every clause, constitutional/statutory/policy distinction, rights-holder and institution map, eligibility/exclusion and delivery chain, intersectional and regional variation, named Indian evidence, accountability and qualified conclusion.

Detailed examiner-grade model answer:

Introduction and thesis: Primary care is where prevention, early detection and continuity are produced, so the answer must show that hospital-led systems buy treatment without buying health, and must anchor the claim in named Indian primary-care institutions.

Analytical body:

1. **Claim and named evidence:** The owners route community health through the Alma-Ata commitments of equity, participation, inter-sectoral action and appropriate technology, and map them onto accredited social health activists, auxiliary nurse midwives, village health, sanitation and nutrition committees and the upgraded wellness centres; this mapping is what allows a community-health demand to be answered with named Indian institutions rather than with an abstract restatement of health for all, and it also explains why inter-sectoral determinants such as water, sanitation and nutrition sit inside a health answer rather than outside it. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
2. **Claim and named evidence:** The first pillar of Ayushman Bharat upgrades sub-centres and primary health centres into Health and Wellness Centres, now carried as Ayushman Arogya Mandirs, which deliver comprehensive primary health care through twelve service packages that extend beyond reproductive and child health to non-communicable diseases, mental health, ear, nose and throat care, ophthalmology, oral health, geriatric and palliative care and emergency care, staffed by a community health officer or mid-level health provider; the design intent is to shift care-seeking from hospitals to a local first-contact layer, which is why describing this pillar as reproductive and child health alone understates it. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.

3. **Claim and named evidence:** The public health system is tiered from the sub-centre through the primary health centre and the community health centre to the sub-district and district hospital, and this hierarchy is the referral spine on which the first pillar's upgrade and the second pillar's empanelment both sit; naming the tier at which a failure occurs is what converts a general complaint about public hospitals into a diagnosis, because a vacancy at the sub-centre, a diagnostic gap at the primary health centre and a specialist shortage at the community health centre call for three different corrections. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
4. **Claim and named evidence:** The owners record that the health mission, free drugs and diagnostics initiatives and the health-infrastructure mission strengthen systems while uneven State capacity and workforce vacancies limit outcomes, and that Union financing and standards depend on State delivery capacity because health is delivered by the States; the answer consequence is that a national financing announcement is a necessary but not sufficient condition, and that a graded verdict on any national health programme must be conditioned on the delivery capacity of the State in question. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.

Counter-position / limit: A constitutional promise, prohibition, reservation provision, scheme catalogue, allocation, registration count or headline statistic cannot alone establish accessible implementation, adequate benefit, dignity, remedy or attributable outcome; test eligibility, institutions, frontline capacity, intersectionality, regional variation, grievance and evidence status.

Qualified conclusion: Primary care is where prevention, early detection and continuity are produced, so the answer must show that hospital-led systems buy treatment without buying health, and must anchor the claim in named Indian primary-care institutions.

Executable exam-length answer / compression plan: For 10 marks, spend one-sixth of the time decoding the directive and drawing right/need -> institution -> eligibility -> delivery -> outcome -> remedy; state a thesis; write four to seven claim -> named evidence -> analysis -> qualification points; reserve the final minute for dignity, accessibility, portability, grievance, denominator, fiscal and causal limits.

Why this earns marks: The answer obeys the directive, explains mechanisms rather than listing schemes, uses named India-centric evidence and preserves legal, institutional, group, eligibility, implementation, fiscal, data and outcome distinctions.

How to improve this answer: For "Analyse why a functioning primary health structure is a precondition for sustainable...", replace the weakest scheme-catalogue point with one named rights-holder, duty-bearer, eligibility bottleneck, safeguard, grievance route, process/outcome indicator and source-status qualification.

ORIGINAL MAINS 3 - 15 MARKS

Question: Suggest measures through which the State can contain the adverse impact of marketisation and extend public healthcare to the grassroots level. Answer in about 250 words.

Model thesis: Containment requires provision and regulation together, so the answer must define marketisation and its harms, propose expansion of comprehensive primary care and mission-funded public provision, and pair each regulatory instrument with the adoption gap it must close.

Claim -> named evidence -> analysis -> qualification:

- Marketisation refers to the dominance of private providers in curative care, often unregulated or under-regulated, and the owners record its consequences as high costs, unnecessary procedures, catastrophic spending and inequitable access; this is the exact concern raised by the 2024 demand, so an answer to it must define the phenomenon and its harms before proposing measures, and must avoid the two symmetrical errors of denying the private sector's real capacity contribution and of treating regulation as automatic once a statute exists.
- The regulatory instruments recorded by the owners are the Clinical Establishments Act, 2010, a Central Act providing for the registration and regulation of clinical establishments which States must adopt or replace with

equivalent legislation, price capping of the kind exercised by the national pharmaceutical pricing authority over devices, and standard treatment guidelines that promote rational care and cost control; each carries a stated enforcement condition, because State adoption of the Central Act varies and some States have not adopted or notified equivalent rules, and adoption of treatment guidelines varies across public and empanelled private providers, so a recommendation to regulate must name the adoption gap it is closing.

- The National Health Mission's own official page, read on 2 September 2026, records that the National Rural Health Mission was launched by the Prime Minister on 12 April 2005 to provide accessible, affordable and quality health care to the rural population and especially to vulnerable groups, and that the Union Cabinet, by its decision dated 1 May 2013, approved the launch of the National Urban Health Mission as a sub-mission of an over-arching National Health Mission with the National Rural Health Mission being the other sub-mission; the same page records special focus on the Empowered Action Group States, the North Eastern States, Jammu and Kashmir and Himachal Pradesh, a thrust on a fully functional, community-owned, decentralised health delivery system with inter-sectoral convergence acting on determinants such as water, sanitation, education, nutrition and social and gender equality, and measurement of facilities against the Indian Public Health Standards, so the mission is urban as well as rural and treating it as rural-only is a factual error.
- The first pillar of Ayushman Bharat upgrades sub-centres and primary health centres into Health and Wellness Centres, now carried as Ayushman Arogya Mandirs, which deliver comprehensive primary health care through twelve service packages that extend beyond reproductive and child health to non-communicable diseases, mental health, ear, nose and throat care, ophthalmology, oral health, geriatric and palliative care and emergency care, staffed by a community health officer or mid-level health provider; the design intent is to shift care-seeking from hospitals to a local first-contact layer, which is why describing this pillar as reproductive and child health alone understates it.

Qualified conclusion: Containment requires provision and regulation together, so the answer must define marketisation and its harms, propose expansion of comprehensive primary care and mission-funded public provision, and pair each regulatory instrument with the adoption gap it must close.

Demand decoding: The directive **answer** requires a direct position on “Suggest measures through which the State can contain the adverse impact of marketisation and...”, every clause, constitutional/statutory/policy distinction, rights-holder and institution map, eligibility/exclusion and delivery chain, intersectional and regional variation, named Indian evidence, accountability and qualified conclusion.

Detailed examiner-grade model answer:

Introduction and thesis: Containment requires provision and regulation together, so the answer must define marketisation and its harms, propose expansion of comprehensive primary care and mission-funded public provision, and pair each regulatory instrument with the adoption gap it must close.

Analytical body:

1. **Claim and named evidence:** Marketisation refers to the dominance of private providers in curative care, often unregulated or under-regulated, and the owners record its consequences as high costs, unnecessary procedures, catastrophic spending and inequitable access; this is the exact concern raised by the 2024 demand, so an answer to it must define the phenomenon and its harms before proposing measures, and must avoid the two symmetrical errors of denying the private sector's real capacity contribution and of treating regulation as automatic once a statute exists. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
2. **Claim and named evidence:** The regulatory instruments recorded by the owners are the Clinical Establishments Act, 2010, a Central Act providing for the registration and regulation of clinical establishments which States must adopt or replace with equivalent legislation, price capping of the kind exercised by the national pharmaceutical pricing authority over devices, and standard treatment guidelines that promote rational care and cost control; each carries a stated enforcement condition, because State adoption of the Central Act varies and some States have not adopted or notified equivalent rules, and adoption of treatment guidelines varies across public and empanelled private providers, so a recommendation to regulate must name the adoption gap it is closing. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and

outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.

3. **Claim and named evidence:** The National Health Mission's own official page, read on 2 September 2026, records that the National Rural Health Mission was launched by the Prime Minister on 12 April 2005 to provide accessible, affordable and quality health care to the rural population and especially to vulnerable groups, and that the Union Cabinet, by its decision dated 1 May 2013, approved the launch of the National Urban Health Mission as a sub-mission of an over-arching National Health Mission with the National Rural Health Mission being the other sub-mission; the same page records special focus on the Empowered Action Group States, the North Eastern States, Jammu and Kashmir and Himachal Pradesh, a thrust on a fully functional, community-owned, decentralised health delivery system with inter-sectoral convergence acting on determinants such as water, sanitation, education, nutrition and social and gender equality, and measurement of facilities against the Indian Public Health Standards, so the mission is urban as well as rural and treating it as rural-only is a factual error.

Analysis: Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.

4. **Claim and named evidence:** The first pillar of Ayushman Bharat upgrades sub-centres and primary health centres into Health and Wellness Centres, now carried as Ayushman Arogya Mandirs, which deliver comprehensive primary health care through twelve service packages that extend beyond reproductive and child health to non-communicable diseases, mental health, ear, nose and throat care, ophthalmology, oral health, geriatric and palliative care and emergency care, staffed by a community health officer or mid-level health provider; the design intent is to shift care-seeking from hospitals to a local first-contact layer, which is why describing this pillar as reproductive and child health alone understates it. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.

Counter-position / limit: A constitutional promise, prohibition, reservation provision, scheme catalogue, allocation, registration count or headline statistic cannot alone establish accessible implementation, adequate benefit, dignity, remedy or attributable outcome; test eligibility, institutions, frontline capacity, intersectionality, regional variation, grievance and evidence status.

Qualified conclusion: Containment requires provision and regulation together, so the answer must define marketisation and its harms, propose expansion of comprehensive primary care and mission-funded public provision, and pair each regulatory instrument with the adoption gap it must close.

Executable exam-length answer / compression plan: For 15 marks, spend one-sixth of the time decoding the directive and drawing right/need -> institution -> eligibility -> delivery -> outcome -> remedy; state a thesis; write four to seven claim -> named evidence -> analysis -> qualification points; reserve the final minute for dignity, accessibility, portability, grievance, denominator, fiscal and causal limits.

Why this earns marks: The answer obeys the directive, explains mechanisms rather than listing schemes, uses named India-centric evidence and preserves legal, institutional, group, eligibility, implementation, fiscal, data and outcome distinctions.

How to improve this answer: For "Suggest measures through which the State can contain the adverse impact of marketisation and...", replace the weakest scheme-catalogue point with one named rights-holder, duty-bearer, eligibility bottleneck, safeguard, grievance route, process/outcome indicator and source-status qualification.

ORIGINAL MAINS 4 - 15 MARKS

Question: Examine the claim that insurance-led financing alone can deliver financial protection in health. Answer in about 250 words.

Model thesis: Insurance covers the catastrophic tail while most household spending is recurring and outpatient, so the examination must run the financing chain, use the dated expenditure sequence and end with a conditional rather than a categorical verdict.

Claim -> named evidence -> analysis -> qualification:

- The financing chain runs from revenue collection through risk pooling and strategic purchasing to service provision, and the owners record the decisive consequence that insurance without public primary care leaves prevention, medicines and outpatient out-of-pocket expenditure unresolved; this is the analytical core of any question about financial protection, because hospitalisation cover addresses the catastrophic tail while the recurring outpatient and medicine burden, which is where most household health spending actually occurs, is addressed only by funded primary provision.
- Out-of-pocket expenditure is health spending paid directly by households at the point of service, and the owners record the National Health Accounts sequence as 47.1 per cent of total health expenditure in 2019-20, 44.4 per cent in 2020-21 and 39.4 per cent in 2021-22, with the latest cited official estimate being 39.4 per cent for the accounting year 2021-22; two disciplines follow, first that the publication date must never be confused with the accounting year, and second that a falling national share is a dated aggregate rather than evidence that every household faces a lighter burden, and high out-of-pocket expenditure indicates inadequate public provision rather than high utilisation.
- Catastrophic health expenditure is health spending that exceeds a threshold share of household income or consumption, commonly stated in the range of ten to twenty-five per cent, and it pushes households into poverty or debt, with high out-of-pocket expenditure as its primary driver; this is the mechanism that makes health a social-justice question rather than only a service-delivery question, because a single hospitalisation can undo years of asset accumulation, and it is the bridge that connects this topic to the poverty and nutrition owner.
- Social health insurance pools contribution-linked cover, of which the employees' state insurance system is the Indian example, whereas tax-funded programmes draw on general revenue, which is how both the health mission and the hospitalisation pillar are financed, and the owners record that India uses a mixed and fragmented architecture rather than one comprehensive national pool; the standing objective-question trap is to describe the hospitalisation pillar as contribution-based social health insurance, which reverses the two financing families.

Qualified conclusion: Insurance covers the catastrophic tail while most household spending is recurring and outpatient, so the examination must run the financing chain, use the dated expenditure sequence and end with a conditional rather than a categorical verdict.

Demand decoding: The directive **examine** requires a direct position on "Examine the claim that insurance-led financing alone can deliver financial protection in...", every clause, constitutional/statutory/policy distinction, rights-holder and institution map, eligibility/exclusion and delivery chain, intersectional and regional variation, named Indian evidence, accountability and qualified conclusion.

Detailed examiner-grade model answer:

Introduction and thesis: Insurance covers the catastrophic tail while most household spending is recurring and outpatient, so the examination must run the financing chain, use the dated expenditure sequence and end with a conditional rather than a categorical verdict.

Analytical body:

1. **Claim and named evidence:** The financing chain runs from revenue collection through risk pooling and strategic purchasing to service provision, and the owners record the decisive consequence that insurance without public primary care leaves prevention, medicines and outpatient out-of-pocket expenditure unresolved; this is the analytical core of any question about financial protection, because hospitalisation cover addresses the catastrophic tail while the recurring outpatient and medicine burden, which is where most household health spending actually occurs, is addressed only by funded primary provision. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status,

denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.

2. **Claim and named evidence:** Out-of-pocket expenditure is health spending paid directly by households at the point of service, and the owners record the National Health Accounts sequence as 47.1 per cent of total health expenditure in 2019-20, 44.4 per cent in 2020-21 and 39.4 per cent in 2021-22, with the latest cited official estimate being 39.4 per cent for the accounting year 2021-22; two disciplines follow, first that the publication date must never be confused with the accounting year, and second that a falling national share is a dated aggregate rather than evidence that every household faces a lighter burden, and high out-of-pocket expenditure indicates inadequate public provision rather than high utilisation. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
3. **Claim and named evidence:** Catastrophic health expenditure is health spending that exceeds a threshold share of household income or consumption, commonly stated in the range of ten to twenty-five per cent, and it pushes households into poverty or debt, with high out-of-pocket expenditure as its primary driver; this is the mechanism that makes health a social-justice question rather than only a service-delivery question, because a single hospitalisation can undo years of asset accumulation, and it is the bridge that connects this topic to the poverty and nutrition owner. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
4. **Claim and named evidence:** Social health insurance pools contribution-linked cover, of which the employees' state insurance system is the Indian example, whereas tax-funded programmes draw on general revenue, which is how both the health mission and the hospitalisation pillar are financed, and the owners record that India uses a mixed and fragmented architecture rather than one comprehensive national pool; the standing objective-question trap is to describe the hospitalisation pillar as contribution-based social health insurance, which reverses the two financing families. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.

Counter-position / limit: A constitutional promise, prohibition, reservation provision, scheme catalogue, allocation, registration count or headline statistic cannot alone establish accessible implementation, adequate benefit, dignity, remedy or attributable outcome; test eligibility, institutions, frontline capacity, intersectionality, regional variation, grievance and evidence status.

Qualified conclusion: Insurance covers the catastrophic tail while most household spending is recurring and outpatient, so the examination must run the financing chain, use the dated expenditure sequence and end with a conditional rather than a categorical verdict.

Executable exam-length answer / compression plan: For 15 marks, spend one-sixth of the time decoding the directive and drawing right/need -> institution -> eligibility -> delivery -> outcome -> remedy; state a thesis; write four to seven claim -> named evidence -> analysis -> qualification points; reserve the final minute for dignity, accessibility, portability, grievance, denominator, fiscal and causal limits.

Why this earns marks: The answer obeys the directive, explains mechanisms rather than listing schemes, uses named India-centric evidence and preserves legal, institutional, group, eligibility, implementation, fiscal, data and outcome distinctions.

How to improve this answer: For "Examine the claim that insurance-led financing alone can deliver financial protection in...", replace the weakest scheme-catalogue point with one named rights-holder, duty-bearer, eligibility bottleneck, safeguard, grievance route, process/outcome indicator and source-status qualification.

ORIGINAL MAINS 5 - 20 MARKS

Question: Assess India's progress towards universal health coverage across the population, service and financial-protection dimensions. Answer in about 300 words.

Model thesis: Progress is uneven across the three dimensions, so the assessment must test each edge separately with dated evidence, state which edge the available evidence cannot settle and close with a graded verdict carrying an explicit data caveat.

Claim -> named evidence -> analysis -> qualification:

- Universal health coverage means ensuring that all people have access to the promotive, preventive, curative and rehabilitative health services they need, of sufficient quality, without suffering financial hardship, and it is analysed on three dimensions that the World Health Organization presents as a cube: population, which is the breadth of who is covered, services, which is the depth of what is covered, and financial protection, which is the height of how little the user pays at the point of service; the analytical value of the cube is that a system can expand on one dimension while contracting on another, so an answer that reports only beneficiary numbers has measured one edge of a three-dimensional claim.
- The second pillar, Pradhan Mantri Jan Arogya Yojana, provides cashless secondary and tertiary hospitalisation cover of up to five lakh rupees per family per year at empanelled public and private hospitals for its core eligible households, which are identified through the deprivation criteria of the socio-economic and caste census, and the owners separately record a distinct expansion under which all citizens aged seventy years and above can enrol regardless of income or that census status; the discipline required is exact, because the core scheme is eligibility-based while the age-based expansion is universal only for the seventy-plus group, so an age-specific expansion must never be written up as universal coverage for every age.
- Out-of-pocket expenditure is health spending paid directly by households at the point of service, and the owners record the National Health Accounts sequence as 47.1 per cent of total health expenditure in 2019-20, 44.4 per cent in 2020-21 and 39.4 per cent in 2021-22, with the latest cited official estimate being 39.4 per cent for the accounting year 2021-22; two disciplines follow, first that the publication date must never be confused with the accounting year, and second that a falling national share is a dated aggregate rather than evidence that every household faces a lighter burden, and high out-of-pocket expenditure indicates inadequate public provision rather than high utilisation.
- The owners record that cumulative card counts and cumulative claim counts are dashboard outputs and are not by themselves a measure of service quality or of financial protection, so this package asserts no card total, claim total, empanelment count, wellness-centre count, expenditure share beyond the dated national health accounts sequence, or state-level rank. On ownership, four General Studies Mains demands are routed to this topic in the audited ledgers, from 2018, 2020, 2021 and 2024, with the 2020 geriatric and maternal-care demand recorded as cross-cutting ownership shared with the senior-citizens topic, and three objective demands are routed, namely the 2023 maternal-health scheme question and the 2023 public-health-system question whose official keys the ledgers record as unavailable locally, and the 2024 maternal-health scheme question for which the ledger records that the official Set-A key is available locally but expressly states that no answer is recorded or inferred; this package therefore records no option for any of the three.

Qualified conclusion: Progress is uneven across the three dimensions, so the assessment must test each edge separately with dated evidence, state which edge the available evidence cannot settle and close with a graded verdict carrying an explicit data caveat.

Demand decoding: The directive **assess** requires a direct position on "Assess India's progress towards universal health coverage across the population, service and...", every clause, constitutional/statutory/policy distinction, rights-holder and institution map, eligibility/exclusion and delivery chain, intersectional and regional variation, named Indian evidence, accountability and qualified conclusion.

Detailed examiner-grade model answer:

Introduction and thesis: Progress is uneven across the three dimensions, so the assessment must test each edge separately with dated evidence, state which edge the available evidence cannot settle and close with a graded verdict carrying an explicit data caveat.

Analytical body:

1. **Claim and named evidence:** Universal health coverage means ensuring that all people have access to the promotive, preventive, curative and rehabilitative health services they need, of sufficient quality, without suffering financial hardship, and it is analysed on three dimensions that the World Health Organization presents as a cube: population, which is the breadth of who is covered, services, which is the depth of what is covered, and financial protection, which is the height of how little the user pays at the point of service; the analytical value of the cube is that a system can expand on one dimension while contracting on another, so an answer that reports only beneficiary numbers has measured one edge of a three-dimensional claim. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
2. **Claim and named evidence:** The second pillar, Pradhan Mantri Jan Arogya Yojana, provides cashless secondary and tertiary hospitalisation cover of up to five lakh rupees per family per year at empanelled public and private hospitals for its core eligible households, which are identified through the deprivation criteria of the socio-economic and caste census, and the owners separately record a distinct expansion under which all citizens aged seventy years and above can enrol regardless of income or that census status; the discipline required is exact, because the core scheme is eligibility-based while the age-based expansion is universal only for the seventy-plus group, so an age-specific expansion must never be written up as universal coverage for every age. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
3. **Claim and named evidence:** Out-of-pocket expenditure is health spending paid directly by households at the point of service, and the owners record the National Health Accounts sequence as 47.1 per cent of total health expenditure in 2019-20, 44.4 per cent in 2020-21 and 39.4 per cent in 2021-22, with the latest cited official estimate being 39.4 per cent for the accounting year 2021-22; two disciplines follow, first that the publication date must never be confused with the accounting year, and second that a falling national share is a dated aggregate rather than evidence that every household faces a lighter burden, and high out-of-pocket expenditure indicates inadequate public provision rather than high utilisation. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
4. **Claim and named evidence:** The owners record that cumulative card counts and cumulative claim counts are dashboard outputs and are not by themselves a measure of service quality or of financial protection, so this package asserts no card total, claim total, empanelment count, wellness-centre count, expenditure share beyond the dated national health accounts sequence, or state-level rank. On ownership, four General Studies Mains demands are routed to this topic in the audited ledgers, from 2018, 2020, 2021 and 2024, with the 2020 geriatric and maternal-care demand recorded as cross-cutting ownership shared with the senior-citizens topic, and three objective demands are routed, namely the 2023 maternal-health scheme question and the 2023 public-health-system question whose official keys the ledgers record as unavailable locally, and the 2024 maternal-health scheme question for which the ledger records that the official Set-A key is available locally but expressly states that no answer is recorded or inferred; this package therefore records no option for any of the three. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.

Counter-position / limit: A constitutional promise, prohibition, reservation provision, scheme catalogue, allocation, registration count or headline statistic cannot alone establish accessible implementation, adequate benefit, dignity, remedy or attributable outcome; test eligibility, institutions, frontline capacity, intersectionality, regional variation, grievance and evidence status.

Qualified conclusion: Progress is uneven across the three dimensions, so the assessment must test each edge separately with dated evidence, state which edge the available evidence cannot settle and close with a graded verdict carrying an explicit data caveat.

Executable exam-length answer / compression plan: For 20 marks, spend one-sixth of the time decoding the directive and drawing right/need -> institution -> eligibility -> delivery -> outcome -> remedy; state a thesis; write four to seven claim -> named evidence -> analysis -> qualification points; reserve the final minute for dignity, accessibility, portability, grievance, denominator, fiscal and causal limits.

Why this earns marks: The answer obeys the directive, explains mechanisms rather than listing schemes, uses named India-centric evidence and preserves legal, institutional, group, eligibility, implementation, fiscal, data and outcome distinctions.

How to improve this answer: For “Assess India's progress towards universal health coverage across the population, service and...”, replace the weakest scheme-catalogue point with one named rights-holder, duty-bearer, eligibility bottleneck, safeguard, grievance route, process/outcome indicator and source-status qualification.

ORIGINAL MAINS 6 - 20 MARKS

Question: Assess the federal and capacity conditions under which national health financing produces measured health outcomes. Answer in about 300 words.

Model thesis: National money becomes health only through State delivery capacity, so the assessment must connect financing, standards, workforce and inter-sectoral determinants, and must end by naming the condition on which the verdict would reverse.

Claim -> named evidence -> analysis -> qualification:

- The owners record that the health mission, free drugs and diagnostics initiatives and the health-infrastructure mission strengthen systems while uneven State capacity and workforce vacancies limit outcomes, and that Union financing and standards depend on State delivery capacity because health is delivered by the States; the answer consequence is that a national financing announcement is a necessary but not sufficient condition, and that a graded verdict on any national health programme must be conditioned on the delivery capacity of the State in question.
- The National Health Mission's own official page, read on 2 September 2026, records that the National Rural Health Mission was launched by the Prime Minister on 12 April 2005 to provide accessible, affordable and quality health care to the rural population and especially to vulnerable groups, and that the Union Cabinet, by its decision dated 1 May 2013, approved the launch of the National Urban Health Mission as a sub-mission of an over-arching National Health Mission with the National Rural Health Mission being the other sub-mission; the same page records special focus on the Empowered Action Group States, the North Eastern States, Jammu and Kashmir and Himachal Pradesh, a thrust on a fully functional, community-owned, decentralised health delivery system with inter-sectoral convergence acting on determinants such as water, sanitation, education, nutrition and social and gender equality, and measurement of facilities against the Indian Public Health Standards, so the mission is urban as well as rural and treating it as rural-only is a factual error.
- The health mission is a centrally sponsored scheme with Centre-State cost sharing that supports accredited social health activists and auxiliary nurse midwives, free drugs and diagnostics, mobile medical units and maternal-health support of the kind delivered through the maternity protection scheme, while State health agencies implement the hospitalisation pillar at State level and may add State-funded top-up schemes; naming this two-layer arrangement is what allows an answer to explain why identical national schemes produce unequal outcomes across States.
- The Mental Healthcare Act, 2017 supplies a rights framework and the national tele-mental-health service expands access, but the owners record expressly that neither replaces district-level professionals and continuity of care; this is the folder's standard illustration that a statutory right and a technology channel together still leave a human-resource gap, and it is directly usable in any answer that asks whether a legal entitlement has been converted into a service.

Qualified conclusion: National money becomes health only through State delivery capacity, so the assessment must connect financing, standards, workforce and inter-sectoral determinants, and must end by naming the condition on which the verdict would reverse.

Demand decoding: The directive **assess** requires a direct position on “Assess the federal and capacity conditions under which national health financing produces...”, every clause, constitutional/statutory/policy distinction, rights-holder and institution map, eligibility/exclusion and delivery chain, intersectional and regional variation, named

Indian evidence, accountability and qualified conclusion.

Detailed examiner-grade model answer:

Introduction and thesis: National money becomes health only through State delivery capacity, so the assessment must connect financing, standards, workforce and inter-sectoral determinants, and must end by naming the condition on which the verdict would reverse.

Analytical body:

- 1. Claim and named evidence:** The owners record that the health mission, free drugs and diagnostics initiatives and the health-infrastructure mission strengthen systems while uneven State capacity and workforce vacancies limit outcomes, and that Union financing and standards depend on State delivery capacity because health is delivered by the States; the answer consequence is that a national financing announcement is a necessary but not sufficient condition, and that a graded verdict on any national health programme must be conditioned on the delivery capacity of the State in question. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
- 2. Claim and named evidence:** The National Health Mission's own official page, read on 2 September 2026, records that the National Rural Health Mission was launched by the Prime Minister on 12 April 2005 to provide accessible, affordable and quality health care to the rural population and especially to vulnerable groups, and that the Union Cabinet, by its decision dated 1 May 2013, approved the launch of the National Urban Health Mission as a sub-mission of an over-arching National Health Mission with the National Rural Health Mission being the other sub-mission; the same page records special focus on the Empowered Action Group States, the North Eastern States, Jammu and Kashmir and Himachal Pradesh, a thrust on a fully functional, community-owned, decentralised health delivery system with inter-sectoral convergence acting on determinants such as water, sanitation, education, nutrition and social and gender equality, and measurement of facilities against the Indian Public Health Standards, so the mission is urban as well as rural and treating it as rural-only is a factual error. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
- 3. Claim and named evidence:** The health mission is a centrally sponsored scheme with Centre-State cost sharing that supports accredited social health activists and auxiliary nurse midwives, free drugs and diagnostics, mobile medical units and maternal-health support of the kind delivered through the maternity protection scheme, while State health agencies implement the hospitalisation pillar at State level and may add State-funded top-up schemes; naming this two-layer arrangement is what allows an answer to explain why identical national schemes produce unequal outcomes across States. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.
- 4. Claim and named evidence:** The Mental Healthcare Act, 2017 supplies a rights framework and the national tele-mental-health service expands access, but the owners record expressly that neither replaces district-level professionals and continuity of care; this is the folder's standard illustration that a statutory right and a technology channel together still leave a human-resource gap, and it is directly usable in any answer that asks whether a legal entitlement has been converted into a service. **Analysis:** Connect right or deprivation -> institutional duty and eligibility rule -> frontline implementation -> differentiated access and outcome -> grievance, audit or course correction. **Qualification:** State internal group and regional variation, legal/operative status, denominator, fiscal or causal limit, accessibility, dignity, privacy, portability or residual exclusion.

Counter-position / limit: A constitutional promise, prohibition, reservation provision, scheme catalogue, allocation, registration count or headline statistic cannot alone establish accessible implementation, adequate benefit, dignity, remedy or attributable outcome; test eligibility, institutions, frontline capacity, intersectionality, regional variation, grievance and evidence status.

Qualified conclusion: National money becomes health only through State delivery capacity, so the assessment must connect financing, standards, workforce and inter-sectoral determinants, and must end by naming the condition on which the verdict would reverse.

Executable exam-length answer / compression plan: For 20 marks, spend one-sixth of the time decoding the directive and drawing right/need -> institution -> eligibility -> delivery -> outcome -> remedy; state a thesis; write four to seven claim -> named evidence -> analysis -> qualification points; reserve the final minute for dignity, accessibility, portability, grievance, denominator, fiscal and causal limits.

Why this earns marks: The answer obeys the directive, explains mechanisms rather than listing schemes, uses named India-centric evidence and preserves legal, institutional, group, eligibility, implementation, fiscal, data and outcome distinctions.

How to improve this answer: For “Assess the federal and capacity conditions under which national health financing produces...”, replace the weakest scheme-catalogue point with one named rights-holder, duty-bearer, eligibility bottleneck, safeguard, grievance route, process/outcome indicator and source-status qualification.